GOLDEN YEARS

IN-HOME SERVICES AGENCY · HOME CARE

Non-Medical Home Care Policy and Manual

Non-skilled in-home care · Chapter 246-335 WAC (Home Care category)

Golden Years Home Health Supported Living LLC

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Washington

2026 Administrative Policies and Procedures · Updated 09/16/2026

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Date initiated: _____	**Date revised:** _____

ORGANIZATION AND ADMINISTRATION

MISSION STATEMENT

Policy

Golden Years Home Health Supported Living LLC the Owner has adopted a written mission statement that reflects its commitment to the provision of efficient, effective, quality home care services to clients in the communities we serve. The agency will serve its clients with dignity and comfort, prioritize service goals based on client/representative input, and deliver timely, coordinated, and culturally sensitive services.

Purpose

To serve as the guiding principles for the agency's policies, procedures, services, and programs.

Procedure

1. The mission statement is established with appropriate, if applicable, and administrative personnel input and review, and approved by the Governing Body.
2. The mission statement is reviewed, revised (if needed), and approved at least every 36 months by the Owner/Governing Body.
3. The mission statement is included in all orientation.
4. A written copy of the mission statement is available for clients, referral sources, and other interested parties.

Missionstatement

Golden Years Home Health Supported Living LLC is dedicated to providing excellent and respectful home care agency services to clients served in our communities that are provided by qualified and dedicated personnel in a fiscally responsible manner.

POLICY AND PROCEDURE DEVELOPMENT AND IMPLEMENTATION

Definitions

Policy: A statement of principles formulated, approved, and enforced by Owner/designees to direct and limit actions as related to a specific subject.

Procedure: A set of established steps or a prescribed method to be followed routinely for optimally effective achievement of a desired result.

Policy

Golden Years Home Health Supported Living LLC acknowledges its responsibility to review the Washington Administrative Codes (WAC 246-335-300 through 246-335-355) referenced in the development of our policies and procedures (P&P). We affirm our commitment to follow directives provided by regulatory authorities, ensuring compliance and upholding standards of excellence in our operations. We also acknowledge that we have reviewed and agreed to follow the directives in WAC 246-335-405 through 246-335-455 apply to licensed Home Care services agencies. Golden Years Home Health Supported Living LLC follows an established process for the development, approval, implementation, and access to its policies and procedures. Agency policies and procedures reflect an emphasis of quality service, current and ethical standards of practice, client rights, and the agency's mission Statement. Policies and procedures will be reviewed and revised if needed at least annually by the Owner/designees. Administrative policies and procedures delineate lines of authority and responsibilities for governance, planning, public communications, and financial control of the agency. Operational policies and procedures define personnel management, infection control, and compliance with state employment regulations. Agency client care policies and procedures are developed by the clinical management with staff input, reviewed and approved by the Owner/Designees.

Purpose

1. To ensure accountability and responsibility in the oversight, review, and approval of policies and procedures which direct the services of the agency.
2. To delineate the authority for the approval process.

Procedure

1. The home care agency is licensed in the state of Washington
2. Policies and procedures will be written in a standardized format, approved by the Owner, maintained on the Internet, and organized for efficient reference and access.

3. The Owner/designees will review annually all policies and procedures for needed changes.
4. The Supervisor may develop or amend policies and procedures and forward them to the Owner/Administrator for review. All policies that are amended or developed must meet with the Owner's approval. Upon approval, the Owner/Administrator will sign and date the "Acceptance of the." This form will appear at the front of the manual.
5. Once approved, the Owner/Administrator will implement the policy notifying affected persons and distributing related instructions (if needed) to related personnel. If a policy or procedure requires competency testing to confirm new practice, the Owner/Administrator or designee will arrange for that competency testing.
6. Policies and procedures may be shared with community resources and facilities serving the agency clients for continuity of care, clients or their representatives, and state authorities upon request. Personnel will have access to all policies including personnel policies.

REFERENCES

WAC 246-335-415

AGENCY RIGHTS AND RESPONSIBILITIES

Policy

Golden Years Home Health Supported Living LLC Owner/Designees understands that as a licensed home care agency in the state of Washington that has certain rights and responsibilities and will adhere to those responsibilities.

Purpose

To ensure all responsibilities are met under WAC 246-335-335

Procedure

1. An applicant or agency will:
    a. Comply with the provisions of chapter 70.127 RCW and this chapter;
    b. Maintain at least one in-state office location where records are kept, secured, and accessible;
    c. Display the license issued by the Department of Health in an area within the main office that is visible to the public, including branch office locations if applicable.
    d. Cooperate with the Department of Health during surveys which may include reviewing agency records and conducting on-site visits with client or client consent.
    e. Notify the Department of Health in writing within thirty days of any of the following:
        i. Changes of administrator, director of clinical services, or supervisor of direct care services;
        ii. Beginning or ceasing operation of any office location(s);
        iii. Ceasing operation of home care, home care, hospice, or hospice care centers services;
        iv. Decreasing home care, home care, or hospice service areas; and
        v. Survey or monitoring decisions and findings from a Department of Health approved accrediting organization, DSHS, or area agency on Aging for agencies approved through substantial equivalency according to WAC 246-335-350.
5. Request approval to expand state licensing home care, home care, or hospice service areas, by submitting an amended application, on forms provided by the Department of Health, which lists all service areas for which the agency is seeking approval and information based on the criteria in WAC 246-335-415(6), 246-335-515(6), or 246-335-615(6).

a. To request approval to expand services areas for home care or hospice agencies that are certified by the Centers for Medicaid Services, the agency will contact the Department of Health's certificate of need program and follow their separate application and approval process.
b. The agency must receive the Department of Health approval for service area expansion prior to providing services in the proposed expanded service area(s).

6. May: understands that the Department of Health Conduct unannounced surveys at any time and at least once during a licensure period, except for agencies approved through substantial equivalency according to WAC 246-335-350;
   a. Conduct unannounced surveys at any time and at least once during a licensure period, except for agencies approved through substantial equivalency according to WAC 246-335-350
   b. Survey the agency at any time if the Department of Health has reason to believe the agency is providing unsafe, insufficient, inadequate, or inappropriate client or client care;
   c. Investigate any person suspected of advertising, operating, managing, conducting, opening, or maintaining an in-home services agency or providing in-home services, including hospice care center services, without a license unless exempt from licensure under RCW 70.127.040 and 70.127.050;
   d. Investigate complaints against in-home services agencies that allege noncompliance with chapter 70.127 RCW and this chapter;
   e. Investigate allegations of noncompliance with RCW 43.43.830 through 43.43.842, and, when necessary, in consultation with law enforcement personnel; and
   f. Require the agency to complete additional criminal history background checks and disclosure statements for an individual associated with the agency or having direct contact with children under sixteen years of age, people with developmental disabilities, or vulnerable people if the Department of Health has reason to believe that offenses specified under RCW 43.43.830 have occurred since completion of the previous disclosure statement and criminal background inquiry.
7. AGENCY cooperates with the Department of Health during surveys which may include reviewing agency records and conducting on-site visits with clients or client consent
8. AGENCY understands that to ensure compliance with chapters 246-335 WAC and 70.127 RCW, the Department of Health may:

a. Issue a statement of deficiencies following a survey which identifies noncompliance with chapter 70.127 RCW and this chapter. The statement of deficiencies will include a brief statement of what is required to achieve compliance, the date by which the Department of Health requires compliance to be achieved, contact information for any technical assistance services provided by the Department of Health, and notice of the process and timeline for requesting an extension of time for good cause to achieve compliance.

b. Require the agency to submit a written plan of correction within ten working days of receipt of the statement of deficiencies to address deficiencies that are determined to not be significant, broadly systemic, or recurring.

c. Under this chapter, a "plan of correction" is a form developed by the Department of Health and completed by the agency that describes:

i. How each deficiency will be or was corrected;

ii.What measures or monitors will be put in place to ensure the deficiencies do not occur;

iii. Who is responsible for correcting each deficiency; and

iv. When each deficiency will be or has been corrected.

d. The agency will complete all corrections within sixty days unless the Department of Health authorizes a longer time frame for correction of some of the deficiencies. Implementation of the plan of correction is subject to verification by the Department of Health.

9. Issue a directed plan of correction following a survey which identifies

a. Deficiencies that are determined to be significant, broadly systemic, and are not in the categories specified in RCW 70.127.180(3) for which the agency may be subject to a formal enforcement action.

b. The Department of Health may choose to issue a directed plan of correction if the Department of Health deems the plan of correction submitted by the agency to be unacceptable because the actions proposed will not correct the deficiencies, will take too long, or fails to protect the public health, safety, and welfare;

c. Implementation, progression, and completion of the directed plan of correction may be verified by the Department of Health.

10. Deny, restrict, condition, modify, suspend, or revoke a license and assess civil monetary penalties as authorized in RCW 70.127.170 and 70.127.180(3).

11. Summarily suspend or limit a license pending an enforcement action as authorized in RCW 34.05.479 if deficiencies are identified that pose an imminent risk of harm to clients or patients.

12. The agency will sign and return the completed plan of correction or directed plan of correction to the Department of Health on or before the required date.
13. Neither the requirement to submit a plan of correction nor a directed plan of correction is an agency action for which an ad-judicative hearing must be afforded to the agency.
14. The failure to correct the deficiencies in the statement of deficiencies or the failure to comply with the plan of correction or directed plan of correction may be grounds for an action against the license or a civil monetary penalty under RCW 70.127.170 and 70.127.180(3).
15. If the Department of Health initiates action to deny, restrict, condition, modify, suspend, or revoke a license or to assess civil monetary penalties as authorized in RCW 70.127.170 and 70.127.180(3), the agency will be afforded an adjudicative hearing opportunity under chapter 34.05 RCW, RCW 43.70.115, and chapter 246-10 WAC.
16. 13.Responding to a "Directed plan of correction" developed by the Department of Health, based on a current statement of deficiencies and the agency 's survey history, which specifies the corrective actions the agency will take and time frames in which those actions are to be completed by submitting to the Department of Health within ten working days of receipt, a written plan of correction for each deficiency. It is further understood that all corrections will be completed within sixty days after the survey exit date, unless otherwise specified by the Department of Health; and no longer than ninety days after the survey exit date, a progress report describing corrections made and ongoing monitoring actions, unless otherwise specified by the Department of Health.
    a. A written service area approval or denial
17. It is understood that the licensed Home Care agency and The Department of Health are responsible to provide the agency with:
    a. A written statement of deficiencies found during a survey; and
18. may:
    a. Discuss findings observed during a survey with the surveyor; and
    b. Discuss the statement of deficiencies, denial of service area, or denial of an exemption with the

Department of Health's manager;

19. Further it is understood that the agency has the right to respond to and contest a statement of charges according to the following provisions:
    a. RCW 43.70.115, Department of Health authority for license approval, denial, restriction, conditioning, modification, suspension, and revocation;
    b. Chapter 34.05 RCW, the Administrative Procedure Act; and

c. Chapter 246-10 WAC, Adjudicative proceedings.

DEPARTMENT OF HEALTH RESPONSIBILITIES.

1. The Department of Health may, in accordance with chapter 70.127 RCW:
   a. Issue an initial license including the in-home services category(ies) and Department of Health approved service area(s), if applicable, for twelve months following submission of a completed application and appropriate fee, and following a survey that documents the applicant meets all the requirements of this chapter;
   b. Issue a renewal license including the in-home services category(ies) and Department of Health approved service area(s), if applicable, for a twenty-four-month period following submission of a completed application and appropriate fee;
   c. Issue a license for change of ownership including the in-home services category(ies) and Department of Health approved service area(s), if applicable, to the new agency for the remainder of the current license period following submission of the required information and appropriate fee, under WAC 246-335-330.
2. The Department of Health may:
   a. Conduct surveys at any time and at least once during a licensure period to determine compliance with chapter 70.127 RCW and this chapter, except for agencies with deemed status;
   b. Conduct one licensing survey inclusive of all in-home services categories;
   c. Investigate any person suspected of:
      i. Advertising, operating, managing, conducting, opening or maintaining an in-home services agency or providing in-home services, including hospice care center services, without a license unless exempt from licensure under RCW 70.127.040 and 70.127.050; or
      ii. Survey the agency at any time if the Department of Health has reason to believe the agency is providing unsafe, insufficient, inadequate, or inappropriate care;
   d. Investigate allegations of noncompliance with RCW 43.43.830 through 43.43.845, when necessary, in consultation with law enforcement personnel;
   e. Require agencies to complete additional disclosure statements and background inquiries for an individual associated with the agency or having direct contact with children under sixteen years of age, people with developmental disabilities, or vulnerable adults if the Department of Health has reason to believe that offenses specified under RCW 43.43.830 have occurred since completion of the previous disclosure statement and criminal background inquiry’

f. Approve, deny, or revoke requests by home care, hospice or home care agencies for initial service area or service area expansion based on:
   i. The agency's demonstrated ability or inability to comply with this chapter as illustrated by substantiated complaint history, survey outcomes or enforcement action; and
   ii. Evidence of the agency 's ability or inability to manage and supervise services throughout the approved service area under criteria listed in WAC 246-335-415;
g. Approve, deny, restrict, condition, modify, suspend, or revoke a license under this chapter under RCW 70.127.170 and 70.127.180(3);
h. Issue a statement of deficiencies following a survey which identifies noncompliance with chapter 70.127 RCW and this chapter; and
   i. Prepare and serve upon the agency or applicant at the earliest practical time a statement of charges following a survey which identifies noncompliance with chapter 70.127 RCW and this chapter. The statement of charges shall be accompanied by a notice that the agency or applicant may request a hearing to contest the charges.

3. An in-home services agency that is approved to provide home care services under contract with and monitored by DSHS or the AAA to provide home care services is not subject to the state survey requirement when the following have been met:
   a. The Department of Health has determined that the applicable survey standards of DSHS or the AAA are substantially equivalent to those required by this chapter and chapter 70.127 RCW;
   b. Verification that an on-site DSHS or AAA monitoring has been conducted during the previous twenty-four months;
   c. The agency has submitted to the Department of Health a copy of the final executed contract; and
   d. DSHS or the AAA submits to the Department of Health copies of monitoring reports and other relevant reports or findings that indicate compliance with licensure requirements, and the monitoring reports include samples of private pay clients, if applicable.

REFERENCES

WAC 246-335-350 WAC 246-335-415 WAC 246-335-335

PLAN OF OPERATIONS

Policy

Golden Years Home Health Supported Living LLC has an established plan of operations for the agency and has a system in place to assure updates of the plan of operation that reflects current practice, and services provided by the Golden Years Home Health Supported Living LLC, and state and local laws.

Purpose

1. To ensure accountability and responsibility for compliance with state requirements.
2. To delineate the authority for the approval process.

Procedure

1. Golden Years Home Health Supported Living LLC has established and implemented policies and procedures that, with a written plan of operation, includes:
    e. A description of the organizational structure;
    f. Personnel job descriptions according to WAC 246-335-425(2);
    g. Responsibilities of contractors and employees.
    h. Services to be provided;
    i. The days and hours of agency operation; and
    j. Criteria for management and supervision of home care services throughout all approved service areas, which includes:
        i. How the initial intake and development of the plan of care will be completed per WAC 246-335-440;
        ii. How supervision of personnel and volunteers and monitoring of services provided by contractors will occur which meet the requirements of WAC 246-335-445;
        iii. How performance evaluations for personnel and volunteers and evaluation of services provided by contractors will be conducted per WAC 246-335-425 (14) and (15); and
        iv. How the quality improvement program required in WAC 246-335-455 will be applied throughout all approved service areas.
    k. A process to inform clients of alternative services prior to ceasing operation or when the agency is unable to meet the client's needs;
    l. A plan for preserving records, including the process to preserve or dispose of records prior to ceasing operation according to WAC 246-335-450 (7) and (8); and

m. Time frames for filing documents in the client records;

n. Emergency preparedness that addresses service delivery when natural disasters, man-made incidents, or public health emergencies occur that prevent normal agency operation. Include, at minimum: Risk assessment and emergency planning, communication plan, informing local officials of clients in need of evacuation, and staff training and drills to test emergency plan;

2. Golden Years Home Health Supported Living LLC has an identified Administrator who is responsible for:

a. Oversee the day-to-day operation and fiscal affairs of the agency;

b. Implement the provisions of this section;

c. Designate in writing an alternate to act in the administrator's absence;

d. Provide management and supervision of services throughout all approved service areas according to state rules;

e. Arrange for necessary services;

f. Keep contracts current and consistent with WAC 246-335-425(4);

g. Serve as a liaison between the agency, personnel, contractors, and volunteers;

h. Ensure personnel, contractors and volunteers are currently credentialed by the state of Washington, when appropriate, according to applicable practice acts and consistent with WAC 246-335-425(5);

i. Ensure personnel, contractors and volunteers comply with the agency 's policies and procedures;

j. Implement a quality improvement process consistent with WAC 246-335-455;

k. Manage recordkeeping according to WAC 246-335-450;

l. Ensure supplies and equipment necessary to client care are available, maintained, and in working order;

m. Ensure the accuracy of public information materials; and

n. Ensure current written policies and procedures are accessible to personnel, contractors, and volunteers during hours of operation.

o. Agency Administrator is responsible for continuing to update its plan of operation to reflect current practice, services provided by the agency, and state and local laws.

REFERENCES

WAC 246-335-415 WAC 246-335-425 WAC 246-335-440 WAC 246-335-445 WAC 246-335-450 WAC 246-335-455

ORGANIZATIONAL STRUCTURE

Policy

Golden Years Home Health Supported Living LLC is a Home Care Agency operated by administrative personnel. The agency follows its established plan of authority and responsibility.

Purpose

1. To ensure accountability and responsibility in the provision of effective home care services.
2. To delineate lines of authority.

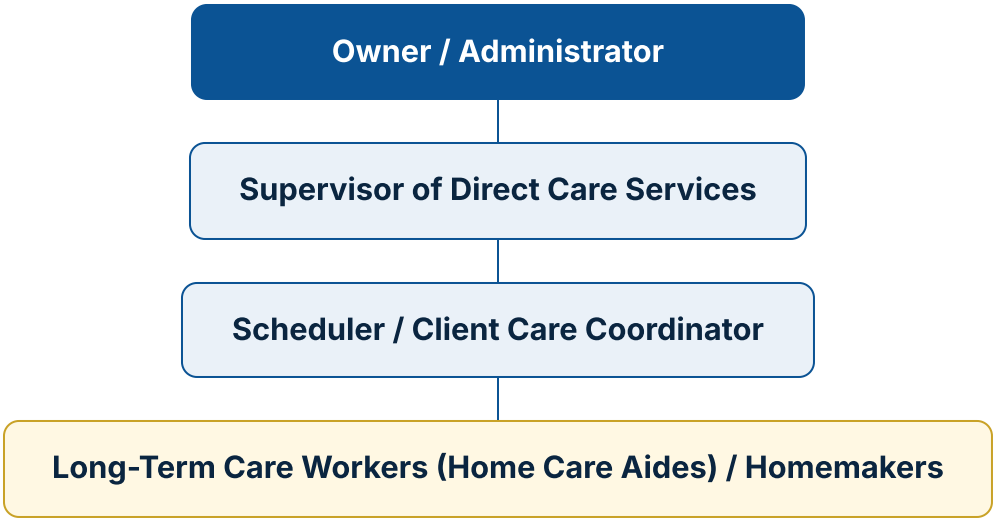
Procedure

1. The agency is a licensed home care agency with articles of incorporation and legal authority to operate under the laws of the state of Washington
2. The Owner is ultimately responsible for all aspects of the home care agency operations and appoints the Owner/Administrator to handle day-to-day operations.
3. The Owner/Administrator assumes responsibility for the daily operations of the agency.
4. The agency's hours of operation are as follows:
    a. The agency's business hours are Monday through Friday from 8:00 AM a.m. to 4:00 PM p.m.
    b. The office will be closed on all public holidays.
    c. Agency services are accessible 24 hours per day, 7 days per week, including weekends and holidays.
    d. On-call services are available after regular business hours including weekends and holidays.
5. The agency serves the following counties: King and Pierce.
6. The client services and personnel operations of the agency are the responsibility of the Supervisor. In the absence of the Owner/Administrator, the Supervisor acts as the alternate and has been designated by the Owner to assume responsibility for the agency and the services provided. In the absence of both the Owner/Administrator and the Supervisor, an individual with the necessary experience may be designated to assume supervisory responsibilities on a temporary basis.
7. The owner/administrator is responsible for reviewing and making identified changes in policies and procedures manual.

8. The agency maintains a current organizational chart that reflects the lines of authority and accountability of all personnel. Amendments will be documented as changes in the agency occur.
9. Appropriately qualified personnel are employed and report to the Supervisor to deliver client services.

REFERENCE

WAC 246-335-425



ORGANIZATIONAL CHART

PUBLIC DISCLOSURE

Policy

Golden Years Home Health Supported Living LLC will provide information related to the agency policies and services to personnel, clients/family, and community resources and facilities upon request, or as part of its ongoing educational materials, admission packets, and other materials as deemed appropriate. In addition, the agency will provide full disclosure, upon request, and as required for applications or state inquiries about its ownership and control including the names and addresses of its administrative personnel.

Purpose

To release full disclosure to the public and other interested parties.

Procedure

The following information is available upon request:

1. Mission statement and philosophy
2. Client rights
3. All licensing, credentials, and accreditation reports
4. The annual report on services provided
5. List of offices: parent, sub-unit, and branches
6. The agency's ownership
7. Services provided, hours of operation, and geographic coverage area

LEGAL REQUIREMENTS

Policy

Golden Years Home Health Supported Living LLC shall establish procedures for monitoring its continued compliance with all applicable local, state laws and regulations, including agency and personnel licensure regulations.

Purpose

To ensure compliance with the legal requirements for state laws.

Procedure

1. The Administrator or designee is responsible for assuring that licensing renewal is done at the designated time and with required documents.
2. At least thirty days before the expiration date of the current license, the agency must submit it to the Department of Health:
   a. A completed renewal of licensing application on forms provided by the Department of Health.
   b. Evidence of continuing insurance coverage according to WAC 246-335-320 (2)(b);
   c. A completed and signed full-time equivalent employee worksheet on forms provided by the Department of Health
   d. Copies of the most current criminal history background checks and disclosure statements for the administrator, director of clinical services, or supervisor of direct care services in accordance with WAC 246-335-320 (2)(d). All criminal history background checks and disclosure statements required under this chapter must be renewed within two years from the date of the previous check.
   e. A description of the agency's current organizational structure. For example, an organizational chart or narrative describing key positions and duties.
   f. Documentation required under WAC 246-335-350, if initially applying or reapplying for substantial equivalency status;
   g. Other information as required by the Department of Health; and
   h. Fees specified in WAC 246-335-990.
3. The Administrator or designee will be responsible for setting up a tracking system to monitor compliance with agency services and a system for response to any variances identified, including but not limited to:
   a. DOH licensing, Business, state, and individual licenses, and registrations.

   b. All required postings: Family Leave Act, Equal Employment Opportunity Act, Occupational Safety and Health Administration (OSHA) forms, Job Safety and Health Protection Act, Minimum Wage, Polygraph Protection Act, American with Disabilities Act, Civil Rights Act, Fair Labor Standards Act, Blood Borne Regulations and Procedures, and state poster requirements.
   c. OSHA and Medical Device Act mandates
   d. Employment regulations
   e. Immigration requirements including revised I-9 form for all new hires
   f. Centers for Disease Control (CDC) Infection Control
   g. Professional practice standards and state practice acts
   h. Accrediting body standards
   i. Health Insurance Portability and Accountability Act (HIPAA) rules
   j. Protected Health Information (PHI) regulations
   k. Law enforcement requests and subpoenas
   l. Open investigation(s) by any regulatory or governmental authority

4. If an employee is contacted concerning a legal matter, which might involve the home care agency either directly or indirectly—either in the form of a subpoena, by an attorney, or other person—the employee will notify his/her supervisor and/or the Administrator before taking any action.
5. If a survey has found deficiencies, a "Plan of correction" means an agency authored document based on a statement of deficiencies, and includes specific corrective actions that must be taken to correct identified deficiencies to include, but not limited to:
   a. How the deficiency will be or was corrected.
   b. What measures or monitors will be put in place to ensure the deficient practice does not recur.
   c. Who is responsible for the correction; and
   d. When each deficiency will be corrected.

REFERENCES

WAC 246-335-305 WAC 246-335-320 WAC 246-335-350

CLOSURE/TRANSFER OF OWNERSHIP

Policy

In the event that Golden Years Home Health Supported Living LLC must cease operations, it will submit in writing 30 days' notice to the client and all related parties off its intention to either transfer ownership to another entity or facilitate safe transfer of the agency's clients to another agency that serves the community and is in good standing with its licensure and/or accreditation.

A change of ownership of an in-home services agency includes any of the following:

1. Transferring ownership, either whole or part, to a new owner.
2. Dissolving a partnership or corporation.
3. Merging with another entity taking on that entity's identity.
4. Consolidating with another entity, creating a new identity; or
5. A change of the unified business identifier number and/or state employer identification number.

Purpose

To ensure that client services are not interrupted and there is time for the transition of services from the agency to another entity.

Procedure

1. At least thirty days prior to changing ownership of an in-home services agency, the Licensed Agency will submit in writing to the Department of Health:
   e. The full name, address, and phone number of the current and prospective owner.
   f. The name, address, and phone number of the currently licensed home care agency and the name under which the prospective agency will operate.
   g. Date of the proposed change of ownership; and
   h. Any change in office location and service area, if relevant.
2. At least thirty days prior to changing ownership of an in-home services agency, the prospective new owner must submit a change of ownership application, on forms provided by the Department of Health: The prospective new owner will be required to submit:
   a. Information listed in 246-335-320 (2)(a) through (f)) through (d); and
   b. The change of ownership fee specified in WAC 246-335-990.

3. The Department of Health may conduct a survey after the change of ownership approval to assess the new ownership's delivery of care to clients or patients.
4. Should the agency cease operations, clients will be transferred to another home care of their choice.
5. Closed records will be stored or destroyed according to the state requirement for record retention.
6. All records shall be retained or disposed of in a confidential manner according to the time frames specified in RCW70.02.

REFERENCES

WAC 246-335-330

WAC 246-335-320

PERSONNEL AND CONTRACTORS

EQUAL EMPLOYMENT AND AFFIRMATIVE ACTION

Policy

It is the policy of Golden Years Home Health Supported Living LLC that all hiring, placements, promotions, and other personnel-related activities will be consistent with state Equal Opportunity and Affirmative Action regulations. The agency will provide equal employment opportunity to all without regard to age, sex, sexual orientation, marital status, disabled veteran or Vietnam Era Veteran Status, race, creed, religious affiliation, color, national origin, or the presence of any sensory, mental, or physical handicap unless based upon a bona fide occupational qualification.

Purpose

To ensure compliance with all Washington state regulations as they relate to fair and equal employment opportunities.

Procedure

1. Job applicants will be assessed based on their ability to perform the job for which they are interviewed without regard to non-job-related criteria. The employee's opportunity to work at the agency will not be jeopardized by the employee's refusal to answer a non-job-related question, except for questions regarding their criminal record.
2. Additionally, no person shall be selected on the basis of age, sex, sexual orientation, marital status, race, creed, religious preference, color, national origin, or the presence of any sensory, mental, or physical handicap be denied services, benefits, or be discriminated against in any manner.
3. The Americans with Disabilities Act guidelines will be followed in making reasonable accommodation for qualified applicants.

REFERENCES

WAC 246-335-425

CRIMINAL BACKGROUND CHECKS AND DISCLOSURES

Policy

Golden Years Home Health Supported Living LLC will comply with state requirements and obtain criminal history from WSP, and a state approved disclosure statement on all new employees.

Purpose

1. To follow Washington state laws regarding criminal history, disclosure, and background inquiries.
2. To protect the clients, personnel, and reputation of the agency.

Procedure

1. Agency will comply with Washington state patrol criminal background inquiries and State approved disclosure statements under RCW 43.43.830 through 43.43.845 for the administrator, supervisor of direct care services per WAC 388-78A-2020 initial license, 388-78A-2025 renewal, and 388-78A-2040, 388-78A-2045 Change of ownership.
2. These regulations include personnel, contractors, volunteers, students, and any other individual associated with the agency having direct contact with children under sixteen years of age, people with developmental disabilities or vulnerable persons according to RCW 43.43.830 through 43.43.842.
3. All criminal background checks and disclosure statements will be repeated at least every two years.
4. All applicants will fill out the authorization for the criminal background check and sign indicating acknowledgment and acceptance of the background check and disclosure statement.
5. The agency will only conditionally employ or contract with individuals until the criminal history background inquiry is returned clean by the Washington State Patrol.
6. The agency will not employ or contract with persons having direct contact with children or vulnerable adults if they have been:
    a. Convicted of any crime against children or other persons
    b. Convicted of a crime relating to financial exploitation of a vulnerable adult
    c. Convicted of crimes related to drugs as defined in RCW 43.43.834(2) (a-e)
    d. Found guilty in any dependency action to have sexually assaulted/exploited or physically abused any minor.

e. Found by a court, in domestic relations proceedings, to have sexually assaulted/exploited or physically abused any minor.

f. Found in any disciplinary board final decision to have physically abused or exploited any minor or developmentally disabled person or financially exploited any vulnerable adult

7. Character, competence, and suitability determination conducted for personnel, contractors, volunteers, and students whose background check results reveal non-disqualifying convictions, pending charges, or negative actions. Factors to consider when making a determination include, but are not limited to:
   a. Whether there is a reasonable, good faith belief that they would be unable to meet the care needs of the client.
   b. Level of vulnerability of the client under their care.
   c. Behaviors from convictions, charges, negative actions, or other adverse behaviors.
   d. Pattern of offenses or other behaviors that may put the client at risk.
   e. Number of years since the conviction, negative action, or other issue.
   f. Whether they self-disclosed the conviction(s), pending charge(s) or negative action(s).
   g. Other health and safety concerns; and
   h. Although the agency may determine, based on their assessment process, that an employee is suitable to work with vulnerable clients, the Department of Health has the final authority to deny, revoke, modify, or suspend any professional credential issues based on application and criminal background check information.
8. All disclosure statements and criminal history background inquiries will be maintained in a confidential and secure manner in agency personnel files.
9. Disclosure statements and background investigations shall be used for employment purposes only and shall not be disclosed except to the individual of inquiry, or an authorized state agent, or an auditor of the WSP.
10. The disclosure statement and criminal background investigation shall be made available to the DOH agent for review during employment and shall be retained for not less than 2 years after termination.
11. Employees will be verbally informed of the results of the background inquiry within 72 hours of receipt of a report and offered a copy of the inquiry report within ten (10) days of its receipt.

12. Additional disclosure statements or background inquiries for an individual having direct contact with vulnerable adults may be required if the agency has reason to believe that offenses specified under RCW 43.43.830 have occurred since completion of the initial criminal background inquiry.
13. The agency may require an additional background check if it has reason to believe specified offenses have occurred since the last background check.

REFERENCES

WAC 246-335-325 WAC 246-335-415 WAC 246-335-425 WAC 246-335-430

FINGERPRINTING

Policy

Golden Years Home Health Supported Living LLC is compliant with the State of Washington rule requiring finger printing for some employees. The Department of Health requires that all individuals who have not lived in the state in the last three years must go through fingerprinting.

After completing the WA State check, Golden Years Home Health Supported Living LLC will be notified by the department if it is determined that the applicant is to move forward with the fingerprinting process. Golden Years Home Health Supported Living LLC will provide the applicant with the fingerprint appointment and fingerprint notification forms printed out of BCS. The applicant or the agency will make a fingerprint appointment RIGHT AWAY. The applicant must bring his/her fingerprint appointment form and picture ID to the nearest IdentoGO site for fingerprinting. Applicants have the option of using law enforcement to complete fingerprints instead of using DSHS's contractor IdentoGO. Instructions on Submitting Fingerprint Hard Cards to BCCU. There may be a cost associated with this option. Once the applicant’s fingerprints have been taken, they are sent to the Washington State Patrol and the FBI. Golden Years Home Health Supported Living LLC will receive a copy of the results in BCS when the process is completed. Golden Years Home Health Supported Living LLC will check the status in the working grid in BCS for the status of the results.

Based on the applicant’s Result Letter, Golden Years Home Health Supported Living LLC will determine whether the candidate is qualified, disqualified, or require a Character, Competence and Suitability Review.

Purpose

To ensure that applicants for employment are free of criminal history and thus ensure the safety of the clients.

Procedure

1. All applicants for employment are required to complete a criminal background and state declaration at the time of hire. The Department will determine if the applicant is required to obtain fingerprint check through the Department of Health.
2. The applicant will pay the cost of the background check.
3. The Department of Health will notify applicants who need to undergo this background check. For these applicants, failure to submit fingerprints will delay processing of their applications.

4. No applicant for employment that has this requirement will be employed until such testing is completed, and the applicant has a clean criminal background history.

REFERENCES

WAC 246-335-425

PERSONNEL SELECTION

Policy

Golden Years Home Health Supported Living LLC considers its personnel to be the key component to its success in serving its clients and is committed to attract and retain qualified personnel to provide home care services. The personnel selection process will be consistently applied and include a completed application, interview, at least three reference checks, license, or certification verification (if applicable), educational requirements, competency checks (if applicable), criminal background check, immigration status, and such other items as appropriate to the position being filled and/or regulatory mandates.

The agency complies with all the requirements of EEOC and does not discriminate against anyone regardless of age, sex, sexual orientation, marital status, disabled veteran or Vietnam Era Veteran Status, race, creed, religious affiliation, color, national origin, or the presence of any sensory, mental, or physical handicap unless based upon a bona fide occupational qualification.

Purpose

1. To ensure a fair and consistent process for the selection of qualified agency personnel.
2. To comply with Equal Opportunity and Affirmative Action requirements.

Procedure

1. An application for employment will be completed by each applicant and reviewed by the appropriate management personnel.
2. The dates of employment, work experience, education, and training will be verified and documented before or during the interview and before a position is offered.
3. The applicant will be asked to give three references and grant permission to the agency to request verbal and written reference checks.
    a. No applicant will be hired without verification of two positive references.
    b. The interviewer will request that all verbal references be confirmed in writing.
4. The applicant must complete all pre-employment paperwork, including a criminal history background check form and disclosure form.
5. The agency will submit a criminal background check with a national or state firm for each employee. At a minimum, the background inquiry will include a search of state(s) where he/she has lived for felonies, misdemeanors, warrants, convictions, and traffic violations; a state warrant sweep; OIG exclusion list; and National Sex Offender list. A financial background review may also be required, if deemed necessary.

6. The agency considers character, competence, and suitability of all home caregivers and personnel who will have unsupervised access to clients.
7. Employment is conditional until completion of the background check and the candidate successfully completes all employment requirements and orientation for their position.
8. When all the required paperwork and background checks are completed and clear of any issues that would prevent hiring, the applicant will be offered employment, and an orientation session will be scheduled. The new employee will not be assigned to client services or their position until they have successfully completed the minimum components of the orientation process.
9. The agency may complete additional disclosure statements or background inquiries for an individual having direct contact with vulnerable adults if it has reason to believe that there may have been an incident warranting a repeat investigation.
10. All health-related forms will be kept in a separate health file for each employee separate from the general personnel files. For employees with direct client contact, the health assessment form shall:
    a. Be maintained by the agency.
    b. Include the records and pertinent documentation of health examinations.
    c. Be stored to be protected from loss, destruction, or unauthorized disclosure or use.
    d. Be retained for a minimum of three years following termination of employment.
11. The agency will be responsible for the TB screening confirmation and the Hepatitis B Vaccine offer to each new employee. (See Infection Control Policies.)
12. The Americans with Disabilities Act guidelines will be followed in making reasonable accommodation for qualified applicants.

REFERENCES

WAC 246-335-425

PERSONNEL REQUIREMENTS/CRITERIA

Policy

Golden Years Home Health Supported Living LLC has established requirements for all personnel and contractors. A non-discriminatory recruitment and selection process is followed. All applicants must be qualified for the positions they are seeking.

Purpose

To ensure that all personnel employed and contracted by the agency meet and maintain the appropriate qualifications for employment.

Procedure

All personnel, contractors and volunteers shall meet all the following criteria:

1. Employment criteria regarding discrimination consistent with chapter 49.60 RCW;
2. Meet the Job descriptions that contain responsibilities and are consistent with health care professional credentialing and scope of practice as defined in relevant practice acts and rules.
3. Positive references for personnel, contractors, and volunteers.
4. For agencies using a contracted employee, the agency will have a contract that at a minimum includes a description of the duties the contractor will perform, and a statement indicating that the contractor, not the employer, is responsible for withholding any necessary taxes. As with personnel and volunteers, contractors are subject to all applicable requirements in this chapter.
5. Credentials of health care professionals that are current and in good standing.
6. Criminal history background checks and state approved disclosure statements are required on hiring and must be renewed within two years from the date of the previous check and evert two years thereafter.
7. In-person orientation to current agency policies and procedures and verification of skills or training prior to independently providing client care. Examples of skills verification include written testing, skills observation, and evidence of previous training and experience such as long-term care worker training as detailed in RCW 74.39A.074 and certified nursing assistant training as detailed in WAC 246-841-400;
8. Ongoing training pertinent to client care needs.

9. Orientation and training in safe food storage, preparation, and handling practices consistent with the United States Food and Drug Administration's recommendations for "food safety at home" for personnel, contractors, and volunteers involved in food preparation services on behalf of clients. Personnel, contractors, and volunteers may not provide clients with homemade food items or baked goods that they themselves prepared.
10. Orientation and training in infection control practices, communicable disease testing, and vaccinations. Policies and procedures must include, at minimum:
    a. Standard precautions such as hand hygiene, respiratory hygiene and cough etiquette, and personal protective equipment.
    b. Availability of personal protective equipment and other equipment necessary to implement client plans of care.
    c. Tuberculosis (TB) infection control program. Key elements include, but are not limited to:
        i. Conducting a TB risk assessment for all new employees upon hire. Agencies must use a tuberculosis risk assessment form provided by the Department of Health. Based on risk assessment results, the agency's responsibility is to conduct TB testing of new employees. If TB testing is required, follow the Department of Health's tuberculosis risk assessment form testing recommendations.
        ii. Conducting an annual assessment of new TB risk factors for all employees. Agencies must use a tuberculosis risk assessment form provided by the Department of Health. Based on risk assessment results, the agency’s responsibility to conduct TB testing of employees. Retesting should only be done for persons who previously tested negative and have new risk factors since the last assessment; and
        iii. Ensuring workers receive TB related training and education at the time of hire or during new employee orientation. Training and education must be consistent with the Department of Health's tuberculosis program's online posted educational materials.
    d. Actions to take when personnel, volunteers, contractors, or clients exhibit or report symptoms of a communicable disease in an infectious stage in accordance with chapters 246-100 and 246- 101 WAC;
    e. Exposure to bloodborne pathogens such as Hepatitis B and HIV and other potentially infectious materials in compliance with the Department of Health and Labor and industries, chapter 296-823 WAC. Key elements include, but are not limited to:

i. Conducting an initial risk assessment of the environment in which personnel, volunteers, and contractors perform their assigned duties to determine occupational exposure. The results of the risk assessment will inform policy and procedure development and the level of employee training and education. Annually, agencies must determine if significant changes have occurred and that would require a new risk assessment to be carried out.

ii. If the risk assessment concludes that workers have a reasonably anticipated risk of occupational exposure to blood and other potentially infectious materials, agencies must offer workers the Hepatitis B vaccine series at the agency's expense. Workers have the right to decline the Hepatitis B vaccine series; and

f. The agency will document an annual review of applicable state and state health authority recommendations related to infection control practices, communicable disease testing, and vaccinations and update trainings and policies and procedures, as necessary.

11. All employees will have completed an application, reference checks, skills list, criminal disclosure, permission for the criminal background check permission, disclosure statement and supplied the necessary information to validate that the applicant is eligible to legally work in the United States.
12. All employees and contacted personnel will receive a formal written evaluation after one (1) year and then annually thereafter. The Supervisor completes the written evaluations and provides a face-to-face conference with the employee. Each employee receives a copy of their evaluation, and the original is placed in the employee's file. Evaluations are based upon but are not limited to:
    a. Adherence to program policies and procedures.
    b. Client's appreciation or complaints.
    c. Performance relevant to adherence to job tasks and care plan requirements.
    d. Record of attendance and adherence to client schedules.
    e. On-site observation of skills and care given to the clients.
13. All personnel applying for a field position will complete a competency skills checklist for their position. All skills validated through the licensure or certification process will be accepted as verified. All required competencies will be performed and documented.
14. As required by Washington State, current cardiopulmonary resuscitation (CPR) training per agency requirements.
15. Verification of current TB screening will be done for all direct care personnel as may be required by the local Health Department of Health. The results are stored in the employee health file.

16. All caregiving staff will be offered the Hepatitis B vaccination at the agency's expense if they have not already received the vaccine. Employees have a right to decline the vaccine. A record of the decline or acceptance must be kept in the employee health record which is separate from the employee file.
17. HIV/AIDS education is required of all direct care personnel and is verified through state licensing or certification.
18. Washington state I-9 Naturalization and Immigration forms will be completed for all personnel hired by the agency and maintained in a separate binder.
19. All direct care employees who have not already been vaccinated will be encouraged to obtain the Hepatitis vaccine paid for by the agency. Employees rejecting inoculation will document their choice.
20. All new employees shall be provided with disposable gloves, disposable CPR mask and PIN identification to be used while on assignment for the agency.
21. Employee performance and client care are monitored on an ongoing basis.

REFERENCES

WAC 246-335-420 WAC 246-335-425

CONDITIONS OF EMPLOYMENT

Policy

Golden Years Home Health Supported Living LLC strives to make it clear to potential personnel what to expect from the agency and what the agency expects from personnel so that the decision to join our personnel is a positive experience.

Purpose

To define expectations for a positive working experience for both the employee and the agency in a fair and clear manner.

Procedure

1. The agency provides potential employees with information about the company and its work environment.
2. The agency treats its personnel with respect, providing written information about its pay and benefit programs, its working hours and specific job responsibilities, an orientation to the position, its grievance process and ongoing supervisory support.
3. In return, the agency expects personnel to treat agency personnel and clients with respect, to honor their commitment for working hours and job performance as defined in their job description, to maintain client, employee, and agency confidentiality, notify the agency of unplanned absences as soon as possible, and ask for assistance if needed.
4. Employment may be full-time, part-time, on-call, contingent, salaried, per visit, per hour, or per diem. The specific category each employee fills are included in the written offer of employment.
5. Personnel will be asked to sign a statement that they will not solicit money or work from clients or other personnel or attempt to encourage clients or personnel to move to another agency for service.
6. Every effort will be made by the agency to provide as many hours of work as the employee committed to and desires. The employees must let the agency know of their availability for work each Friday for the following week.
7. The agency hopes that the employee enjoys working here and stays as long as that is true. If the employee decides to leave, the agency asks for at least one week’s notice to secure alternate coverage for the position. An exit interview will be offered to help the agency learn about why personnel leave.

PERSONNEL FILE CONTENT AND MANAGEMENT

Policy

Personnel files will be maintained for personnel and independent contractors in a complete and confidential manner. Business contracts providing more than one contractor will maintain their own employee files, but the contract will stipulate access to filing information for survey and certification purposes and qualifications validation. Health reports and I-9 forms will be maintained separately.

Purpose

To ensure that all personnel employed and contracted by the agency meet and maintain the appropriate qualifications for employment.

Procedure

A tracking system will monitor for currency of personnel file items that may expire. The personnel file will contain the following items to verify compliance with Washington state, accrediting body, and agency requirements for employment.

1. Employment application On hire
2. Copy of offer letter with wages and benefits On hire
3. Pre-hire screening and proof of pre-employment interview On hire
4. References three sought (two positive) On hire
5. Professional license, certification, or registration verification at renewal On hire
6. Job description (signed/dated by employee) On hire
7. Verification of skills/education/training, including diplomas On hire
8. Competency skills checklist On hire/Annually
9. Criminal background, OIG Exclusion list, and National Sex Offender list checks On hire/Q2years
10. I-9 Form (In separate I-9 File) On hire
11. Social security card If changed On hire/
12. Dated and signed withholding (W-4) forms On hire
13. Driver’s License At renewal On hire/
14. Proof of auto insurance (if applicable) At renewal On hire/
15. Signed and dated confidentiality statements On hire/Annually
16. CPR, On hire/Renewal
17. Proof of training for non-professionals On hire

18. Blood borne pathogens and infection control education On hire/Annually
19. Proof of orientation (new and reassigned personnel) Skills On hire/New
20. Performance evaluations First 90 days/ Annually
21. Compliments and/or corrective action notes As applicable
22. TB screening/PPD/TB education (In separate health file) As required. On hire/
23. Hepatitis B Vaccine Accept/Decline (In separate health file) On hire
24. Flu Vaccination (In separate health file) Annually
25. Certification of specialty areas of practice, if applicable as obtained On hire/
26. In-service, and continuing education proof On completion
27. Proof of policies and procedures review orientation During
28. Receipt of AGENCY NAME, Name badge and equipment On hire
29. Evidence of exit interviews (past personnel) During/ Following exit
30. All personnel documents will be retained either on paper or electronically and at a minimum shall containupdated:
    a. Current practice certification, credential, or licensure.
    b. documentation of references.
    c. Evidence of orientation to current agency policies and procedures.
    d. Verification of skills.
    e. Evidence of WSP criminal background check and disclosure statement.
    f. Training in current and revised agency policies and procedures.
    g. Current CPR training.
    h. Communicable disease testing immunizations and vaccination.
    i. Annual evaluation.

PERSONNEL, CONTRACTOR, AND VOLUNTEER RECORDS

Policy

The agency maintains records on all personnel and volunteers and has access to records on all contractors. Records are saved as per state rules on retention of records for employees.

Purpose

1. To ensure that personnel, contractor, and volunteer records are complete and retrievable.
2. To ensure that the agency meets state requirements for personnel records and retention there of

Procedure

All personnel, contractor and volunteer records shall at a minimum include:

1. Initial Application.
2. Resume.
3. Documentation of references.
4. Current practice certification, credential, or licensure, as applicable.
5. Current driver's license and.
6. Proof of current auto insurance
7. Contracts when using contactors.
8. Evidence of orientation to current agency policies and procedures.
9. Current practice certification, credential, licensure, or documentation that non-credentialed long-term care workers meet the exemption criteria in chapter 246-980 WAC, as applicable.
10. A current food worker card per chapter 246-215 WAC; or j. (b) Training equivalent to United States Food and Drug Administration; or k. (c) Home care aide certification for personnel, contractors, and volunteers involved in food preparation services on behalf of clients.
11. Verification of personnel, contractor, and volunteer skills or training specific to meeting the care needs of patients or clients.
12. Evidence of initial and subsequent and Washington state patrol criminal background inquiry according to RCW 43.43.830 through 43.43.842 and state approved disclosure statement.
13. Training on current and revised agency policies and procedures, including patient or client care issues.

14. Current CPR training for direct care personnel and contractors in home care and hospice agencies, and hospice care centers.
15. Communicable disease testing, immunization, and vaccination according to current local health authorities;

and

16. Documentation of evaluations of personnel and volunteers providing direct patient or client care and evaluations of services provided by contractors providing direct patient or client care as required in WAC 246-335-425 (15) and (16)
17. All personnel files will be audited annually for completeness with the personnel file audit tool and when a personnel file is closed.

REFERENCES

WAC 246-335-420 WAC 246-335-425 WAC 246-335-430

CONTRACTED SERVICES

Policy

Golden Years Home Health Supported Living LLC will maintain control of the services provided by contract. The contracts will be signed and dated by authorized principals of each party and reviewed annually. Contract personnel may be used to augment personnel for high volume of staff coverage and to support student experiences in the community, if applicable. Contracts are reviewed before expiration and when needed for revisions.

Purpose

1. To ensure contracted services are appropriate and effective.
2. To ensure contracted services meet home care standards of practice and state law.

Procedure

1. The agency may establish written contracts or agreements for personnel services under hourly or per visit arrangements, and products under a fee arrangement. The contract or agreement will include, but is not limited to the following:
   a. Specific services or products to be supplied at the agreed fee for each service or product are provided by qualified personnel in a safe and effective manner as ordered in the plan of services.
   b. Statement indicating that the contractor, not the agency, is responsible for withholding the necessary taxes.
   c. Proof of required liability insurance.
   d. Contractors are required to adhere to applicable agency policies and procedures and state regulations.
   e. Contractor personnel are required to adhere to the PHI requirements of HIPAA and the Privacy Act Notice, and they can be held accountable for violation fines and penalties.
   f. Assurance by the contractor of the education, training, qualifications, and identification of personnel designated to provide services or products.
   g. Method for the contractor to participate in performance improvement activities as appropriate.
   h. Procedures for scheduling visits and documentation and submission of required notes including periodic summaries that verify the provisions of services or products in accordance with the written contract.
   i. Required billing procedures, time frame for submission, and payments.

j. The agency retains exclusive authority to admit or discharge clients, establish the frequency and duration of services, and supplies or equipment needs for agency clients.

k. The agency retains administrative, fiscal, and professional oversight of the personnel and services provided under arrangement.

l. The agency retains responsibility for payment of contracted services by the contract terms.

m. Definition of the respective roles, if any, of the agency and the contracted individual or agency in the client admission process, assessment, plan of care development, review, and revision, client care conferences, and discharge planning.

n. Lists effective date of the contract, duration, renewal terms, and/or termination.

o. The rights and responsibilities of the agency or individual providing services and of the contracting agency in the coordination, supervision, and evaluation of the care or services provided.

p. The contractor's agreement not to provide service or to make any alteration to the scope and limitations of services set forth in the service plan without prior approval of the agency.

q. Clients are accepted for services by the agency only, which remains responsible for ensuring that any service provided adheres to the client's service plan.

r. The geographic area in which services will be provided.

s. The method of supervision of contracted personnel and the way services will be controlled, coordinated, and evaluated by the agency.

t. For staffing contracts, the client service plan is provided to the contracted personnel.

u. The written staffing contracts will define the contractor's and/or agency's responsibilities for compliance with state regulations, including:

    i. Compliance with client rights and confidentiality of PHI

    iii. HIPAA compliance

    iv. Job descriptions

    v. Criminal background check

    v. Verification of professional licensure in good standing vi. Legal status to work in the United States

Vii. Verification of work references

viii. Tuberculosis and Hepatitis B virus requirements

ix. Applicability of Universal Precautions

x. Orientation and ongoing training regarding client services policies and procedures

xi. CPR per state requirement or every two-year renewal

2. Contracts will be retained for five years following contract termination.

REFERENCES

WAC 246-335-420

ORIENTATION TO HOME CARE AGENCY

Policy

Golden Years Home Health Supported Living LLC provides a structured orientation for all new personnel, contractors, and Governing Body members.

Purpose

1. To assure agency newcomers understand their roles and responsibilities.
2. To assist newcomers in understanding the agency's organization, mission, goals, policies, and procedures.

Procedure

1. The Owner/Administrator/designee assures that newly hired personnel meet the qualifications for their job responsibilities before orientation is scheduled.
2. Upon hire and prior to beginning client service, each employee will participate in an In-person orientation to current agency policies and procedures and verification of skills or training prior to independently providing client care. Examples of skills verification include written testing, skills observation, and evidence of previous training and experience such as long-term care worker training as detailed in RCW 74.39A.074 and certified nursing assistant training as detailed in WAC 246-841-400;
3. Verification of skills or training will be done prior to independently providing client care.
4. The agency will provide and document individualized orientation for each position. Support personnel may not need to complete orientation to asterisked (*) items. For direct service job descriptions, this orientation will include:
   a. The agency's mission statement and ethical issues process
   b. Performance improvement activities/duties
   c. Organizational structure, scope of services, and lines of authority
   d. Review of specific job description responsibilities and related training
   e. Observation of required skill competencies
   f. Review of policies and procedures manuals including personnel policies
   g. Client rights, advance directives, and grievance policies and procedures
   h. Screening and intervention for abuse and neglect*
   i. Documentation requirements related to job responsibilities.
   j. Client admission, transfer, and discharge procedures*
   k. Agency compliance program

l. Confidentiality and PHI privacy agreement
m. Washington state guidelines including HIPAA regulations.
n. Conflict of interest policy*
o. OSHA and safe work practices
p. Emergency preparedness, safety, and disaster plans
q. Telephone etiquette, handling inquiries, and incidents/complaint policies and procedures.
r. Payroll process
s. Cultural diversity in home care agency services
t. Behavior and stress management/communication barriers
u. Maintaining professional boundaries
v. Discussing charges for services with clients/families*
w. Community resources*
x. Equal Employment Opportunities Employer (EEOE)/Affirmative Action
y. Drug-free and smoke-free workplace.
z. Intimidation, discrimination, and harassment in the workplace aa. Employee grievance procedures bb. Prevention of violence in the workplace cc. Clinical and personnel record access dd. Infection control and prevention, including universal precautions, personal hygiene, aseptic procedures, communicable diseases, and use, cleansing, disinfecting of equipment and supplies* ee. Equipment management, including safe and appropriate use of equipment; identifying, handling, and disposing of hazardous or infectious or waste ff. Observation of skills competency gg. Training in prevention, transmission, and treatment of blood borne pathogens* hh. Supervisory functions
ii. Documentation of services requirements*

4. Contracted personnel providing client services will receive an agency orientation through their employer and the home care agency covering all the above items except the employee specific policies and procedures.
5. Documented evidence that the employee or contractor was oriented and is knowledgeable about applicable policies, procedures, and position requirements shall be maintained in the personnel file.
6. Those in orientation will be provided with the opportunity to evaluate the process.
7. Each position's orientation will be individualized to emphasize the areas of responsibility and competency skills test duties as required.

REFERENCES

WAC 246-335-420 WAC 246-335-425

CONTINUING EDUCATION

Policy

Golden Years Home Health Supported Living LLC provides ongoing continuing education for all personnel who have direct contact with clients. Additionally, the agency encourages all personnel to maintain their skills through continuing education offered through other organizations as well. Continuing education is also available on a need basis for agency support personnel including mandated annual in-services. Direct service personnel will receive 12 hours per year of in-service. Non-direct service personnel will receive a minimum of 8 hours per year of in-services. The Supervisor /Director of Client Services completes a minimum of 10 hours yearly related to supervision and the delivery of care.

Purpose

1. To ensure competent and qualified personnel are serving agency clients.
2. To help personnel remain current with changes in care and regulatory updates.

Procedure

1. All personnel are required to attend orientation and annually, in-service training on universal precautions and blood borne pathogens, CPR, TB control measures and procedures, HIPAA and confidentiality, OSHA, and other Washington state, and industry annual review requirements.
2. All caregiving staff will receive on-going training and education pertinent to the current client's care needs.
3. A readily accessible written annual continuing education schedule with monthly offerings is prepared with personnel input.
4. All personnel will be oriented to policies and procedures that directly affect their work requirements at the time of orientation and whenever a policy change is made.
5. Performance improvement identified trends requiring policies and procedures changes as corrective action will be the basis for required in services.
6. In-service programs will be offered in written, lecture, discussion, return demonstration video, or audio format based on need.
7. A log of attendance and the materials presented will be kept in the in-service file. Proof of attendance is available to the personnel upon request.
8. All Home Care Staff will be required to complete at least 12 hours of in-service each year. Homemakers are required to attend four to eight hours of in-services annually. Non-direct client services personnel are required to have eight hours each year.

9. Personnel failing to meet their requirement for yearly in-services training will be subject to disciplinary action and suspended from service until requirements are met.
10. Direct service non-licensed personnel not having the required infection control education will be required to attend a minimum of a one-hour infection control education program designed for non-licensed personnel. This will be accomplished either at orientation or within 30 days of hire.
11. Staff failure to attend or plan to make-up required in-services are subject to corrective action.
12. Care staff failing to meet the 10 hours of yearly in-service training or 12 hours for CNAs will be subject to disciplinary action and this will be reflected in the yearly evaluation.

REFERENCES

WAC 246-335-420 WAC 246-335-445

PERFORMANCE EVALUATIONS

Policy

Golden Years Home Health Supported Living LLC provides each employee with an evaluation upon completion of orientation and annually thereafter. Performance evaluation is an ongoing process and includes evidence of compliance with policies and procedures, client satisfaction, and interpersonal and written communication skills.

Purpose

1. To ensure that all personnel and direct service contract personnel have timely and complete feedback regarding their performance.
2. To ensure during performance evaluations, there is the opportunity to discuss performance with management and to arrange for needed training, support for improvement, and development of goals to improve their skills, if needed.
3. To confirm that the agency is placing qualified, efficient, and effective personnel in client services and support positions to achieve the agency's mission and goals.

Procedure

1. The agency performs annual performance evaluations of all personnel and volunteers providing direct client care, including on-site observation of care and skills specific to the care needs of clients and;
2. Annual evaluations of services provided by contractors providing direct client care.
3. The employee or contractor performance evaluation will be based on the job description, and, if applicable, will include at least one documented in-home observation and competency skill check per year. Client satisfaction surveys may also be used.
4. Performance evaluations for direct service contractors are performed by the agency in collaboration with the contracting company at least annually, prior to the contract renegotiation to help the agency determine if there are problems or opportunities for improvement and to decide whether to renew, revise, or end the contract.
5. A qualified supervisor will review the performance evaluation with the employee. Based on the discussion, adjustments may be made and documented.
6. Measurable goals for the coming year will be developed with the employees, specifying any needed training or support identified.
7. The employee will be asked to sign the evaluation indicating that it has been discussed with him/her. Signature does not indicate employee agreement with the evaluation only that they have read it. All personnel will receive a copy.

8. The performance evaluation will be documented and become a permanent part of the employee's personnel file.
9. Any employee taking a leave of absence will have his/her evaluation deferred until he/she has been back on the job 30 days.
10. The annual in-home observation shall be done by a qualified agency supervisor or designated peer observer and will include documentation review and a competency skill check.
11. The Owner/Administrator will conduct evaluations of Services Supervisor, Office Manager, and in-office personnel with peer review as appropriate.

REFERENCES

WAC 246-335-425

SUPERVISION OF HOME CARE SERVICES

Policy

Golden Years Home Health Supported Living LLC Home Care staff report to the Supervisor, who has been employed by the agency to direct the care provided by agency staff. The supervisor of direct care services will designate in writing a similarly qualified alternate to act in the supervisor's absence.

Purpose

1. To ensure the Home Care Staff follows the service plan.
2. To assess client/representative relationships with the Home Care Staff

Procedure

1. The Agency ensures the supervisor of direct care services and the designated alternate complete a minimum of ten hours of training annually.
2. Training is documented and maintained in personnel files. Training may include a combination of topics related to supervisory duties and the delivery of home care services. Examples of appropriate training include, but are not limited to:
   a. Agency sponsored in-services.
   b. Community venues.
   c. Community classes.
   d. Conferences.
   e. Seminars.
   f. Continuing education related to the supervisor's professional credential, if applicable; and
   g. Supervisory responsibilities in the event of a natural dis-aster, man-made incident, or public health emergency.
3. The Supervisor or designer is available during all hours of client care.
4. The Supervisor is responsible for the Caregivers supervision and tracks the annual required 12 hours of in-services training, Aides not maintaining their certification and the required continuing education will be notified by the supervisor that they are deficient and will no longer be able to take shifts until the requirements are met.
5. The Supervisor of direct care services ensures:
   a. Supervision of all client care provided by personnel and volunteers by making home visits at least every

6 months and more often if the need is identified.

b. Evaluation of services provided by contractors by quarterly review of documentation and communication of any deficiencies to the contractor. If performance is deficient, the performance will be monitored weekly for at least a month. if the performance is not improved, the contractor will be terminated.
c. Coordination, development, and revision of written client care policies.
d. Services coordination when more than one agency is providing care to the client by weekly or monthly phone calls between the agencies, whichever is indicated.
e. Compliance with the plan of care.
f. All direct care personnel, contractors, and volunteers observe and recognize changes in the client's needs and report any changes to the director or designee; this is accomplished by quarterly chart reviews and incident reports of issuers clients changes in health status.
g. All direct care personnel, contractors, and volunteers initiate emergency procedures according to agency policy; monitored as above.
h. Each Caregiver reviews the plan of care or written instructions for the care of each client prior to providing home care aide services and whenever there is a change in the plan of care; this is monitored using the client orientation log for each client; and
i. Each home care aide assists with medications according to WAC 246-335-310 or 246-335-510, and agency policy.

6. The Supervisor documents the supervision conducts and documents client contact by phone or visit, every six months to evaluate compliance with the plan of care and to assess client satisfaction with the care.
7. The Supervisor or designer is available during all hours of client care.

REFERENCES

WAC 246-335-445 WAC 246-335-310 WAC 246-335- 425

SUPERVISION OF HOMEMAKERS

Policy

Homemakers (Environmental Support Staff) work under the Supervisor.

Purpose

1. To assure the Homemaker follows the Homemaker service plan.
2. To assess client and family relationships with the Homemaker.

Procedure

1. The Scheduler/Client Care Coordinator assigns the Homemaker to a specific client.
2. Homemaker duties are outlined in the job description and include but are not limited to:
   a. Cleaning, laundry, homemaking functions, and food preparation.
   b. Assistance in ambulation or exercises and medication reminders.
3. The Homemaker service plan is:
   a. Completed by a qualified Supervisor or designer.
   b. Reviewed with the Homemaker before the initial visit to confirm understanding of the service plan. Supervisor or designer will provide training or demonstration of service plan requirements as needed.
   c. Placed in the client's home for reference by the Homemaker.
   d. Reviewed at least every six months after the start of services for the duration of service and updated whenever needed. Any changes to the service plan must be made on the copy in the client's home and on the original in the agency's client record.
4. Supervision of Homemakers:
   a. Tracks the eight hours of annual training required.
   b. Responsible for confirming the orientation, training, competency skills testing, and annual evaluation.
   c. Perform supervisory visits at least every six months. The caregiver MUST be present for the home visit. All supervisory encounters are documented according to agency policy in the client's record.
5. Supervision will consist of the following:
   a. Monthly telephone contact with Homemaker clients for the first six months of service to confirm Homemaker compliance with the service plan and client satisfaction.
   b. Update of the home care aide service plan when indicated.

c. Demonstration or training in any new tasks required on the Homemaker service plan or for tasks the Homemaker requires additional remedial training.

d. Orientation and annual on-site visit evaluations with input from the personnel. Homemakers will do a self-evaluation and set their goals, and the Supervisor will complete the form and evaluation discussion.

REFERENCES

WAC 246-335-445 WAC 246-335-425

CORRECTIVE ACTION

Policy

It is the policy of Golden Years Home Health Supported Living LLC that all professional and support personnel could identify and correct non-compliant job-related action in a consistently fair process that promotes employee retention and positive home services outcomes. Most workplace performance issues are handled by informal discussion and counseling between the supervisor and employee. This policy applies to situations that require more formal action.

Procedures

1. Corrective action is a progressive process starting with the simplest interventions and progressing to the most severe only as necessary.
2. To be done in five days or less, the supervisory investigation of an employee aims to disprove or confirm elements that failed to comply with policy or procedure.
3. The employee has the right to have a non-attorney witness present at any corrective action meeting, to ask questions, or explain their actions orally or in writing.
4. Corrective action steps may be skipped if the severity of the incident warrants.
5. Steps in the corrective action process include:
    e. Written Warning and Corrective Action Plan: This step may be repeated if progress is being made, but resolution is not complete.
    f. Final written warning and suspension with pay that includes time frame and conditions for return to work.
    g. Demotion with retraining and corrective action plan.
    h. Termination with time frame and information about employee rights.
6. Suspension and termination actions require Owner/Administrator’s approval.
7. Documentation of the employee actions must be in writing, complete, specific, and include employee response to allegation, substance of supervisory/ employee discussion of implications of the actions, and final resolution with a copy of the final resolution provided to the employee.
8. Corrective action documentation is maintained in confidential personnel files.
9. With administrative approval, circumstances may warrant removal from the workplace during corrective action investigation including but not limited to:
    a. Safety or security of persons or property is in question.
    b. The alleged misconduct is of a serious and/or criminal nature.

c. There is the likelihood that the employees' presence will disrupt the workplace.

10. If removal from the workplace is authorized, the employee's time off is with pay until investigation determines suspension without pay or termination is justified.
11. Prior corrective actions less than 12 months old may be considered at the time of a new incident. This limit does not apply if the incident involves criminal or egregious offenses.

EMPLOYEE GRIEVANCES AND COMPLAINTS

Policy

Golden Years Home Health Supported Living LLC responds appropriately to all complaints by personnel and contracted personnel and maintains an accurate, dated record of the episode.

Purpose

1. To address individual employee complaints and grievances fairly and appropriately.
2. To provide a means for early identification and, if possible, satisfactory resolution of problems.

Procedure

1. Staff will be informed verbally and in writing of agency's complaints and grievance policies and procedures at the time of orientation to the agency.
2. Anyone may lodge a grievance or complaint in person, by phone, or in writing. The employee is asked to discuss their concern with a supervisor before submission of a formal complaint and then submit the details in

writing.

3. The Owner/Administrator or designer investigates complaints or grievances made by an employee within 48 hours and will work diligently in a timely manner to complete the investigation. A proposed date for addressing the issue is set with the employee.
4. All complainants will be notified in writing within 30 days of the results of the agency's investigation.
5. If the issue is not resolved in a timely manner or to the employee's satisfaction, the employee will have the right to take the grievance to the Owner/Governing Body.
6. Staff complaints will be documented and maintained in an administrative file and will be included in the agency's overall Performance Improvement Program.

REFERENCES

WAC 246-335-455

DRUG TESTING

Policy

Golden Years Home Health Supported Living LLC desires to provide a safe, drug and alcohol-free environment for personnel and clients. To ensure the accuracy and fairness of our testing program, all testing will be conducted according to the Substance Abuse and Mental Health Services Administration (SAMHSA) guidelines where applicable and will include a screening test, a confirmation test, the opportunity for a split sample, review

by a qualified medical reviewer, a documented chain of custody, and the opportunity for those who test positive to provide a legitimate medical explanation, such as a physician's prescription for the positive result. This policy is intended to comply with the Drug-free Workplace Act of 1988 and all state laws governing drug testing and is designed to safeguard employee privacy rights.

Purpose

To ensure that personnel, both in the office and those assigned to service of clients, are functioning unimpaired and at an optimal level for performance of job duties.

Procedure

1. Under this policy, current and prospective personnel, as a condition of employment, will be required to participate in pre-employment, random, and reasonable suspicion testing upon request of management. No prospective employee will be asked to submit to testing unless an offer of employment has been made. The offer of employment is conditioned on the applicant testing negative for drugs.
2. All alcohol and drug-testing information will be kept in separate confidential records.
3. The agency will cover the cost of the required employee drug testing.
4. Before being asked to submit to a drug test, the employee will receive written notice of the request or requirements.
5. A laboratory licensed by the state will conduct any drug testing required or requested by the agency. The employee, prior to being scheduled for testing, will be given the name and location of the laboratory that will analyze the employee’s test sample.
6. Testing for the presence of alcohol will be conducted by breath or blood.
7. Drug substances that will be included in the blood or urine testing are; Cocaine Amphetamines, Cannabinoids (THC), Opiates, and Phencyclidine (PCP).

8. If the employee is asked to submit to a drug test, the agency will notify the employee of the results after it receives them from the laboratory. To preserve confidentiality, the employee will be notified whether the test was negative or confirmed positive and, if confirmed positive, what the next option will be.
9. The results of testing for an employee with a positive test will not be released without the employee's written consent.
10. Test results may be used in arbitration, administrative hearings, and court cases arising because of the employee's drug testing. Also, results will be sent to state agencies as required by the state law. If the employee is to be referred to a treatment facility for evaluation, the employee's test results will also be made available to the employee's counselor. The results of drug testing in the workplace will not be used against the employee in any criminal prosecution.
11. Besides being outlined in this manual, the agency's drug policy is posted in the office where personnel may review it. In addition, copies are available from the agency during regular business hours.

JOB DESCRIPTION: OWNER/ADMINISTRATOR

The Owner/Administrator is an employee of Golden Years Home Health Supported Living LLC possess education and experience required by the agency's policies and is designated in writing by the Owner and is obligated to organize and direct the services and functions of the agency's programs in accordance with the policies determined by the Owner and state rules.

SUPERVISES: Supervisor

QUALIFICATIONS

- Educational preparation preferred is a master's degree in a healthcare administration, human services, or related business field and at least two years' administrative experience in a health-related field; or a bachelor's degree in a health-related field with a minimum of five years of healthcare administrative experience.
- Possess evidence of training in health care related administration and programs
- Possesses experience and knowledge of applicable local, state laws
- Computer literate
- Familiar with quality, home care agency, and financial reports
- Provide written proof of health status and ability to lift, bend, and squat the level and amount determined by each job assigned.
- Possess effective communication skills and ability to lead a team of personnel with a variety of backgrounds and educational experiences.
- Possess good time management, problem-solving, and work ethic skills.
- Possess a current driver's license and a dependable insured automobile.

OWNER/ADMINISTRATOR RESPONSIBILITIES

1. Demonstrates support for the agency's mission statement to promote quality, efficiency, comprehensive, compassionate, and effective client service.
2. Responsible to:
    a. Oversee the day-to-day operation and fiscal affairs of the agency.
    b. Implement the provisions of this section.
    c. Designate in writing an alternate to act in the administrator's absence.
    d. Provide management and supervision of services throughout all approved service areas according to state rules.
    e. Arrange for necessary services.

f. Keep contracts current and consistent with WAC 246-335-425(4);
g. Serve as a liaison between the agency, personnel, contractors, and volunteers;
h. Ensure personnel, contractors and volunteers are currently credentialed by the state of Washington, when appropriate, according to applicable practice acts and consistent with WAC 246-335-425(5).
i. Ensure personnel, contractors and volunteers comply with the agency 's policies and procedures.
j. Implement a quality improvement process consistent with WAC 246-335-455.
k. Manage recordkeeping according to WAC 246-335-450.
l. Ensure supplies and equipment necessary to client care are available, maintained, and in working order.
m. Ensure the accuracy of public information materials; and
n. Ensure current written policies and procedures are accessible to personnel, contractors, and volunteers during hours of operation.

3. The Administrator is responsible for continuing to update its plan of operation to reflect current practice, services provided by the agency, and state and local laws.
4. Plans, develops, implements, administers, evaluates, organizes, and directs the operational budget and activities to ensure adequate service availability and viability.
5. Directs and organizes the agency’s personnel performance evaluation process.
6. Ensures agency compliance with legal, regulatory, and accreditation regulations.
7. Provides personnel with access to current industry information and practices.
8. Establishes and maintains effective lines of communications, including use of integration of new technology.
9. Recruits, employs, and retains qualified personnel to maintain adequate staffing.
10. Provides personnel development including orientation, in-services, continuing education, competency testing, and performance improvement opportunities.
11. Protects client and personnel rights including but not limited to PHI confidentiality.
12. Resolves conflicts and manages complaints.
13. Promote coordination with other organizations working with agency clients.
14. Ensures appropriate personnel supervision is available 24hour/day, 365 days/year.
15. Monitors service utilization to ensure delivery of comprehensive service.

16. Ensures that home care services whether provided directly or under contract are authorized and provided as ordered.
17. Promotes personnel and client safety and provides a safe workplace with adequate support personnel, se, supplies, home care library materials, well maintained equipment, and other resources, as needed.
18. Directs human resource activities and completes or delegates performance evaluations in accordance with agency policies and procedures.
19. Promotes agency excellence and high ethical standards by providing current client information and standards of practice through orientation, in-service, continuing education, and overseeing the ethics committee activities.
20. Ensures client coordination of service.
21. Assurance of timely accurate billing and collection and accounting practices.
22. Delegates specific responsibilities to other personnel, when appropriate.
23. Presents a professional appearance and promotes a positive work environment.

EMPLOYEE ACKNOWLEDGEMENT AND ACCEPTANCE

I acknowledge I have received and read my job description and understand and accept the responsibilities, qualifications, physical demands, and work environment this position requires. I understand that this is not an exclusive list of the job functions and that I am expected to complete all duties as assigned; I understand management without notice may alter the job functions. I understand that this job description in no way constitutes an employment agreement and that I am an at-will employee.

Employee signature	Printed name	Date

REFERENCES

WAC 246-335-410 WAC 246-335-415 WAC 246-335-425 WAC 246-335-450 WAC 246-335-455

JOB DESCRIPTION: SUPERVISOR

The Supervisor is a salaried employee and is directly responsible to the Owner/Administrator. She/he is responsible for the overall client service operations and assigned personnel supervision. If this position is combined with the Supervisor, employee must possess an RN license.

REPORTS TO: The Owner/Administrator or designated individual

SUPERVISES: Scheduler/Client Care Coordinator and field personnel as assigned on organization chart.

QUALIFICATIONS

1. Must have on hire and maintain CPR certification.
2. Must be computer literate, and familiar with quality, client, and financial reports.
3. Must be able to provide written proof of health status and ability to lift, bend, squat the level and amount determined by each job assigned
4. Must possess knowledge of home care regulations and procedures.
5. Must possess effective communication skills and ability to lead a team of personnel with a variety of backgrounds and educational experiences.
6. Must possess client assessment, good time management, problem-solving, and work ethic skills.
7. Must possess a current driver’s license and a dependable insured automobile.
8. Must present a professional appearance and promote a positive work environment.

RESPONSIBILITIES

1. Demonstrates support for the agency mission statement to promote quality, efficient, comprehensive, and effective client services.
2. May serve as alternate Owner/Administrator in the absence of the Owner/Administrator.
3. Supervises and serves as a role model to all assigned personnel.
4. Available on premises or via telecommunications to supervise and serves as a role model to all assigned personnel.
5. Participates in and supports the agency’s performance improvement activities.
6. Participate in employee orientation, competency testing, and in-services.
7. Use effective communication skills to follow up on complaints by clients or personnel.
8. Maintains confidentiality of client, employee, and agency operations.
9. Keeps all records up to date, organized, and ready for inspection at any time.

10. Promotes compliance with all agency standards, policies, and procedures.
11. Promotes client, personnel, and office safety precautions.
12. Presents a professional appearance and promotes a positive work environment.
13. Oversee Scheduler/Client Care Coordinator client service assignments.
14. Participates in the quarterly record review and works with personnel to initiate needed changes to maintain records in accordance with state and accreditation standards.
15. Reviews all admissions, transfers, discharges, and services to ensure that they meet agency policies and procedures and standards of practice.
16. Negotiates with insurance companies regarding client coverage as needed.
17. Facilitate referrals to other agencies.
18. Supervises Home Care Aids and Homemakers in a frequency as required including peer review as applicable. All aspects of training (if provided), competency testing, direct observation, documentation review, and monitoring of required annual in-service training are included in the supervisory duties.
19. Represents the agency at community functions.
20. Participates in the development of the annual program evaluation and strategic plan.
21. Participates in interviews and selection of new personnel as delegated.
22. Evaluates assigned personnel performance and provides corrective action if needed.
23. Performs other duties as requested.

EMPLOYEE ACKNOWLEDGEMENT AND ACCEPTANCE

I acknowledge I have received and read my job description and understand and accept the responsibilities, qualifications, physical demands, and work environment this position requires. I understand that this is not an exclusive list of the job functions and that I am expected to complete all duties as assigned; I understand management without notice may alter the job functions. I understand that this job description in no way constitutes an employment agreement and that I am an at-will employee.

Employee signature	Printed name	Date

REFERENCES

WAC 246-335-410 WAC 246-335-415 WAC 246-335-425 WAC 246-335-450 WAC 246-335-455

JOB DESCRIPTION: HOME CARE AIDES

The Home Care Aides of Golden Years Home Health Supported Living LLC provide activities of daily living (ADL) and instrumental activities of daily living (IADL) service to clients who pay privately or are covered by insurance that does not require a certified home care staff to provide the services.

REPORTS TO: Supervisor or designee

SUPERVISES: No one

QUALIFICATIONS

1. Must be at least 18 years of age.
2. Prefer a high school degree or GED.
3. Must have at least one year of employment experience as a Basic Personal Care Assistant
4. Proof of current CPR training
5. Proof of successful completion a minimum of 40 hours of training and/or demonstrated competency in and knowledge of:
   a. Responsibilities and limitations of communication techniques
   b. Client rights confidentiality of PHI
   c. Respect for person and property.
   d. Observation for client changes to report
   e. Standard infection control
   f. Home safety precautions
   g. Basic body functions and variances
   h. Procedures for clean, safe environment
   i. Emergency response
   j. Physical, emotional, and mental client care
   k. Bed, sponge, tub, or shower bath.
   l. Skin care, oral hygiene, toileting
   m. Basic nutrition, meal preparation
   n. Encouraging independent care
   o. Documentation techniques
   p. Supervisory support and duties
   q. Advance directives implications

6. Ability to communicate with clients and to remain calm under stress.
7. Ability to read and write in English.
8. A state Certified Home Care Aide or state Certified Nursing Assistant or pass the agency requirements for a Home Care Aide may perform these duties.
9. Ability to follow oral and written directions and complete work as assigned.
10. Ability to maintain confidentiality about client services and personal information.
11. Maintain current state driver's license and dependable insured automobile.
12. Provide written proof of health status and ability to lift, bend, squat the level and amount determined by each job assigned.
13. Accurately document Home Care Aide services provided.
14. Present a neat appearance and positive work attitude.

RESPONSIBILITIES

1. Demonstrates support for the agency mission statement to promote quality, efficient, comprehensive, and effective client services.
2. Participates in and supports the agency's performance improvement activities.
3. Provides Home Care Aide services to clients as identified by the agency in the plan of services per agency policies, procedures, and Home Care Aide skill level.
4. Prepares and maintains accurate and timely client notes, documenting interventions, client pain score, outcomes, and client response per agency policy.
5. Assists with personal care activities (ADLs) and IADLs allowing client independence to tolerance as indicated in the service plan including:
    a. Personal care and hygiene
    b. Universal precautions
    c. Locomotion/ambulation/fall prevention oversight
    d. Nutritional support, meal preparation, feeding assistance.
    e. Bed mobility and positioning, range of motion exercises.
    f. Toileting.
    g. Continence care
    h. Dressing and undressing
    i. Bathing
    j. Foot care (nails filed only)

k. Skin care

l. Medication reminder assistance

m. Light housekeeping includes dusting, vacuuming, linen changes.

n. Laundry of client's clothing and bedding.

o. Social and emotional support

6. Reports on client condition changes to supervision.

7. Appropriately seeks supervisory assistance as necessary to effectively perform duties.

8. Maintains confidentiality of client, employee, and agency operations.

9. Meets productivity requirements established for Basic Personal Care Assistant.

10. Participates in 8 hours of in-services and training as assigned.

11. Responsible for proper use and care of equipment.

12. Practices safety, universal precautions, and infection control measures.

13. Maintains a neat appearance and positive attitude with clients, family, and personnel.

14. Performs other duties as qualified and requested.

EMPLOYEE ACKNOWLEDGEMENT AND ACCEPTANCE

I acknowledge I have received and read my job description and understand and accept the responsibilities, qualifications, physical demands, and work environment this position requires. I understand that this is not an exclusive list of the job functions and that I am expected to complete all duties as assigned; I understand management without notice may alter the job functions. I understand that this job description in no way constitutes an employment agreement and that I am an at-will employee.

Employee signature	Printed name	Date

REFERENCES

WAC 246-335-425

JOB DESCRIPTION: HOMEMAKER

The Golden Years Home Health Supported Living LLC Homemaker is responsible for the day-to-day provision of homemaking services to clients. Certain state and state programs support Homemaker services to assist seniors and the disabled to remain independent at home for as long as possible. These services may also be paid for privately. The Homemaker works under the direction of a supervisor to ensure that the agency is achieving its identified outcomes and goals for our clients.

REPORTS TO: Supervisor and Scheduler/Client Care Coordinator for schedule

SUPERVISES: No one

QUALIFICATIONS

1. Must be at least 18 years of age.
2. Prefer high school degree or GED.
3. Must have at least one year of employment experience as a Homemaker or life experience as a Homemaker/Caregiver, which will be competency assessed on hire.
4. Ability to communicate with clients and to remain calm under stress.
5. Ability to read and write in English.
6. A Certified Home Care Aide or Certified Nursing Assistant registered in the state or has pass the agency requirements for a personal care worker may perform the Homemaker duties.
7. Ability to follow oral and written directions and complete work as assigned
8. Ability to maintain confidentiality about client services and personal information.
9. Maintain current state driver's license and have dependable insured automobile.
10. Provide written proof of health status and ability to lift, bend, squat the level and amount determined by each job assigned.
11. Accurately document Homemaker services provided.
12. Present a neat appearance and positive work attitude.

Exception Procedure:

If a good candidate does not have confirmable training and experience as a Homemaker, the agency will offer conditional employment based on successful completion of the agency provided Homemaker training and competency testing which minimally includes:

1. Job descriptions review with overall responsibilities and limitations.

2. Client rights and responsibilities include respect for clients and their property, ethics, advance directives.
3. Communication techniques
4. Service plans use and documentation
5. Hand hygiene, universal precautions, and infection control
6. Safety in the home
7. Cleaning procedures
8. Homemaker duties
9. Shopping, food preparation and storage
10. Laundry
11. Companion activities
12. Supervisory availability and direction
13. Emergency plan implementation

RESPONSIBILITIES

1. Demonstrates support for the agency Mission Statement to promote quality, efficient, comprehensive, and effective client services.
2. Participates in and supports the agency's performance improvement activities.
3. Performs light housekeeping tasks including:
    a. Planning and preparing nutritious meals and assisting in feeding clients as needed.
    b. Light housekeeping of client's environment, including dusting, vacuuming, mopping, cleaning kitchen and bathroom, linen changes, and making the bed.
    c. Laundry of client's clothing and bedding.
    d. Shopping/Transportation with signed release.
4. Provides and documents services following the Homemaker service plan.
5. Uses household equipment and supplies safely.
6. Social interaction /behavior modification activities as instructed on the service plan.
7. Maintenance of a clean, safe client environment.
8. Practices safety precautions and infection control measures.
9. Implement emergency or disaster measures, if necessary.
10. Reports any incidents or client concerns to the supervisor.

11. Appropriately seeks supervisory assistance as necessary to effectively perform duties.
12. Maintains confidentiality of clients, personnel, and agency operations.
13. Meets productivity requirements established for Homemaker caseload.
14. Participation is required for four hours of in-services and training as assigned.

EMPLOYEE ACKNOWLEDGEMENT AND ACCEPTANCE

I acknowledge I have received and read my job description and understand and accept the responsibilities, qualifications, physical demands, and work environment this position requires. I understand that this is not an exclusive list of the job functions and that I am expected to complete all duties as assigned; I understand management without notice may alter the job functions. I understand that this job description in no way constitutes an employment agreement and that I am an at-will employee.

Employee signature	Printed name	Date

REFERENCES

WAC 246-335-425

JOB DESCRIPTION: SCHEDULER/CLIENT CARE COORDINATOR

The Golden Years Home Health Supported Living LLC Scheduler/Client Care Coordinator is responsible for the day-to-day scheduling and coordinating of Home

Care Aide and Homemaker assignments. A critical component of this role is to demonstrate the agency's culture of compassion, accountability, and results by displaying the highest standards of integrity, superb follow-through, consistency, first-rate customer service, and exceptional client satisfaction.

REPORTS TO: Supervisor

SUPERVISES: No one

QUALIFICATIONS

1. Must have high school diploma or GED.
2. Must be at least 18 years old.
3. Prefer at least one-year in-home service scheduling.
4. Must have effective oral and written English communication skills to work with clients/family and ability to listen without imposing personal values or beliefs.
5. Must be able to follow instructions to complete work as assigned.
6. Ability to maintain confidentiality about clients receiving agency services.
7. Maintains current state driver's license and a dependable insured automobile, if needed
8. Demonstrates computer literacy and able to use scheduling software.
9. Provides written proof of health status and ability to lift, bend, squat the level and amount determined by each job assigned.
10. Demonstrated self-motivation, self-direction, organizational skills, flexibility, and the ability to coordinate multiple tasks simultaneously.
11. Presents a neat appearance and positive work attitude.
12. Experience working in a high stress environment with multiple demands.
13. Coordinate multiple tasks simultaneously.

RESPONSIBILITIES

1. Demonstrates support for the agency's mission statement to promote quality, efficient, comprehensive, and effective client services.
2. Participates in and supports the agency's performance improvement activities.
3. Comply with all agency policies and procedures.

4. Maintains confidentiality of client, employee, and agency operations
5. Promote clients, personnel, and office safety.
6. Schedules service visits per direction of supervisor or designee.
7. Assists with the referral/intake process for clients needing agency services.
8. Verifies third-party insurance coverage (if required)
9. Ensure all visits are scheduled in a timely manner and assigned as needed.
10. Presents a professional appearance and promotes a positive work environment.
11. Serves as receptionist/operator as assigned.
12. Assists in obtaining client satisfaction surveys.
13. Performs other duties as requested.

EMPLOYEE ACKNOWLEDGEMENT AND ACCEPTANCE

I acknowledge I have received and read my job description and understand and accept the responsibilities, qualifications, physical demands, and work environment this position requires. I understand that this is not an exclusive list of the job functions and that I am expected to complete all duties as assigned; I understand management without notice may alter the job functions. I understand that this job description in no way constitutes an employment agreement and that I am an at-will employee.

Employee signature	Printed name	Date

REFERENCES

WAC 246-335-425

CLIENT SERVICES POLICIES AND PROCEDURES

SCOPE AND AVAILABILITY OF AGENCY SERVICES

Policy

Committed to its mission statement, Golden Years Home Health Supported Living LLC provides services, either directly or through contract, to address the physical, social, and functional ADL and IADL needs of clients to assist in maximizing independence within the limits of their circumstances. The agency will ensure that Home Care Aide and Homemaker services are available to its clients, as needed. These services will adhere to the standards of home care practice for the respective disciplines. Home care services are defined as non-medical services that are provided to ill, disabled, or vulnerable individuals that enable them to remain in their residences. Home care services include but are not limited to: Personal care such as assistance with dressing, feeding, and personal hygiene to facilitate self-care; homemaker assistance with household tasks, such as housekeeping, shopping, meal planning and preparation, and transportation; respite care assistance and support provided to the family; or other nonmedical services or delegated tasks of nursing under RCW 18.79.260 (3)(e).

Purpose

To ensure that appropriate and effective home care services are available to address the client's identified needs 24 hours per day, 365 days per year within the service area of the agency.

Procedure

1. The Owner/Administrator is responsible for the hiring or contracting of qualified personnel and providing supplies and equipment necessary to fulfill the mission statement, state statutes, and accrediting body standards for a home care agency.
2. All services may include after-hour services (per service plan and/or client needs) and 24-hour supervisory access.
3. Agency will provide qualified personnel to perform agency services.
4. The client/representative will be involved with the development of the service plan, including what services and at what frequency will best serve

the client's needs.

5. The agency will provide Home Care Aide and Homemaker services.

6. Evaluation of the effectiveness of services provided based on goal achievements and client satisfaction, and record reviews part of the performance improvement initiatives.
7. The agency's hours of operation are as follows:
    1. The agency established business hours are Monday through Friday from 8 AM a.m. to 4:00 PM p.m.
    2. The office will be closed on [OBSERVED HOLIDAYS].
    3. New Year's Day: Celebrated on January 1.,
    4. Martin Luther King, Jr. Birthday: Observed on the third Monday in January.

Presidents' Day: Celebrated on the third Monday in February.

Memorial Day: Falls on the last Monday in May.

Juneteenth: Recognized on June 19.

Independence Day: Happens on July 4.

Labor Day: Takes place on the first Monday in September.

Veterans Day / Armistice Day: Honored on November 11.

Thanksgiving Day: Celebrated on the fourth Thursday in November.

Native American Heritage Day: Follows Thanksgiving Day.

Christmas Day: Celebrated on December 25.

e. Agency services are accessible 24 hours per day, 7 days per week, including weekends and holidays.
f. On-call services are available after regular business hours including weekends and holidays.

REFERENCES

WAC 246-335-415 RCW 18.79.260

REFERRAL AND INTAKE

Policy

All referrals made to Golden Years Home Health Supported Living LLC will be reviewed for appropriateness prior to accepting for services.

Purpose

1. To ensure that referrals are accepted by Golden Years Home Health Supported Living LLC are appropriate. are
2. To ensure that referrals taken by Golden Years Home Health Supported Living LLC are adequately serviced in the time needed by the client.

Procedure

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1. All referrals made to Golden Years Home Health Supported Living LLC will be taken by an assigned referral intake person. In the absence of the referral intake person, referrals may be taken by alternate personnel as designated by the Owner/Administrator.
2. Home care services must be started within seven calendar days of receiving and accepting a physician or practitioner referral for services. Longer time frames are permitted when one or more of the following is documented:
   a. A longer time frame for the start of services is requested by physicians or practitioners.
   b. Longer time frame for the start of services is requested by the client, designated family member, legal representative, or referral source; or
   c. Start of services was delayed due to agencies having challenges contacting clients, designated family members, or legal representatives.
   d. Home care agencies under contract with DSHS or the AAA may follow different time frames for notice of discharge as established in the terms of the contract.
3. All referrals shall include the following information:

a. Client’s name
b. Address and phone number.
c. Social Security number
d. Diagnosis
e. Primary physician
f. Services needed.

g. Relevant psychosocial, physical or safety issues
h. Coordination of services with other providers
i. Days and hours of services requested.
j. Source of payment
k. Name of person making the referral

4. The referral is given to the Supervisor or designee, who will establish an initial on-site visit to determine if the care required can be safely provided by the agency.
5. All referrals will be responded to within 24 hours and either opened to care or referred to a more appropriate agency within 48 hours.

REFERENCES

WAC 246-335-415 WAC 246-335-420

SCHEDULING AND VISITS

Policy

Golden Years Home Health Supported Living LLC ensures that only scheduled times or visits authorized or requested by the client or their representative are provided by the agency. If assigned visits or scheduled shifts cannot be filled by the regularly assigned staff, a back-up care response will be to assign a caregiver from the staffing pool to meet the client's needs. In the case of an emergency and no staff can attend the client, the back-up plan will be to have on file an identified family member or neighbor will be notified and requested to attend the client. If the visit can be made at a different time, every attempt will be made to assign the regular staff to the new time.

Back-up care which requires assistance with client ADLs must be provided by staff with minimum credentialing or workers who meet the exemption criteria in chapter 246-980 WAC. Non-credential staff may provide back-up care only when assisting a client with IADLs or in emergency situations;

Purpose

To define the procedure for establishing scheduled hours and days or visits requested by the client and/or their representative and changes to the same.

Procedure

1. The requested hours and days of service and type of services to be provided are established in concert with the client and/or their representative at the time of admission.
2. The Plan of Care that includes the days, hours or visits is developed at the time of the assessment and is signed by both the agency representative and the client or their representative.
3. Only hours, days or visits requested will be scheduled by the agency Client Care Coordinator.
4. Requested changes to the schedule will be documented by the agency staff either in the dialogue section of the scheduling program or on a narrative note in the client's file.
5. Agency staff will ensure that only persons authorized to make those changes are making the request prior to instituting the change.
6. The change in the Plan of Care will have the change, the date of the change and the initials of the individual making the changes.

REFERENCES

WAC 246-335-420

ADMISSION, ASSESSMENT, PLAN OF CARE

Policy

Golden Years Home Health Supported Living LLC is committed to providing timely, competent, quality, and supportive service by utilizing its personnel to meet the identified needs of its clients. The agency admits a client to service on re request and verification of client privately pay agreement or authorized reimbursement by the insurance payer.

1. The Agency develops and implements a written home care plan of care for each client with input and written approval by the client, designated family member, or legal representative.
2. Ensure each care plan is developed by appropriately trained or credentialed agency personnel, lists services requested or recommended to meet client needs, and is based on an on-site visit and according to agency policies and procedures.
3. The agency ensures the home care plan of care includes:
   a. The client's functional limitations.
   b. Nutritional needs and food allergies for meal preparation.
   c. Home medical equipment and supplies relevant to the care plan.
   d. Indication that the client has a signed advanced directive or POLST, if applicable.
   e. Nurse delegation tasks, if applicable; and
   f. Specific nonmedical services to be provided and their frequency.
4. Develop and implement a system to:

a. Ensure the plan of care is reviewed on-site, updated, approved and signed by appropriate agency personnel and the client, designated family member, or legal representative every twelve months and whenever significant changes to client care needs are identified; and
b. Inform the supervisor of direct care services regarding changes in the client's condition that indicate a need to update the care plan.

5. Home care agencies providing a one-time visit for a client may provide the following written documentation in lieu of the home care plan of care requirements in subsection (3) of this section:

a. Client name, age, current address, and phone number.
b. Confirmation that the client was provided with a written bill of rights under WAC 246-335-435.

c. Client consent for services to be provided; and
d. Documentation of services provided.

6. Home care agencies that have a contract with the AAA to provide home care services to Medicaid eligible clients may use the DSHS CARE assessment details and service summary, or successor assessment tool, as the plan of care if it covers all items required by rule.
7. Clients will be considered for admission without regard to race, sexual orientation, color, national origin, handicap, age, political affiliation, or veteran status.
8. Requirements for agency admission include:

a. The client resides in the agency's service area.
b. The client completes the informed consent form after provision of the home care agency materials and discussion of the services available with the Supervisor.
c. There is a person willing to accept the caregiver responsibilities for the client. If there is no willing/available primary caregiver, a mutually agreed upon alternative plan must be approved by the agency and client/ representative.

9. AGENCY understands that once the client is admitted as evidenced by a signed admission form and plan of care, that the agency cannot end the care relationship without referring to an appropriate alternative agency or caregiver or by not following applicable discharge requirements in WAC 246-335-420, 246-335-520 and 246-335-620. Failure to comply with these rules will be judged as Client Abandonment.
10. To minimize the possibility of client abandonment, clients must be given at least forty-eight hours' written or verbal notice prior to discharge that will be documented in the client record.
11. Forty-eight hours-notice is not required if home care agency worker safety, significant client noncompliance, or client's failure to pay for services rendered are the reason(s) for the discharge.

Purpose

1. To ensure that referrals accepted by the agency are appropriate.
2. To ensure that referrals accepted by the agency can be adequately serviced in the time frame and to the extent needed by the client.

Procedure

Referral/Intake Phase

1. All referrals to the agency will be taken by the Scheduler/Client Care Coordinator or qualified alternate and responded to within 24 hours. If the referral exceeds the agency's ability to serve, the referral will be referred to other providers who may be able to serve them.
2. The agency will accept Medicaid, private insurance, and private pay. A sliding scale and free service may be considered on an individual basis.
3. Referrals deemed to be appropriate for the agency to be opened within 7 days unless a longer time frame is needed. Longer time frames are permitted when one or more of the following is documented:
   a. A longer time frame for the start of services is requested by physicians or practitioners.

b. Longer time frame for the start of services is requested by the client, designated family member, legal representative, or referral source; or

c. Start of services was delayed due to agency having challenges contacting clients, designated family member, or legal representative.

4. The Agency contract with DSHS or the AAA follows the time frames for the start of services as established in the terms of the contract.

5. Referrals may be received from clients, family, significant others, state, or community services, or acute or extended care facilities.

6. All referrals for Home Care Aides services will include an assessment by a qualified agency representative and includes the following information.

a. Referral source

b. Client's name, address, and phone number

c. Diagnoses

d. Primary physician

e. POLST [if applicable]

f. Advanced Directives

g. Days and hours of services requested.

h. Services needed, including equipment and supplies.

i. Specific functional limitations related to ADLs and IADLs

j. Relevant psychosocial, physical or safety issues

k. Allergies, nutritional status, medication regime

l. Coordination of services to address client preferences.

m. Source of payment

7. Referrals for Homemaker services will include an assessment by a qualified agency representative within 48 hours of the referral or date requested by the client/representative and include the following:

a. Client demographics
b. Specific services requested.
c. Emergency contact
d. Environmental safety issues
e. Physician orders if required for state funding or insurance programs.
f. Payment source

Admission

1. The admission process begins with a supervisor visit within 48 hours of referral or longer if above identified issues exist, to explain all available agency services, client rights and responsibilities, payment for services, PHI confidentiality protection, advance directive, and Privacy Act materials, grievance process, and agency access availability 24 hours per day with contact numbers.
2. Upon receipt of the client's signed informed consent form, an assessment will be conducted.

Plan of Care

1. Based on the assessment with input from the client/family, the Supervisor will develop and implement a written home care plan of care for the client with written approval by the client or designated family member. The Supervisor will:
   g. Assure each plan of care is developed by appropriate agency personnel, lists services requested or recommended to meet client needs, and is based on an on-site visit, under agency policies and procedures.
   h. Ensure the home care plan of care includes:
      i. The client's functional limitations.
      iii. Nutritional needs and food allergies for meal preparation.
      iv. Home medical equipment and supplies relevant to the care plan.
      iv. Indication that the client has a signed advanced directive or POLST, if applicable.
      v. Nurse delegation tasks, if applicable; and
      vi. Specific nonmedical services to be provided and their frequency.
      vii. Type and schedule of services to be provided.

2. Ensure the plan of care is reviewed on-site, updated, approved, and signed by appropriate agency personnel and the client, designated family member, or legal representative every twelve months and whenever significant changes to client care needs are identified; and inform the supervisor of direct care services regarding changes in the client's condition that indicate a need to update the plan of care.
3. Home care agencies providing a one-time visit for a client may provide the following written documentation in lieu of the home care plan of care and client record requirements in WAC 246-335-450(3):

a. Client name, age, current address, and phone number.
b. Confirmation that the client was provided with a written bill of rights under WAC 246-335-435.
c. Client consent for services to be provided; and
d. Documentation of services provided.

4. Home care agencies that have a contract with the AAA to provide home care services to Medicaid eligible clients may use the DSHS CARE assessment details and service summary, or successor assessment tool, as the plan of care if it covers all items in sub-section (3) WAC 246-335-440.
5. For non-skilled Homemaker services, the service plan includes:

a. Specific duties to be performed.
b. Schedule of services
c. Client specific safety concerns, client preferences, and goals

REFERENCES

WAC 246-335-410 WAC 246-335-435 WAC 246-335-440 WAC 246-335-450

CLIENT RIGHTS AND RESPONSIBILITIES

Policy

Golden Years Home Health Supported Living LLC supports the principle that each client has the right to dignity, respect, and involvement in his or her service plan. The client has the right to be informed of his or her rights. The agency will protect and promote the exercise of these rights. The agency will provide the client with a written notice of the client's rights in advance of furnishing care to the client or during the initial evaluation visit before the initiation of treatment. The admitting agency representative will review a written Client Bill of Rights with the client prior to initiating service. The client or their legal guardian will sign confirmation of receipt of their rights and will be given a copy. The text of the Client Bill of Rights conforms to accrediting body, state mandates, if required. Agency admission is non-discriminatory. The agency will inform the client orally and in writing of any liability for payment not covered by insurance prior to the start of services or within 30 days of the agency learning that there will be client liability for payment.

Purpose

To promote the ability of the client/representative to understand and exercise their rights.

Procedure

Client Bill of Rights

The Agency at the time of admission will provide each patient or client, or designated family member, with a written bill of rights affirming each individual's right to:

1. Receive effective treatment and quality services from the home care agency for services identified in the care plan.
2. Be cared for by appropriately trained or credentialed personnel, contractors, and volunteers with coordination of services.
3. A statement advising of the right to ongoing participation in the development of the care plan.
4. A statement advising of the right to have access to the department's listing of licensed home care agencies and to select any licensee to provide care, subject to the individual's reimbursement mechanism or other relevant contractual obligations.
5. A listing of the total services offered by the home care agency and those being provided to the client.
6. Refuse specific treatments or services.

7. The name of the individual within the home care agency responsible for supervising the client's care and the way that individual may be contacted.
8. Be treated with courtesy, respect, and privacy.
9. Be free from verbal, mental, sexual, and physical abuse, neglect, exploitation, and discrimination.
10. Have property treated with respect.
11. Privacy and confidentiality of personal information and health care related records.
12. Be informed of what the home care agency charges for services, to what extent payment may be expected from health insurance, public programs, or other sources, and what charges the client may be responsible for paying.
13. A fully itemized billing statement upon request, including the date of each service and the charge. Agencies providing services through a managed care plan are not required to provide itemized billing statements.
14. Be informed about advanced directives and POLST and the agency's scope of responsibility.
15. Be informed of the agency's policies and procedures regarding the circumstances that may cause the agency to discharge a client.
16. Be informed of the agency's policies and procedures for providing back-up care when services cannot be provided as scheduled.
17. A description of the agency's process for clients and family submitting complaints to the home care agency about the services and care they are receiving and to have those complaints addressed without retaliation.
18. Be informed of the department's complaint hotline number (1-800-6336828) to report complaints about the licensed agency or credentialed health care professionals; and
19. Be informed of the DSHS end harm hotline number (1-866-363-4276) to report suspected abuse of children or vulnerable adults.
20. ensures that the client rights are implemented and updated as appropriate. ensures that the client rights under this section are implemented and updated as appropriate.

Client Responsibilities

1. Providing complete and accurate information about illnesses, hospitalizations, medications and other matters relating to your health.
2. Inform our agency when you will not be able to keep home care visits.
3. Treating our agency's personnel with respect.

4. Cooperating with our agency's personnel and asking questions if you do not understand instructions or information given to you.
5. Following your home care plan.
6. Participating in the planning of your home care and treatment.

REFERENCES

WAC 249-335-435 WAC 246-335-420

ADVANCE DIRECTIVES

Definitions

Advance Directive: A document in which a person either states choices for medical treatment or designees who should make treatment choices if the person loses decisional capacity. Advance directives may include a Living Will, and/or Durable Power of Attorney for Health Care, POLST, or other document recognized by state law. In addition, any oral statement by a competent client of his or her desire for treatment or withholding of treatment may be considered an advance directive.

Do Not Resuscitate (DNR): A medical order to refrain from cardiopulmonary resuscitation if a client's heart stops beating.

Surrogate Decision Maker: A person legally appointed to make decisions for someone else, as in a durable power of attorney for health care (also called an agent).

Policy

Golden Years Home Health Supported Living LLC recognizes that all persons have a fundamental right to make decisions relating to their own services, including the right to accept or refuse service. It is the policy of the agency to encourage clients to participate in decisions regarding agency services. Valid advanced directives, such as Living Wills, Durable Powers of Attorney for Health Care, and DNR Physician Order for Life-Sustaining Treatment (POLST) will be followed to the extent permitted and required by state laws. In the absence of advanced directives, the agency will provide appropriate emergency procedures, including calling 911 and providing CPR by qualified professionals. Staff will be notified of clients who have a physician signed DNR order. The agency will not condition the provision of services or otherwise discriminate against a client based on whether the individual has executed an advance directive. Agency clients will be informed of their right to use state or accrediting body hotline numbers to lodge complaints concerning the implementation of the advanced directive rules.

Purpose

1. To determine a client/representative's preferences for health care decisions and to honor those wishes.
2. To inform personnel of the advance directives content so it will be honored.

Procedure

1. At the time of admission, the agency representative will provide the client/representative with written information about advance directives and related rights and discuss and respond to any questions the client may have materials.
2. If the client has a signed POLST form, any section of the POLST form not completed implies full treatment for that section.
3. In the event of a client medical emergency and agency staff are present, provide emergency medical personnel with a client's signed POLST form.
4. The written materials for Advanced Directives will contain:
   a. The definition of advance directive.
   b. The client’s rights under state law to make decisions about medical care including the right to formulate an advance directive.
   c. The agency will not condition services or otherwise discriminate against the client based on whether the client has executed an advance directive.
   d. The agency’s policies related to advance directives include any limitations caused by the agency’s conscientious objection to implementing a particular advance directive element. The agency will clarify for the client if the state allows the agency legal authority permitting an objection and the range of medical conditions affected by the objection.
   e. Competent adults, 18 years of age or older, have the right to refuse or accept medical services or treatment. Clients under 18 years of age shall have decisions made by their parents/legal guardians.
   f. To ensure compliance with state laws, the agency will inform the client of their right to lodge complaints concerning advanced directives via agency, state, and/or accrediting body hotlines.
5. No agency personnel are permitted to give medical or legal advance directive advice.
6. The agency will document whether the client has executed an advance directive and request a copy for the client's record. If a client is transferred to another agency or facility, pertinent information concerning the advance directives or a copy of it will be forwarded to the facility.
7. If amended, the agency is required to update and disseminate the new advance directive instructions in 30 days, but no later than 90 days from notice of the amendment.
8. The agency will inform involved personnel of the client’s advanced directive content, including DNR orders, so it will be followed, and client’s wishes honored.

9. If a client is incapacitated at the time of admission or start of services, the family or surrogate may give advance directive information. Follow-up procedures must be in place to provide advance directive information to the individual once he or she is no longer incapacitated.
10. If a client is deemed to be incompetent to make decisions, then a legal surrogate will be identified. The agency will inform the surrogate of the plan of services and include the surrogate in client-related decisions.
11. A separate signed DNR or no-code (POLST) order will be obtained for clients who do not want CPR.
12. Updates in state laws affecting advance directives will be provided to clients/family within 30 days of the effective date of the law change
13. Each client is encouraged to participate in all aspects of decision-making regarding service. Statements by a competent client of his or her desire to accept or refuse service will be documented in the client's record.
14. Withdrawal or forgoing life sustaining measures will be the sole responsibility of the physician and client/representative and entered in the record.
15. A client may revoke an advanced directive at any time either orally or in writing. The Supervisor will notify the physician of any revocation. Revocation documentation will be in the client record.
16. As part of its mission, the agency provides community education about advance directives and related client rights in written materials and public speaking engagements as requested.

REFERENCES

WAC 246-335-420 WAC 246-335-435 WAC 246-335-440

ON-CALL SERVICES

Policy

Golden Years Home Health Supported Living LLC provides In Home Care Aides and Homemaker services. Services are provided on an hourly basis, per visit, or per diem basis in the client's place of residence or assisted living facility. Access to service coverage is available 24 hours/day. During the week, the Scheduler/Client

Care Coordinator is available during business hours at the agency's office phone number. After hours, weekends, and holidays, the agency provides coverage through on-call personnel and can be accessed through the on-call phone number provided to client/representative at the time of admission.

Purpose

To ensure that clients have agency personnel available to address their needs regardless of the time of day or day of the week.

Procedure

On-Call Procedure

1. A qualified employee is on call to accept client calls, informational calls, or referrals for service; to address emergency services; and to provide support to personnel making home visits after normal business hours. A qualified agency supervisor is always available to provide supervisory assistance to the on-call coordinator and other staff working after hours.
2. The on-call coordinator is responsible for documenting all calls received and the actions taken.
3. On admission, the agency personnel provide clients with accurate written information that includes the agency's business hours and telephone numbers.
4. When judged necessary by the on-call supervisor, emergency visits will be made.

HOME CARE AIDE SERVICES

Policy

Golden Years Home Health Supported Living LLC provides directly or by arrangement, Home Care Aid services that may be used in lieu of a certified Home Care Aide. Home care services are described as non-medical services and assistance provided to ill, disabled, or vulnerable individuals that enable them to remain in their residences. Home care services include but are not limited to: Personal care such as assistance with dressing, feeding, and personal hygiene to facilitate self-care; homemaker assistance with household tasks, such as housekeeping, shopping, meal planning and preparation, and transportation; respite care assistance and support provided to the family; or other nonmedical services or delegated tasks of nursing under RCW 18.79.260 (3)(e). Services for In-Home Aide care are typically supplied by long-term care workers. "Long-term care workers" are individuals who provide hands-on personal care services to the elderly or persons with disabilities according to RCW 74.39A.009. A long-term care worker employed by a home care agency is either credentialed as a certified home care aide under chapters 18.88A RCW and 246-980 WAC or meets the exemption criteria according to chapter 246-980 WAC. If the home care agency chooses to offer assistance with taking vital signs, it is understood that:

1. Assistance with taking vital signs for informational purposes only, due to client being unable to complete tasks independently. Home care agency workers may only assist clients in the process of taking their own vital signs. Examples of assistance include, but are not limited to, handing client a digital thermometer, sliding blood pressure cuff over client's arm, turning on a device, recording digital readings and communicating those readings back to the client, designated family member, or legal representative. Devices used must be electronically operated with digital readouts; and

2. Assistance with taking vital signs to determine when to take or exclude prescribed medications, or what dosage of medication to take due to client being unable to complete tasks independently. Agencies must contract with a registered nurse to determine if nurse delegation is appropriate and train agency worker(s) on taking specific vital signs and to understand how the readings relate to the medications that the client needs to take. If the agency chooses to offer assistance with passive range of motion exercises for maintenance purposes only, then relevant policies and procedures must comply with the following minimum requirements: (a) Ensure the client provides the agency with a copy of their passive range of motion exercise plan established by a physical therapist licensed under chapter 18.74 RCW, an occupational therapist licensed under chapter 18.59 RCW, or qualified registered nurse licensed under chapter 18.79 RCW. The date of the plan must be within twelve months of requesting assistance with passive range of motion. The plan must clearly state that the passive range of motion is for maintenance purposes only. Passive range of motion for purposes of restoring joint function is outside the scope of a home care agency to provide:

a. If the exercise plan is older than twelve months or does not clearly state for maintenance purposes only, the agency will direct clients to get an updated or new passive range of motion plan from their health care provider.
b. Ensure and document passive range of motion skills verification of assigned agency workers, consistent with WAC 246-335- 425(9), prior to the provision of these services; and
c. Ensure clients receiving passive range of motion submit to the agency an updated exercise plan from their health care provider at least annually.

Purpose

To ensure services occur according to state statutes, Medicaid requirements, if applicable, agency policies, and accrediting body standards.

Procedure

1. The Home Care Aid follows the service plan established by the Supervisor.
2. The Home Care Aid must meet all requirements for competency and training required by state statutes and accrediting body standards if applicable.
3. The Home Care Aid is selected on such factors as, a sympathetic attitude toward the care of the sick, ability to read, write, and carry out directions, maturity, and ability to deal effectively with the demands of the job.
4. The Home Care Aid may not replace a certified Home Care Aide and is generally used for clients with limited ADL and IDLS but not requiring skilled professional services.

5. The first visit will be within seven days of referral unless otherwise specified or requested by the client.
6. The Home Care Aid duties include:
    d. Activities of daily living" or "ADL" means routine activities performed around the home or in the community and include:
        i. "Ambulation" means how an individual moves between locations in their immediate living environment and how they move to and return from more distant areas. Assistance with ambulation includes supervising or guiding the client or patient when walking alone or with the help of a mechanical device such as a walker, assisting with difficult parts of walking such as climbing stairs, supervising, or guiding the client or patient if they can propel a wheelchair, pushing of the wheelchair, and providing constant or standby physical assistance to the client or patient if totally unable to walk alone or with a mechanical device

ii. "Bathing" means how an individual takes a full-body bath or shower, sponge bath, and transfers in and out of the tub or shower. Assistance with bathing includes supervising or guiding the client or patient to bathe, assisting the client or patient with difficult tasks such as getting in or out of the tub or shower, washing their back and other hard to reach areas, and completely bathing the client or patient if they are totally unable to wash self.

iii. "Body care" means how an individual performs applications of dressings and ointments or lotions to their body, trims their toenails, and applies lotion to their feet. Assistance with body care includes general skin care and the application of over-the-counter ointments or lotions. Body care excludes foot care for clients or patients who are diabetic or have poor circulation and changing bandages or dressings when sterile procedures are required. [must be done by an LPN or RN]

iv. "Dressing" means how an individual puts on, fastens, and takes off all items of clothing, including donning or removing a prosthesis. Assistance with dressing includes supervising or cuing the client or patient to dress and assisting them with difficult tasks such as putting on socks, pants, shoes, and fastening, zipping, or tying clothing related items.

v. "Eating" means how an individual eats and drinks, regardless of skill. Assistance with eating includes supervising or guiding the client or patient when they can feed themselves, assisting with difficult tasks such as cutting food or buttering bread, and orally feeding the client or patient when they are unable to feed themselves.

vi. "Medication management" means how an individual ingests or applies medications or herbal supplements. Assistance with medication management includes reminding, coaching, and handing medication containers to the client or patient.

vii. "Personal hygiene" means how an individual maintains their personal hygiene. Assistance with personal hygiene includes helping the client or patient with combing hair, brushing teeth, shaving, applying makeup, washing, and drying face, trimming fingernails, applying nail polish, and perineum (menses care).

viii. "Positioning" means how an individual moves to and from a lying position, turns side to side, and positions their body while in bed, in a recliner, or other type of furniture. Assistance with positioning includes helping the client or patient to assume a desired position, helping with turning, and setting up for the client or patient to perform exercises or active range of motion. Positioning assistance may also include passive range of motion to maintain joint flexibility or prevent complications, such as contractures and pressure sores

ix. "Toileting" means how an individual uses the toilet room, commode, bedpan, or urinal. Assistance with toileting includes helping the client or patient to and from the bathroom, assisting with bedpan routines and urinals, using incontinent briefs, cleaning after elimination, and assisting the client or patient on and off the toilet.

x. "Transfer" means how an individual moves between surfaces such as to and from a bed, chair, wheelchair, or standing position. Assistance with transferring includes helping the client or patient with getting in and out of a bed or wheelchair or on and off the toilet or in and out of the bathtub. Transfer includes supervising or guiding the client or patient when they can transfer, providing steady assistance, and helping the client or patient when they are able to assist in their own transfers. This does not include transfers when the client or patient is unable to assist in their own transfer or needs assistive devices unless specific training or skills verification has occurred consistent with agency policies and procedures.

xi. Meal preparation" means how an individual prepares their meals. Assistance with meal preparation includes planning meals for clients or patients, cooking, assembling ingredients, setting out food, utensils, and cleaning up after meals.

xii. "Ordinary housework" means how an individual performs ordinary work around the house. Assistance with ordinary housework includes washing dishes and cookware, dusting, vacuuming, mopping, making bed, tidying up, laundry, taking out garbage, or other activities like.

xiii. "Essential shopping" means how an individual completes shopping tasks to meet their health and nutritional needs. Assistance with essential shopping includes trips in the local area to shop for food, medical necessities and household items required specifically for a client or patient's health, maintenance, or wellbeing. Shopping assistance can be provided with a client or patient or on their behalf.

xiv. Wood supply" means how an individual supplies their home with wood when wood is used as the sole source of fuel for heating or cooking. Assistance with wood supply includes splitting, stacking, carrying wood, or other like activities.

xv. "Travel to medical services" means how an individual travels by vehicle, bus, or taxi to a physician's office or clinic in the local area to obtain medical diagnosis or treatment. Assistance with travel to medical services includes driving the client or patient for traveling as a support person in a personal vehicle, bus, or taxi. Within the context of IADL services, travel to medical services does not include client or patient transfer assistance.

xvi. Managing finances" means how an individual manages their personal finances. Assistance with managing finances includes helping the client or patient to pay bills, balance checkbook, or other like activities.

Xvii. Telephone use" means how an individual makes and receives telephone calls and text messages. Assistance with telephone use includes bringing a phone to the client or patient, helping with dialing numbers, helping with sending and retrieving text messages, helping with general phone operation, or other like activities.

xvii. Medication self-administration with assistance" means reminding or coaching the client or patient to take their medication, handing the medication container to the client or patient, opening the medication container, using an enabler, or placing the medication in the hand of the client or patient, along with other assistance, as defined by the pharmacy quality assurance commission in chapter 246-888 WAC.

7. If the Client wishes assistance with vital signs, the assistance will be for informational purposes only, due to the client being unable to complete tasks independently.
8. Home care agency workers will only assist clients in the process of taking their own vital signs.

a. Examples of assistance include, but are not limited to, handing clients a digital thermometer, sliding blood pressure cuff over client's arm, turning on a device, recording digital readings and communicating those readings back to the client, designated family member, or legal representative.
b. Devices used must be electronically operated with digital readouts.

9. If assistance with taking vital signs to determine when to take or exclude prescribed medications, or what dosage of medication to take due to client being unable to complete tasks independently the agency will contract with

a. registered nurse to determine if nurse delegation is appropriate and train agency worker(s) on taking specific vital signs and to understand how the readings relate to the medications that the client needs to take;

10. The agency will offer assistance with passive range of motion exercises for maintenance purposes only, then relevant policies and procedures must comply with the following minimum requirements:

a. Ensure the client provides the agency with a copy of their passive range of motion exercise plan established by a physical therapist licensed under chapter 18.74 RCW, an occupational therapist licensed under chapter 18.59 RCW, or qualified registered nurse licensed under chapter 18.79 RCW. The date of the plan must be within twelve months of requesting assistance with passive range of motion. The plan must clearly state that the passive range of motion is for maintenance purposes only. Passive range of motion for purposes of restoring joint function is outside the scope of a home care agency to provide.

b. If the exercise plan is older than twelve months or does not clearly state for maintenance purposes only, the agency will direct clients to get an updated or new passive range of motion plan from their health care provider.

c. Ensure and document passive range of motion skills verification of assigned agency workers, consistent with WAC 246-335425(9), prior to the provision of these services.

d. Ensure clients receiving passive range of motion submit to the agency an updated exercise plan from their health care provider at least annually.

11. Home Care Aid services clients are contacted every month for the first 12 months to evaluate the services, and a qualified supervisor will visit the client every three months to observe services being performed and determine if any service plan changes are needed.

REFERENCES

WAC 246-335-310 WAC 246-355-410 WAC 246-335-420 WAC 246-335-425

HOME CARE AIDE CERTIFICATION REQUIREMENTS

Policy

Any person who is hired on or after January 7, 2012, as a long-term care worker for the elderly or persons with disabilities, regardless of the employment title, must obtain certification as a home care aide, unless exempt under WAC 246-980025. This includes, but is not limited to:

1. An individual provider of home care services who is reimbursed by the state.
2. A direct care employee of a home care agency.
3. A provider of home care services to persons with developmental disabilities under Title 71A RCW.
4. A direct care worker in a state licensed assisted living facility.
5. A direct care worker in a state licensed adult family home.
6. A respite care provider who is reimbursed by the state or employed by a private agency or facility licensed by the state to provide personal care services; and
7. Other direct care workers provide home or community-based services to the elderly or persons with developmental disabilities.

The following individuals are not required to obtain certification as a home care aide. If they choose to voluntarily become certified, they must successfully pass the entry level training required by RCW 74.39A.074 and meet the requirements of WAC 246-980-040 (1)(b) and (c).

1. An individual provider caring only for a biological, step, or adoptive child or parent.
2. An individual provider who provides twenty hours or less of care for one person in any calendar month.
3. An individual employed by a community residential service business.
4. An individual employed by a residential habilitation center licensed under chapter 71A.20 RCW or a facility certified under 42 C.F.R. Part 483.
5. A direct care worker who is not paid by the state or by a private agency or facility licensed by the state to provide personal care services.
6. A person working as an individual provider who only provides respite services and works less than three hundred hours in any calendar year.
7. Any direct care worker exempts under RCW 18.88B.041(1).

The following long-term care workers are not required to obtain certification as a home care aide. If they choose to voluntarily become certified, they must meet the requirements of

WAC 246-980-040 (1)(b) and (c). The training requirements under RCW 74.39A.074(1) are not required.

a. An individual who holds an active credential by the department as a:
   i. Registered nurse, a licensed practical nurse, or advanced registered nurse practitioner under chapter 18.79 RCW; or
   ii. Nursing assistant-certified under chapter 18.88A RCW.
b. A home care aide who was employed by a certified home care agency within the year before being hired as a long-term care worker and has met the requirements of 42 C.F.R. Part 484.36.
c. A person who is in an approved training program for certified nursing assistant under chapter 18.88A RCW, provided that the training program is completed within one hundred twenty calendar days of the date of hire and that the nursing assistant certified credential has been issued within two hundred calendar days of the date of hire.
d. An individual with special education training and an endorsement granted by the superintendent of public instruction under RCW 28A.300.010 and is approved by the secretary.
e. An individual employed as a long-term care worker on January 6, 2012, or who was employed as a long-term care worker between January 1, 2011, and January 6, 2012, and who completed all the training requirements in effect as of the date of hire. This exemption expires if the long-term care worker has not provided care for three consecutive years.

8. The department may require an exempt long-term care worker as a long-term care worker between January 1, 2011, and January 6, 2012, to provide proof of that employment. Proof may include a letter or similar documentation from the employer that hired the long-term care worker between January 1, 2011, and January 6, 2012, indicating the first and last day of employment, the job title, a job description, and proof of completing training requirements. Proof of training will also be accepted directly from the approved instructor or training program, if applicable.

9. For an individual provider reimbursed by the department of social and health services, the department will accept verification from the department of social and health services or the training partnership.

Purpose

To ensure appropriately trained staff are providing care to the clients.

Procedure

1. To qualify for certification as a home care aide, the applicant must:
   a. Successfully complete all training required by RCW 74.39A.074(1) within one hundred twenty calendar days of the date of hire as a long-term care worker.
   b. Successfully pass the home care aide certification examination, after completing training; and
   c. Become certified within two hundred days of date of hire, or two hundred sixty days if granted a provisional certificate under RCW 18.88B.041.
2. An applicant for certification as a home care aide must submit to the department:

a. A completed application for both certification and the examination on forms provided by the department.
b. The exam fee set by the examination vendor and required fees under WAC 246-980-990; and
c. A certificate of completion from an approved training program indicating that the applicant has successfully completed the entry level training required by RCW 74.39A.074. The certificate of completion or other official verification may also be submitted directly from the approved instructor or training program.

3. An applicant must submit to a state background check as required by RCW 74.39A.056.
4. An applicant exempts from certification under WAC 246-980-025(2) who voluntarily chooses to be certified must provide documentation of qualification for the exemption. The applicant is not required to take the training required in subsection (1)(a) of this section or provide proof of training completion to the department. Working while obtaining certification as a home care aide.

1. A long-term care worker may provide care before receiving certification as a home care aide if all the following conditions are met:
   a. Before providing care, the long-term care worker must complete the training required by RCW 74.39A.074 (1)(d)(i)(A) and (B).
   b. The long-term care worker must submit an application for home care aide certification to the department within fourteen calendar days of hire. An application is submitted on the date it is post-marked or, for applications submitted in person or online, the date it is accepted by the department.
2. A long-term care worker is no longer eligible to provide care without credential under the following circumstances:

a. The long-term care worker does not successfully complete all the training required by RCW 74.39A.074(1) within one hundred twenty calendar days from their date of hire;
b. The long-term care worker has not obtained their certification within two hundred calendar days from their date of hire, or two hundred sixty calendar days if granted a provisional certificate under RCW 18.88B.041.

3. This section does not apply to long-term care workers exempt from certification under WAC 246-980-025. Home care aide provisional certification.

1. The department may issue a provisional certification to a long-term care worker who is limited-English proficient and submits:
   a. A request for a provisional certification; and
   b. Verification of his or her date of hire as a long-term care worker, as follows:
      i. For individual providers, verification of the applicant's date of hire submitted directly by the department of social and health services; or
      ii. For all other applicants, a form supplied by the department and completed by the employer who hired the applicant as a long-term care worker, verifying the applicant's date of hire.
2. A provisional certification will be issued only once.
3. A provisional certification will take two hundred days from the applicant's date of hire as a long-term care worker, as defined in WAC 246980-010(2).
4. A provisional certification will expire two hundred sixty days from the applicant's date of hire as a long-term care worker. The applicant must stop working on the two hundred sixtieth day of employment if the certification has not been granted for any reason.
5. A request for provisional certification may be denied pursuant to chapter 18.130 RCW, the Uniform Disciplinary Act, or based on RCW 18.88B.080.
6. For the purposes of this section, "limited-English proficient" means that an individual is limited in his or her ability to read, write, or speak English.

Examination and re-examination for home care aide certification.

1. The certification examination will consist of both a written knowledge test and a skills demonstration.
2. The certification examination will test the core competencies, including but not limited to:
   a. Communication skills.
   b. Worker self-care.

c. Problem solving.
d. Maintaining dignity.
e. Consumer directed care.
f. Cultural sensitivity.
g. Body mechanics.
h. Fall prevention.
i. Skin and body care.
j. Home care aide roles and boundaries.
k. Supporting activities of daily living; and
l. Food preparation and handling.

3. An applicant must apply to take the examination by completing the application for both certification and the examination and returning it to the department. The department will notify the examination contractor once an applicant meets all requirements to take the certification examination.

4. The examination contractor will notify an applicant of the date, time, and place of the examination.

5. The examination contractor will notify both the department and applicant of the examination results.

a. An applicant who does not successfully pass any portion of the examination can follow the examination contractor's procedures for review and appeal.
b. An applicant who does not successfully pass any portion of the examination may retake that portion of the examination two times.
i. To retake the examination, an applicant must apply for reexamination, along with the required reexamination fee directly to the examination contractor.

ii. An application for reexamination may be submitted any time after an applicant receives notice of not successfully completing any portion of the certification examination.

6. An applicant who does not successfully pass both portions of the certification examination within two years of successfully completing the required training or who does not successfully pass both portions of the certification examination after completing the certification examination three consecutive times:

(i) Must retake and successfully complete the core competencies portion of the entry-level training as required by RCW 74.39A.074 before retaking both portions of the certification examination; and

(ii) Cannot continue to provide care as a long-term care worker until the certification has been issued. Continuing education.

1. A home care aide must demonstrate completion of twelve hours of continuing education per year as required by RCW 74.39A.341. The required continuing education must be obtained during the period between renewals. Continuing education is subject to the provisions of chapter 24612 WAC, Part 7.
2. Verification of completion of the continuing education requirement is due upon renewal. If the first renewal period is less than a full year from the date of certification, no continuing education will be due for the first renewal period.

Renew or reinstate an expired certification.

1. To renew a home care aide certification the practitioner must:
   a. Renew the certification every year by the home care aide's birthday as provided in chapter 246-12 WAC, Part 2;
   b. Submit a completed application as provided by the department; and
   c. Provide verification of twelve hours of continuing education as required by RCW 74.39A.341 and WAC 246-980-110 with the renewal application.
2. To reinstate an expired certification:

a. If the certification has been expired for less than three years, the practitioner must submit proof of twelve continuing education hours as required by RCW 74.39A.341 and WAC 246-980-110 for each year, and meet the requirements of chapter 246-12 WAC, Part 2.
b. If the certification has expired for three years or more, the practitioner must successfully repeat the training and examination requirements in WAC 246-980-040 and meet the requirements of chapter 246-12 WAC, Part 2.
c. A practitioner previously exempts from certification by WAC 246980-025(2) who voluntarily chooses to be certified is not required three 246-980-025(2) at the time of reapplying.

Provision for delegation of certain tasks to a home care aide.

1. A home care aide-certified may perform tasks delegated by a registered nurse for clients in community-based care settings or in-home care settings, each as defined in RCW 18.79.2603)(e).
2. Before performing any delegated task, a home care aide-certified must show the certificate of completion of the core delegation training from the department of social and health services to the registered nurse delegator.

3. A home care aide-certified person who is performing nurse delegation tasks must comply with all applicable requirements of the nursing care quality assurance commission in WAC 246-840-910 through 246-840-970.
4. A home care aide-certified person who may be performing insulin injections must show a certificate of completion of diabetic training from the department of social and health services to the registered nurse delegator.
5. A home care aide-certified must meet any additional training requirements identified by the department of social and health services.
6. For the purposes of this section, delegated nursing care tasks must be performed:
    d. Only for the specific client for whom those tasks are delegated.
    e. Only with the client's consent; and
    f. In compliance with all applicable requirements in WAC 246-840910 through 246-840-970.
7. A home care aide-certified may consent or refuse to consent to perform a delegated nursing care task. The home care aide-certified is responsible for his or her own actions with the decision to consent or refuse to consent and the performance of the delegated nursing care task.
8. A home care aide-certified must not accept delegation of, or perform, the following nursing care tasks:

a. Administration of medication by injection, except for insulin injections.
b. Sterile procedures.
c. Central line maintenance.
d. Acts that require nursing judgment.

9. A person who is working as a long-term care worker but has not received a home care aide certification must have either an active nursing assistant certified or an active nursing assistant-registered credential issued by the department and comply with WAC 246-841-405 to perform delegated tasks.

Scope of practice for long-term care workers.

1. A long-term care worker performs activities of daily living or activities of daily living and instrumental activities of daily living. A person performing only instrumental activities of daily living is not acting under the long-term care worker scope of practice.
    a. "Activities of daily living" means self-care abilities related to personal care such as bathing, eating, medication assistance, using the toilet, dressing, and transfer. This may include fall prevention, skin and body care.

   b. "Instrumental activities of daily living" means activities in the home and community including cooking, shopping, house cleaning, doing laundry, working, and managing personal finances.

2. A long-term care worker documents observations and tasks completed, as well as communicates observations.

3. A long-term care worker may perform medication assistance as described in chapter 246-888 WAC.

4. A long-term care worker may perform nurse delegated tasks, to include medication administration, if he or she meets and follows the requirements in WAC 246-980-130.

5. A long-term care worker may provide skills acquisition training on instrumental activities of daily living and the following activities of daily living tasks: Dressing, application of deodorant, washing hands and face, hair washing, hair combing and styling, application of makeup, menses care, shaving with an electric razor, tooth brushing or denture care, and bathing tasks excluding any transfers in or out of the bathing area.

6. This section applies to all long-term care workers, whether required to be certified or exempt.

Standards of practice.

1. A long-term care worker must demonstrate behavior which maintains and respects client or resident rights and safety. This includes, but is not limited to, the following:
   a. A long-term care worker may not solicit, accept or borrow money, material or property from a client or resident. This subsection does not apply to a long-term care worker who is in an established personal relationship with the client, preexisting the provision of services, where there is no evidence of exploiting the client.
   b. A long-term care worker may not accept from a client or resident gifts of value greater than twice the current hourly minimum wage in Washington state. Gifts are limited to customary gift giving times, such as birthdays or major holidays. This subsection does not apply to a long-term care worker who is in an established personal relationship with the client, preexisting the provision of services, where there is no evidence of exploiting the client.
   c. A long-term care worker may not accept, borrow, or take alcohol or drugs (prescription or nonprescription), including marijuana, from a client or resident.

d. A long-term care worker may not ingest, inject, inhale, or consume in any manner any substance, including prescribed medicine, that impairs their ability to perform their job duties during the time in which they are paid to provide care.

e. A long-term care worker may not solicit or accept a role that gives them power over a client's or resident's finances, legal matters, property, or health care decisions. This includes, but is not limited to, acting as power of attorney, legal guardian, payee, insurance beneficiary, or executor or beneficiary of a will. This subsection does not apply to a long-term care worker who is in an established personal relationship with the client, preexisting the provision of services, where there is no evidence of exploiting the client.

f. A long-term care worker may not be the landlord for a client or resident they provide care to. This does not apply to adult family homes licensed by the department of social and health services so long as the adult family home license is active and in good standing. This section does not apply to a long-term care worker who is in an established personal relationship with the client, preexisting the provision of services, where there is no evidence of exploiting the client.

g. A long-term care worker shall respect a client's or resident's privacy and shall not take or disseminate photos or videos of a client or resident that do not respect the client's or resident's dignity and rights. This includes, but is not limited to, social media. A long-term care worker must obtain the written permission of the client or resident, or their legal guardian, prior to taking or disseminating any photo or video of the client or resident, unless the long-term care worker is in an established personal relationship with the client, preexisting the provision of services, where there is no evidence of exploiting the client.

2. For the purposes of this section, "landlord" means having a formal, written lease agreement between the lessor and lessee. It does not apply to situations in which cohabitants voluntarily contribute financially to household expenses without a lease agreement.

REFERENCES

WAC 246-335-420 WAC

246-335-425 WAC

246-335-430

HOMEMAKER SERVICES

Policy

The Golden Years Home Health Supported Living LLC Homemaker is an employee who provides in-home chores and support activities for agency clients as defined in the service plan to promote client independence at home for as long as possible. A Home Care Aide may provide these services as added service plan items. The Homemaker services may be covered in some state Medicaid or local assistance programs.

Purpose

To define the services provided by the agency's Homemaker.

Procedures

1. The Homemaker is qualified by evidence of successful completion of training, orientation, and competency testing by a qualified supervisor covering as a minimum:
   a. Overall limitations and responsibilities
   b. Communication techniques
   c. Confidentiality of client information
   d. Client rights
   e. Universal precautions/hand hygiene
   f. Infection control
   g. Homemaking functions
   h. Shopping, food preparation
   i. Laundry
   j. Age-appropriate companion activities
   k. Advance directive implications
   l. Safety precautions
2. The Homemaker is selected on such factors as a sympathetic attitude toward sick or elderly clients, ability to read and write, carry out directions, and maturely deal with the demands of the job.
3. The Homemaker does not provide Home Care Aide services.
4. Homemaker services shall include but not be limited to:

a. Provision of Homemaker services to clients reimbursed privately or by insurance sources which cover Homemaker services as outlined in the service plan per agency policies, procedures, and Homemaker skill level.
b. Preparing and submitting accurate and timely Homemaker notes on tasks performed and client response to services per agency policy.
c. Assisting in maintaining a clean, safe environment in client's residence by performing household chores such as vacuuming, dusting, mopping, laundry, cleaning kitchen and bath areas, making beds, and may perform errands that assist in maintaining the home.
d. Providing social interaction and assisting with community access as directed.
e. Assisting with meal preparation and cleaning up as needed.
f. Reporting client condition changes to the Supervisor
g. Responsibility for proper use and care of equipment
h. Using universal precautions, infection control, and appropriate protective equipment to protect clients and self.

5. The Supervisor contacts the client at least every month for the first six months and makes a home visit at least every six months with the Homemaker present.

CONTRACTED SERVICES

Policy

Golden Years Home Health Supported Living LLC will maintain control of the services provided by contract. The contracts will be signed and dated by authorized principals of each party and reviewed annually. Contract personnel may be used to augment personnel for high volume of staff coverage and to support student experiences in the community, if applicable. Contracts are reviewed before expiration and when needed for revisions. All contractors will receive yearly evaluations of services rendered on behalf of the agency. The Agency adheres to the Equal Employment and Affirmative Action requirements of RCW 49.60.180 when selecting contractors.

Purpose

1. To ensure contracted services are appropriate and effective.
2. To ensure contracted services meet home care standards of practice and state law.

Procedure

1. The agency may establish written contracts or agreements for personnel services under hourly or per visit arrangements, and products under a fee arrangement. The contract or agreement will include, but is not limited to the following:
   a. The Job descriptions that contain responsibilities and are consistent with health care professional credentialing and scope of practice as defined in relevant practice acts and rules.
   b. References check for personnel, contractors, and volunteers.
   c. Criminal background checks
   d. Specific services or products to be supplied at the agreed fee for each service or product are provided by qualified personnel in a safe and effective manner as ordered in the plan of services.
   e. Proof of required liability insurance.
   f. Contractors are required to adhere to applicable agency policies and procedures and state regulations.
   g. Contractor personnel are required to adhere to the PHI requirements of HIPAA and the Privacy Act Notice, and they can be held accountable for violation fines and penalties.
   h. Assurance by the contractor of the education, training, qualifications, and identification of personnel designated to provide services or products.

i. Method for the contractor to participate in performance improvement activities as appropriate.

j. Procedures for scheduling visits and documentation and submission of required notes including periodic summaries that verify the provisions of services or products in accordance with the written contract.

k. Required billing procedures, time frame for submission, and payments.

l. The agency retains exclusive authority to admit or discharge clients, establish the frequency and duration of services, and supplies or equipment needs for agency clients.

m. The agency retains administrative, fiscal, and professional oversight of the personnel and services provided under arrangement.

n. The agency retains responsibility for payment of contracted services by the contract terms.

o. Definition of the respective roles, if any, of the agency and the contracted individual or agency in the client admission process, assessment, plan of care development, review, and revision, client care conferences, and discharge planning.

p. Lists effective date of the contract, duration, renewal terms, and/or termination.

q. The rights and responsibilities of the agency or individual providing services and of the contracting agency in the coordination, supervision, and evaluation of the care or services provided.

r. The contractor's agreement not to provide service or to make any alteration to the scope and limitations of services set forth in the service plan without prior approval of the agency.

s. Clients are accepted for services by the agency only, which remains responsible for ensuring that any service provided adheres to the client's service plan.

t. The geographic area in which services will be provided.

u. The method of supervision of contracted personnel and the manner in which services will be controlled, coordinated, and evaluated by the agency.

v. For staffing contracts, the client service plan is provided to the contracted personnel.

w. The written staffing contracts will define the contractor's and/or agency's responsibilities for compliance with state and state regulations, including:

i. Compliance with client rights and confidentiality of PHI

iii. HIPPA compliance

iv. Job descriptions

v.

iv. Criminal background check

v. Verification of professional licensure in good standing

vii. Legal status to work in the United States

viii. Verification of work references

ix. Tuberculosis and Hepatitis B virus requirements

x. Applicability of Universal Precautions. Orientation and ongoing training regarding client services policies and procedures

xi. CPR per state requirement or every two-year renewal

2. Contracts will be reviewed annually to ensure they are current.

3. Contracts will be retained for five years following contract termination.

4. If student program contracts are in effect, they will define the agency's responsibilities, and the school's obligations related to services provided. The contract will include scope of practice for the students, the school's liability insurance coverage, and the applicable clauses of staffing contracts that appear in item 1 above.

REFERENCES

WAC 246-335-425

COORDINATION OF SERVICES

Policy

When Golden Years Home Health Supported Living LLC works with clients that are receiving other home-based services, the agency will promote continuity of services through effective internal and external communications, reporting, and evaluation of client services. This coordination of services includes the client/family, all professional and paraprofessional personnel, equipment and drug suppliers, facilities, and community agencies.

Purpose

1. To provide coordinated services that adheres to the client's plan of service.
2. To provide effective and efficient communications among the various community agencies and/or facilities involved in client services.
3. To maximize the client's achievement of goals by coordination of service.

Procedure

1. No physician orders are needed for non-skilled care. This agency does not directly receive physician orders for its clients unless required by a payor source.
2. Coordination is provided among services provided by the in-home services agency and includes:
   a. Coordination among services being provided by the agency having an additional home care or hospice service category; and
   b. Coordination with other agencies when the care being provided impacts on client health.
3. Respecting the client's right to confidentiality of PHI, the agency will arrange service coordination through meetings, telephone contact, email, facsimile, case conference, or other communication means on a need-to-know basis only, which may include, but is not limited to:

a. Client assessment
b. Client service
c. Client reassessment
d. Plan of service covering diagnosis, mental status, types of home care services and equipment required, nutritional requirements, medications and treatments, and safety measures to protect against injury, changes in frequency, disciplines, level of service, or other home care service based on the client/representative desire and ability and to reach client desired goals.

e. Services provided to the client by other community agencies.

4. If the agency has a contract with another agency for providing non-skilled care to clients of the other agency, the contract shall include an addendum that defines each agency's responsibility and services to the client to prevent duplication and confusion regarding assigned duties. There should be client specific instructions in the client's home for each agency's services.

REFERENCES

246-335-420

MEDICATION ASSISTANCE

Policy

Golden Years Home Health Supported Living LLC complies with the Washington State laws and appropriate professional acts and provisions that stipulate the role that Home Care Aide may provide assistance with client self-administered medication administration.

Self-administration with assistance means assistance with legend drugs and controlled substances rendered by a nonpractitioner to an individual residing in a community-based care setting or an in-home care setting. It includes reminding or coaching the individual to take their medication, handing the medication container to the individual, opening the medication container, using an enabler, or placing the medication in the hand of the individual/resident. The individual/resident must be able to put the medication into his or her mouth or apply or instill the medication.

The individual/resident does not necessarily need to state the name of the medication, intended effects, side effects, or other details, but must be aware that he/she is receiving medications. Assistance may be provided with prefilled insulin syringes. Assistance is limited to handing the prefilled insulin syringe to an individual/resident. Assistance with the administration of any other intravenous and/or injectable medication is specifically excluded. The individual/resident retains the right to refuse medication. Self-administration with assistance shall occur immediately prior to the ingestion or application of a medication.

Independent self-administration occurs when an individual/resident is independently able to directly apply a legend drug or controlled substance by ingestion, inhalation, injection, or other means. In licensed boarding homes, self-administration may include situations in which an individual cannot physically self-administer medications but can accurately direct others per WAC 388-78A-300. These regulations do not limit the rights of people with functional disabilities to self-direct care according to chapter 74.39 RCW.

If an individual/resident cannot knowingly or physically ingest or apply a medication independently or with assistance, then the medication must be administered to the individual by a person legally authorized to do so

(e.g., physician, nurse, pharmacist). All laws and regulations applicable to medication administration apply. If an individual cannot safely self-administer medication or self-administer with assistance and/or cannot indicate an awareness that he or she is taking a medication, then only authorized individuals may administer the

medications.

Purpose

1. To ensure compliance with the state statutes regarding the assistance and administration of medications in the home.
2. To ensure proper documentation of medication used by clients.

Procedure

1. An individual/resident who resides in a community-based care setting or an in-home setting or his or her representative may request self-administration with assistance. A nonpractitioner may help in the preparation of legend drugs and controlled substances for self-administration where a practitioner has determined and communicated orally or by written direction that such medication preparation assistance is necessary and appropriate.
2. No additional separate assessments or documentation of the needs of the individual/resident are required to initiate self-administration with assistance. It is recommended that providers document their decision-making process in the health record of the individual or resident health record.
3. The Supervisor is responsible for maintaining oversight and management of all client medications and treatments if the staff is providing support to those medications or treatments.
4. The Home Care Aide is limited to assistance with medications ordinarily self-administered by the client. This assistance is limited to:
   a. Reminding the client when it is time to take medication.
   b. Reading the label of the medication container.
   c. Handing the client-owned medication container to the client.
   d. Opening the medication container.
   e. Use of an enabler or placing the medication in the hand of the client.
   f. Applying/installing skin, rectal, nose, eye and ear preparations under specific training and directions of a nurse delegator.
   g. Insulin administration under nurse delegation is only permitted by state codes.
5. Enablers are physical devices used to facilitate an individual's/resident's self-administration of a medication. Physical devices include, but are not limited to, a medicine cup, glass, cup, spoon, bowl, prefilled syringes, syringes used to measure liquids, specially adapted table surface, straw, piece of cloth or fabric.
6. An individual's hand may also be an enabler. The practice of "hand-overhand" administration is not allowed.

7. Medication with assistance includes steadying or guiding an individual's hand while he or she applies or instills medications such as ointments and eye, ear, and nasal preparations. Hand over hand is not allowed.
8. The client must be able to put the medication into his/her mouth or apply or instill the medication.
9. If a client cannot physically ingest or apply a medication independently or with assistance, or is not aware that they are taking medications, then the medications must be administered by an individual legally authorized to do so. A Home Care Aide is not considered authorized under these circumstances.
10. Drug alteration for medication assistance shall occur immediately prior to the ingestion or application of a medicine. Drug alteration requirements:

a. Medications may be altered for self-administration with assistance. Such alterations include, but are not limited to, crushing tablets, cutting tablets in half, opening capsules, mixing powdered medications with foods or liquids, and mixing tablets or capsules with food or liquids.
b. A pharmacist or other practitioner working within their scope of practice must determine that it is safe to alter a medication. If the medication is altered, documentation of the appropriateness of the alteration must be on the prescription container or in the client's record.
c. The client must be aware that he/she is receiving the medications and, if altered, that the medication has been added to their food or drink.

11. The Aide can transfer medication from one container to another for the purpose of an individual dose. Examples include Pouring a liquid medication from the medication container to a calibrated spoon or medication cup.
12. Oxygen is not considered a medication and is therefore not covered under this policy. Oxygen liter flow may only have been regulated by the client/representative, respiratory therapist, or RN with physician orders.
13. Insulin administration provided the Basic Personal Care Assistant is a nurse-delegated task only and remains under the supervision of an RN.
14. Medication assistance shall be documented on the appropriate agency form and recorded per client/representative request in the home.
15. Observations of client medication responses are documented and reported to the Supervisor and/or family as per service plan.

16. In the event that the client is exhibiting signs of distress, the Home Care Aide will initiate emergency procedures in accordance with agency policy.
17. A list of all current medications is kept in the client's record and in the home folder.
18. Medication errors by the client or when assisted by a Home Care Aide will be promptly reported to the Supervisor for follow-up and to determine if any remedial actions need to be taken, including physician notification. The

incident will be recorded on an incident report that becomes part of the

QIP database.

REFERENCES

WAC 246-335-420

NURSE DELEGATED SERVICES

Policy

Golden Years Home Health Supported Living LLC will provide Nurse Delegation services for those clients needing such services through a contracted arrangement with an agency providing Nurse Delegation services, which are consistent with the following policy established by and with the State law as may apply to Nurse Delegation. A copy of this policy is supplied to any agency or person contracted with you for Nurse Delegation purposes.

Golden Years Home Health Supported Living LLC understands that Nurse delegation services must confirm the following:

1. Delegation is only permitted for stable and predictable clients requiring specific nursing tasks that do not require clinical judgment.
2. The agency with an approved home care service category may provide nurse delegation in the following ways:
    a. The agency contracts with a registered nurse for any client needing nurse delegation.
    b. If the agency is contracted with DSHS or AAA to provide home care services to Medicaid eligible clients, DSHS or AAA is responsible for establishing contracts with registered nurse delegators for any client needing nurse delegation; and
3. The Home care agency credentialed workers must complete the core delegation training from DSHS prior to participating in the delegation process. Home care agency workers must also comply with any nurse delegation requirements specific to their Department of Health issued credential.

Purpose

To identify the role and activities of the Registered Nurse providing Nurse Delegation.

Procedure

1. A registered nurse under his or her license may perform for compensation nursing care, as that term is usually understood, to individuals with illnesses, injuries, or disabilities.

2. A registered nurse may, at or under the general direction of a licensed physician and surgeon, dentist, osteopathic physician and surgeon, naturopathic physician, optometrist, podiatric physician and surgeon, physician assistant, osteopathic physician assistant, advanced registered nurse practitioner, or midwife acting within the scope of his or her license, administer medications, treatments, tests, and inoculations, whether or not the severing or penetrating of tissues is involved and whether or not a degree of independent judgment and skill is required. Such direction must be for acts which are within the scope of registered nursing practice.
3. A registered nurse may delegate tasks of nursing care to other individuals where the registered nurse determines that it is in the best interest of the client.
4. The delegating nurse shall:

a. Determine the competency of the individual to perform the tasks;
b. Evaluate the appropriateness of the delegation;
c. Supervise the actions of the person performing the delegated task; and
d. Delegate only those tasks that are within the registered nurse's scope of practice.

5. No person may coerce a nurse into compromising client safety by requiring the nurse to delegate if the nurse determines that it is inappropriate to do so. Nurses shall not be subject to any employer reprisal or disciplinary action by the nursing care quality assurance commission for refusing to delegate tasks or refusing to provide the required training for delegation if the nurse determines delegation may compromise client safety.
6. For delegation in community-based care settings or in-home care settings, a registered nurse may delegate nursing care tasks only to registered or certified nursing assistants or home care aides certified under chapter 18.88B RCW. Simple care tasks such as blood pressure monitoring, personal care service, diabetic insulin device set up, verbal verification of insulin dosage for sight-impaired individuals, or other tasks as defined by the nursing care quality assurance commission are exempted from this requirement.
7. "Community-based care settings" includes Community residential programs for people with developmental disabilities, certified by the Department of Health social and health services under chapter 71A.12 RCW; adult family homes licensed under chapter 70.128 RCW; and assisted living facilities licensed under chapter 18.20 RCW. Community-based care settings do not include acute care or skilled nursing facilities.
8. "In-home care settings" include an individual's place of temporary or permanent residence but do not include acute care or skilled nursing facilities and do not include community-based care settings as defined in

(e) (i) of this subsection.

9. Delegation of nursing care tasks in community-based care settings and in-home care settings is only allowed for individuals who have a stable and predictable condition. "Stable and predictable condition" means a situation in which the individual's clinical and behavioral status is known and does not require the frequent presence and evaluation of a registered nurse.

10. The determination of the appropriateness of delegation of a nursing task is at the discretion of the registered nurse. Other than delegation of the administration of insulin by injection for the purpose of caring for individuals with diabetes, the administration of medications by injection, sterile procedures, and central line maintenance may never be delegated.

11. When delegating insulin injections under this section, the registered nurse delegator must instruct the individual regarding proper injection procedures and the use of insulin, demonstrate proper injection procedures, and must supervise and evaluate the individual performing the delegated task weekly during the first four weeks of delegation of insulin injections. If the registered nurse delegator determines that the individual is competent to perform the injection properly and safely, supervision and evaluation shall occur at least every ninety days thereafter.

12. The registered nurse shall verify that the nursing assistant or home care aide has completed the required nurse delegation training required in chapter 18.88A or 18.88B RCW prior to authorizing delegation.

13. Before commencing any specific nursing tasks authorized to be delegated in this section, a home care aide must be certified pursuant to chapter 18.88B RCW and must comply with RCW 18.88B.070.

14. The nurse is accountable for his or her own individual actions in the delegation process. Nurses acting within the protocols of their delegation authority are immune from liability for any action performed during their delegation duties

15. Nursing task delegation protocols are not intended to regulate the settings in which delegation may occur but are intended to ensure that nursing care services have a consistent standard of practice upon which the public and the profession may rely, and to safeguard the authority of the nurse to make independent professional decisions regarding the delegation of a task.

16. The nursing care quality assurance commission may adopt rules to implement this section.

17. Only a person licensed as a registered nurse may instruct nurses in technical subjects pertaining to nursing.

18. Only a person licensed as a registered nurse may hold herself or himself out to the public or designate herself or himself as a registered nurse.

REFERENCES

WAC 246-335-420

COMMUNICATING WITH CLIENTS

Policy

Golden Years Home Health Supported Living LLC personnel will be oriented to techniques for appropriate and effective communication with clients, families, and personnel. The agency will accept persons for services regardless of age, race, color, creed, sex, national origin, or handicap, including sensory impairment.

Purpose

To define the methods that will facilitate effective communication.

Procedure

1. Agency personnel must communicate with clients and/or families in a respectful, calm, and clear manner in a form or language they can reasonably be expected to understand.
2. If a family interpreter is unwilling or unavailable, the agency's Supervisor will take appropriate actions to enhance communication when needed by:
    a. Providing assistance in obtaining special communication devices or aids
    b. Using translated materials, interpreters, or interpreter services
    c. Referral to community agencies that can further assist communication such as:
        i. The Department of Health of Services for the Blind
        ii. The Department of Health of Services for the Deaf
        iii. Library for the Blind and Physically Handicapped
        iv. Local language bank for interpreter services including sign language.
3. The agency will assist in arranging to receive referrals from and provide services to persons with communication disorders, including hearing and speech impairments.
4. The agency will arrange for the lease of a Telecommunication Device for the Deaf (TDD) or Teletypewriter (TTY) or amplified telephone receiver from the local telephone company to facilitate agency-client communication as indicated.

Foreign Speaking Clients/Cultural Considerations

1. The agency recognizes and respects the client's rights when there is a need to facilitate effective communication in a language or form understandable to the client. As necessary, the agency will provide any required written information in the language understandable to its clients.

2. Cultural considerations for all clients shall be respected and observed. Where such considerations impede the provision of client services, personnel shall notify the Supervisor to accommodate client needs.

REFERENCES

WAC 246-335-420

WAC 246-335-435

CLIENT TRANSFER AND/OR DISCHARGE

Policy

Golden Years Home Health Supported Living LLC will transfer clients to other agencies or facilities when it is unable to meet the client's needs, or the client requests a transfer of services. A complete discharge summary with full information will be sent to assure client continuity of services. Apart from discharge for expiration while under agency services, a client may be discharged for a transfer, no longer meeting eligibility requirements for payer source, requires acute care or more service than the agency can provide, request, or for "cause." The agency provides a systematic process for effective transfer or discharge. AGENCY understands that failure to follow the applicable discharge requirements as specified in WAC 246-335-420, could lead to charges of client/patient abandonment. To minimize the possibility of client abandonment, clients must be given at least a forty-eight-hour written or verbal notice prior to discharge that will be documented in the client record. The agency understands that forty-eight-hour notice is not required if home care agency worker safety, significant client noncompliance, or client's failure to pay for services rendered are the reason(s) for the discharge. Home care agencies under contract with DSHS or the AAA may follow different time frames for notice of discharge as established in the terms of the contract. If the agency is discharging a client and is concerned about their ongoing care and safety the agency will submit a self-report to appropriate state agencies which identifies the reasons for discharge and the steps taken to mitigate safety concerns. When AGENCY is discharging a client and is concerned about the client's ongoing care and safety AGENCY may submit a self-report to appropriate state agencies in which AGENCY identifies the reasons for discharge and the steps taken to mitigate safety concerns; Specific home care services, including any nonmedical services, available to meet client or family needs are identified in the plans of care.

Purpose

1. To define the criteria for the transfer or discharge of clients.
2. To ensure continuity of services by providing needed information to client/representative, other agencies, or providers involved in the discharge or transfer, and to complete the client record for the agency services provided.

Procedure

Clients will be transferred to another community agency or facility when:

1. The skills, abilities and resources of the agency are not adequate to meet the needs of the client. The Supervisor with the client/representative will discuss options including transfer.

2. There is a change in the intensity of client's condition requiring hospitalization or custodial services for lack of an able, willing, and available caregiver. The client/representative will receive information and assistance with the concerns. The client has the choice of accepting or rejecting service. The physician or adult protective services may become involved if circumstances warrant assuring client safety.
3. The client/representative requests transfer to a different agency. The client/representative must confirm in writing the date they are requesting the transfer and the name of the agency that will be continuing the client's services.
4. The client is moving to another geographic area. The client will be given a list of the agencies in their desired area that may provide the services needed.

Transfer to Another Agency or Facility

1. The agency will confirm that the other agency or facility will admit the client.
2. Supervisor will make needed arrangements for the transfer.
3. The client record will include the date of the desired transfer, name of the new agency or facility, and date services will start. Complete transfer process per transfer policy
4. A summary of the services provided will be provided within 48 hours of request.
5. The Transfer Discharge summary may include:
    a. Client demographics, reason for transfer, referral
    b. Client's physical and psychosocial status at the time of transfer
    c. Instructions and referrals provided to the client
    d. Existence of advanced directives
    e. The current service plan and payment source
    f. Date, name, and title of the agency representative completing the form
6. When a client is placed "On-Hold" after a transfer to an inpatient facility, a transfer l summary will be required. If the client's inpatient stay becomes permanent, agency discharge paperwork must be completed.

Reason for Discharge

1. The client moves out of the area or transfers to another home care agency.
2. The client requests home care services end. The client has the right to refuse service.
3. If possible, a written note from the client about the reason for the decision to stop services should be obtained. If not, the Supervisor will document the reason. The client may at any time choose to be re-admitted servicing if their eligibility is still valid.
4. The client no longer meets the insurance eligibility requirements.

5. The client's (or another person in the home) behavior is disruptive, abusive, or uncooperative to the extent that client service or the ability of the agency to operate effectively is seriously impaired.
6. The client requires acute or more service than the agency can provide.

Discharge Planning

1. The Supervisor will inform the client/representative when considering discharge.
2. The discharge plan includes any necessary counseling, education, and referral before each client's discharge.
3. If the agency decides to discharge a client, the discharge will be explained to the client in writing with at least a three-day notice.

Prior To Discharge for Cause

1. Advise the client/representative that discharge for cause is being considered and the ramifications of discontinuing services and information about how to obtain community resources to assist the client.
2. Make a serious effort to resolve the concern with the client/representative.
3. Document in the client records concern, efforts to resolve it, and the discharge plan for client follow-up services.

Discharge Summary Content

The discharge summary will include the date and reason for discharge, client's demographic information, and service summary. All discharges will include a discharge summary.

REFERENCES

WAC 246-335-420

CLIENT EXPIRATION AT HOME

Policy

This policy is limited to in-home deaths of terminally ill clients receiving services who apparently died from the terminal illness. This policy does not include suspicious or accidental deaths that must be presorted to local police and/or the coroner about the death and the circumstances warranting coroner involvement. The pronouncement of death is a medical decision that can be made only by a qualified physician or designee. Unless precluded by local or state regulation, a licensed physician may pronounce a person dead without personally examining the body of the deceased if a licensed RN or other authorized observer has recited to the physician sufficient facts to enable the physician to have accurate information about the person's status and the physician is satisfied that death has occurred. If the client has a signed POLST form and any section of the POLST form not completed, then it is implied that there will be full treatment for that section. In the event of a client medical emergency and agency staff are present, provide emergency medical personnel with a client's signed POLST form

Purpose

1. To establish the protocol for pronouncement of death that meets the local and state guidelines and assists agency personnel in appropriate decision making.
2. To balance the privacy rights of the agency and client, and the legal duties of the coroner related to unattended deaths.

Procedure

1. If the client is terminal, the agency on admission will gather the local coroner's office and the client/representative-designated funeral director along with demographic information on the terminally ill clients residing in the coroner's district whose death in the home is a probable event. This will include a copy of the physician's DNR order.
2. If an agency employee is in the home at the time of death of a terminally ill client, they will ask the family to initiate the death in the home plan established for the client. The employee will then contact the Supervisor for instructions.
3. If the employee has any concerns about the circumstances of the death, the agency policy is to contact the local coroner's office via 911, examples may include:
   a. Family or physician request
   b. Violent, accidental, or unexpected death
   c. Suicide or suspected criminal act

4. The client's body will not be disturbed. The coroner will complete the required procedures if the death was considered suspicious or accidental.
5. In an anticipated death with no unusual circumstances, the family will notify the attending physician for further instructions
6. The agency personnel will document in the client record the expiration circumstances, agency notification, family actions taken, date and time, and any family support given. The employee may leave the home as instructed by the Supervisor.
7. If a health care professional was not in attendance at the time of death, the funeral home will be responsible for complying with the local and state requirements for death in the home and may be authorized to declare death using the following criteria:

a. Pupils fixed
b. No palpable pulses for one full minute
c. Detectable rigidity

REFERENCES

WAC 246-335-420

CLIENT RECORDS/INFORMATION MANAGEMENT

INFORMATION MANAGEMENT PLAN

Policy

Golden Years Home Health Supported Living LLC has information management needs including availability, accuracy, and timeliness of information. Systems will be in place to ensure safe and effective management of information. The agency Owner/Administrator and management team is responsible for ensuring that the required information is available in an accessible, secure, and confidential system.

Purpose

1. To define information management processes.
2. To identify the agency's plan to protect and provide information as needed to serve the clients, personnel, and agency operations.

Procedure

1. Internal communication requirements for the agency are:
    d. Performance improvement activities
    e. Infection control
    f. Incident report system
    g. Inventory
    h. Billing
    i. Client information
    j. Internal reporting (admissions by diagnosis, productivity reports, visits by disciplines, financial reports, etc.)
2. External communication requirements for the agency are:

a. Community agencies and referral sources
b. Relevant allied health providers
c. Contracted services
d. Third-party payers
e. State licensure and accrediting body regulatory agencies

3. Data collection/information management is necessary for the following reasons:

a. Client service management
b. Service coordination with other agencies
c. Direction, staffing, and material resource allocation
d. Performance improvement
e. Reimbursement
f. Liability
g. Information assessment, selection, and integration

4. Standardized data collection nomenclatures pertinent to the agency include:

a. Billing coding requirements
b. Taber's Cyclopedic Medical Dictionary
c. Unacceptable terms and abbreviations
d. HIPAA and PHI privacy rules
e. Accrediting body standards
f. Home services standards of practice

5. Sources for external database comparison include:

a. State and national private duty associations

6. State licensure/regulators and surveyors, including accrediting organizations who may access any agency information at the appropriate levels of security

7. Client records are kept in a locked location with limited access. Information stored or transferred via computers with controlled accessed by individual passwords and firewalls.

8. Original client notes are stored in a locked file cabinet. Computerized information is backed up daily to prevent inadvertent loss of vital information.

9. Information is only transferred or released from the agency with written authorization of the client/representative.

10. Information is shared between individuals involved in the current service without additional consent. Only those with a "need to know" have access to PHI.

11. Copies of certain medical information will be utilized in the client's record for continuity of services until signed originals are available. Any documents containing client information will be destroyed by shredding before disposal.

12. Data confidentiality:
    a. All client information and personnel information is strictly confidential.
    b. Confidential data may be used in an aggregate manner if the confidentiality of the client is maintained, e.g., performance improvement purposes.
    c. The staff is oriented to the agency policy on confidentiality of PHI.
    d. Data encryption occurs as required by law and regulation, e.g., for transmission.
13. Groups served by information management function include:
    a. Agency leadership, Supervisor
    b. Agency personnel and authorized contractors
    c. Clients/families/caregivers
    d. Payer/purchaser
    e. Regulatory bodies, e.g., state licensure, OSHA, CDC, etc.
    f. Accrediting bodies
    g. Physicians and relevant allied health providers
14. Support needed for planning purposes includes:
    a. Allocation of personnel time
    b. Allocation of resources
    c. Computer technology
15. National/state guidelines requiring access of information systems and sending equivalent data and information from one location to another include:
    a. Washington State regulatory agencies
    b. State licensure.
    c. Third-party payer
    d. Accrediting body organizations
16. Long-term data and information for reporting purposes include:
    a. Agency records/retention after discharge
    b. Performance improvement results
    c. Cost reports
    d. Personnel records
    e. Employee health records

f. Financial reports
g. Annual Reports
h. Business deeds, bylaws, and other agency documents
i. State, or accrediting body survey results

17. Requirements for internally and externally generated data and information to support continuous improvement in performance include:

a. Allocation of personnel time
b. Allocation of resources
c. Data comparison and interpretation
d. Computer systems

18. Enhancement of workflow activities includes:

a. Process improvement activities.
b. Staff input reviews
c. Recommended information process changes
d. Related personnel education and/or training

19. Support needed for information management systems and processes includes:

a. Human and financial resources
b. Computer technology
c. Manual systems to support lacking computer technology.
d. Staff involvement and commitment

20. Original agency records and administrative information are maintained onsite at the agency office. Should the agency establish a branch or sub-unit, the original agency records and administrative information may be transported to the main office for billing, utilization review, risk management, and performance improvement. If such transport is necessary, records will be transported in a secure lock box or by carrier (e.g., Fed Ex) with return receipt requested. Such transport will be coordinated between locations.

CLIENT RECORDS

Policy

In accordance with best practice guidelines, Golden Years Home Health Supported Living LLC maintains a record of each client consistent with chapter 70.02 RCW Medical records—Health care information access and disclosure.

Purpose

To comply with statutory law regarding the content and safeguarding of client records

Procedure

1. The supervisor of the in-home care agency ensures that all client records are:
   a. Accessible, in an integrated document, in the agency 's office site for review by appropriate direct care personnel, volunteers, contractors, and the Department of Health.
   b. Written legibly in permanent ink or retrievable by electronic means.
   c. On the agency 's standardized forms.
   d. In a legally acceptable manner.
   e. Keep confidential.
   f. Chronological in its entirety or by the service provided.
   g. Fastened together to avoid loss of record contents [if in paper]; and

The supervisor must ensure that all essential information, encompassing personal details, medical history, care plans, assessments, and pertinent communications or incidents, is thoroughly documented in the client's record within a prompt timeframe of three (3) days.;

2. The supervisor surveys all records quarterly to ensure that all client records include documentation of the following in each record:

a. Patient or client's name, age, current address, and phone number.
b. Patient's or client's consent for service, care, and treatment.
c. Payment source and patient or client responsibility for payment.
d. Initial assessment.
e. Plan of Care according to WAC 246-335-440 depending on the services provided.
f. Signed or electronically authenticated and dated notes documenting and describing services provided during each patient or client contact.
g. Supervision of home care aide and home care aide services according to WAC 246-335-445

h. Other documents may be required.

3. For clients receiving a one-time visit, provide the documentation required in WAC 246-335-440(5) in lieu of the requirements in subsection (3) of this rule and shall at a minimum contain.

a. Client name, age, current address, and phone number.
b. Confirmation that the client was provided with a written bill of rights under WAC 246-335-435;
c. Client consent for services to be provided; and
d. Documentation of services provided.

4. Consider the records as property of the agency and allow the patient or client access to his or her own record; and

5. Upon request and according to agency policy and procedure, the Supervisor will provide client information or a summary of care when the client is transferred or discharged to another agency or facility.

6. will maintain records for:

a. Adults—Three years following the date of termination of services; and
b. Minors—Three years after attaining age eighteen-, or five-years following discharge, whichever is longer.
c. Client death - Three years following the last date or termination of services if client was on services when death occurred.

7. Golden Years Home Health Supported Living LLC will:

a. Store records in a safe and secure manner to prevent loss of information and to maintain the integrity of the record and protect against unauthorized use.
b. Maintain or release records after a patient's or client's death to per RCW70.02, Medical records—Health care information access and disclosure; and
c. After ceasing operation, retain or dispose of records in a confidential manner according to the time frames in subsection (6) of 246-335-450 (7).

8. Documentation and Filing of the client records will occur on the same day as the services provided. If the documentation and filing is for an assessment and plan of care or revision, it must be completed within 48 hours.

REFERENCES

246-335-445 RCW 70.02

RECORD RETENTION AND DISPOSAL

Policy

In accordance with best practice guidelines, Golden Years Home Health Supported Living LLC records will be maintained and disposed of according to the sensitivity of the information contained in them. Records are retained and disposed of per rules established by the state.

Purpose

To comply with statutory law regarding the retention of client records.

Procedure

Permanent Records

1. Incorporation documents, including articles of incorporation, licenses, bylaws, stockholder names and percentage of ownership, and related documents.
2. Tax-exemption documents, including application for tax exemption (IRS Form 1023), IRS determination letters, and related documents.
3. Annual information returns (IRS Form 990)
4. Agency policies and procedures manuals (either hard copy or electronic)
5. Annual budget and capital expenditure plan
6. Accounts payable and receivable summary ledgers (hard copy or electronic)
7. Annual Program Evaluation
8. Annual review financial statements
9. Legal correspondence
10. Property records including deeds, mortgages, bills of sale, property appraisals by outside appraisers.
11. Depreciation schedules
12. Retirement plan documents, amendments, and summary plan descriptions
13. Insurance records, current accident reports, claims, policies, etc.
14. Check copies for important payments and purchases.
15. Personnel list and files for current employees
16. Stored human resource files.

Documents Maintained For 10 Years

1. All client records unless state law stipulates a longer period; at least 10 years following discharge or expiration date of the adult or 10 years beyond the age of majority for a minor.
2. Records involved in litigation are retained until after settlement.
3. Personnel files for terminated personnel.

Documents Maintained for Seven Years

1. Bank statements, reconciliations, canceled checks, duplicate deposit slips, check registers, investment statements.
2. General ledger and journals
3. Payroll records and summaries
4. Correspondence-acknowledgement of gifts and bequests
5. Expired contracts and leases
6. Timesheets and withholding tax statements.
7. Conflict of interest disclosure information
8. Press releases.
9. Performance improvement records
10. Workers' compensation claims and settlements

Documents Maintained for Five Years

1. Financial records, including billing files shall be retained for a period of five years following the month on which the cost report for the records is filed with the fiscal intermediary or as determined by the insurer, state law.
2. Governing Body meeting agendas, minutes, membership, and related documents
3. membership, meeting agendas, and minutes
4. Minutes and attendees from all administrative and committee meetings
5. QIP reports, including occurrences, complaints, record audits, and continuing education rosters.
6. Accrediting body and state survey reports

Documents Maintained for Three Years

1. Training records
2. Client Record Review membership for HIPAA compliance
3. Calendars
4. Internally prepared reports and financial statements

5. Expired insurance policies
6. General correspondence
7. Agency demographic statistics

Documents Maintained for One Year

1. Employment applications and resumes
2. Current client roster

Storing Documents

1. All records are confidential and protected from damage and unauthorized use.
2. Documents will be stored in a way that keeps them reasonably safe from access by unauthorized individuals as well as from theft and physical damage caused by fire, flood, insects, rodents, temperature, and humidity.
3. Copies of tax exemption documents and annual information returns are kept and made available for public inspection upon request.
4. Electronic storage may be used as a cost-effective way to maintain documents. The system reliably transfers, stores, indexes, preserves, and retrieves the documents. It allows them to be converted to legible paper copies, has reasonable controls to ensure the accuracy of the records, and not subject to restrictions that would inappropriately limit access to the records.

Disposal

1. Management may dispose of records that no longer need to be kept by shredding, recycling, or waste management procedures depending upon the sensitivity of the information contained in them.
2. An authorized employee will oversee any disposal of records.

CLIENT PRIVACY AND SECURITY

Policy

Golden Years Home Health Supported Living LLC will establish guidelines for personnel treating clients with respect and dignity by providing personal privacy and security while an agency employee is providing service in the home and protecting client information while outside the home. The specific employee guidelines for PHI are included in the HIPAA policy.

Purpose

1. To provide a mechanism to address client privacy and security during service.
2. To identify individuals who will have access to client information and clarify procedures to ensure this information is maintained in a confidential manner.

Procedure

Privacy

1. Personnel are encouraged to keep the client covered at all times and to provide a drape when directly working with the client.
2. Visitors are asked to leave when services are being provided unless the client asks them to stay.
3. Any photos taken are for the purpose of documentation and are taken only with the client/representative's written consent.
4. The agency will ensure client record and data entry personnel are trained to maintain client records and files confidentially.
5. Client information will not be visible on computer monitors when not in use.
6. Client records in the home will be labeled as confidential and kept in a secure place by the client/representative.
7. Information given to personnel will be limited to a "need to know" basis.
8. All personnel will be oriented to the privacy requirements on hire and annually, and the consequences of client privacy violations up to and including termination.

Security

1. Clients and family are encouraged to keep emergency telephone numbers visible and near the telephone.
2. Clients are instructed to notify the agency if property, money, or valuables are missing or damaged.

3. The agency recommends that clients notify police and household insurance carriers in the event of loss or theft. The agency will fully cooperate with the investigation. In the event an admission of theft is made; every attempt will be made to recover the loss from the employee involved. A guilty employee's employment with the agency will be terminated. In the event the agency is to blame regarding property loss or damage, every effort will be made to return or replace property.
4. All information maintained in computer records will be password protected and only authorized personnel will have access. All personnel having passwords to access the agency and client information will keep the password protected and not allow others to know the password or use it.
5. Client records and PHI will be maintained in agency records and on computers as needed. All personnel with access to client information will be required to log in using their personal password when accessing client records. Personnel will have access to only those records needed to complete their work. Passwords will be changed periodically when an employee resigns, or job duties are reassigned. Sharing passwords with others is not allowed and will be treated as a breach in confidentiality resulting in corrective action.
6. If mobile devices are used by field personnel, the agency will enforce the following HITECH ACT controls on mobile devices used to document client service by personnel, contractors, and business associates:
    a. The agency software with PHI access is encrypted.
    b. User authentication and passwords will be required.
    c. The agency has activated wiping/and or remote disabling.
    d. The agency has an active firewall and security system.
    e. The agency has disabled file-sharing applications.
    f. The agency prohibits downloading of mobile applications without permission.
    g. Staff will be required to follow security measures if sending or receiving PHI over public Wi-Fi networks.
    h. Personnel are required to maintain physical control over their assigned mobile devices and store them in a secure and protected location when not in use.
    i. Agency-owned mobile devices may not be used for personal use.
    j. The mobile device must be returned to the agency for deletion of all PHI before discarding or reusing the device.

REFERENCES

WAC 246-335-435

PERFORMANCE IMPROVEMENT

QUALITY IMPROVEMENT PROGRAM

Policy

1. Golden Years Home Health Supported Living LLC has developed and operationalized a quality improvement program to ensure the quality of care and services provided throughout all approved service areas including, at a minimum:
   - k. A complaint process that includes a procedure for the receipt, investigation, and disposition of complaints regarding services provided.
   - l. A method to identify, monitor, evaluate, and correct problems identified by clients, families, personnel, contractors, or volunteers; and
   - m. A system to assess client satisfaction with the overall services provided by the agency.
2. The Owner/Administrator assigns the Supervisor and/or QIP Committee with these responsibilities. Although the agency uses the Centers Medicaid Services (CMS) Quality Assurance Performance Improvement (QAPI) guidelines as a resource for QIP activities and processes.
3. The QIP committee meets quarterly and reviews the issues under correction. If resolved, the committee will then write a plan for an additional issue to be resolved over the next quarter.

Purpose

1. To ensure the delivery of quality services to our client/families.
2. To improve the agency's performance by designing its processes well, to systematically collect data, monitor, aggregate, analyze, and improve client outcomes by comparing itself to other standards and best practices.

Procedure

1. The agency's QIP may consist of:
   - a. Client satisfaction with services [accomplished by survey at least bi yearly and at discharge]
   - b. Client complaints and concerns [evaluated each quarter]
   - c. Client centered outcome- based activities
   - d. Program/personnel performance assessment activities.

e. Staff QIP training and participation
f. Staff recruitment, training, orientation, and continuing education programs
g. Case conferences
h. Management meetings
i. Client record reviews
j. Personnel competency testing and peer review activities.
k. Staff turnover rates, including employee retention and satisfaction.
l. Safety issues
m. Equipment maintenance
n. Management systems that support infection control functions
o. Performance improvement evaluation and plans
p. Risk management program.
q. Annual program evaluation
r. Staff development programs
s. Standards of practice compliance

2. Objectives of the QIP are to maintain and improve the highest quality client service and reduce or eliminate hazards in the client's home by:

a. Managing the QIP to ensure all performance improvement activities are implemented.
b. Identifying opportunities to improve client service using collection, screening, and evaluating of service outcomes and client satisfaction information.
c. Tracking identified problems to ensure improvement or resolution.
d. Developing effective performance improvement mechanisms such as incident reporting and trending and client/physician questionnaires.
e. Assessing process for addressing client/representative complaints.
f. Documenting the findings, recommendations, actions taken, and results of actions using a defined statistical process.
g. Evaluating trends of performance problems
h. Improving employee communication when client problems or opportunities arise.
i. Assuring performance-based credentialing and competency for service providers.
j. Identifying personnel whose performance requires focused training or termination.

k. Evaluating, at least annually, the scope, organization, and effectiveness of the QIP ensuring that actions taken are effective in meeting the program objectives and within the mission and goals of the agency. This evaluation is part of the Annual Program Evaluation and submitted to the Owner for review.
l. Identifying actual or potential client service problems.
m. Identifying the need for revisions in client services, policies, and procedures.
n. Evaluating the adequacy of documentation of client services.
o. Identifying the extent to which the agency program is adequate, effective, appropriate, and efficient in the use of all personnel and financial resources.
p. Developing a structure for the investigation of target client outcomes.
q. Identifying outcomes to benchmark from external sources.
r. Developing best practices based on objective findings and developing action plans for implementation of best practices.
s. Monitoring effectiveness of best practice action plan and plan revision, if needed.

3. The Supervisor/Compliance Officer is appointed by the Owner/Administrator and manages and coordinates performance improvement activities of the agency. They may have these functions added to their job responsibilities:

a. The development and implementation of client satisfaction surveys.
b. The development and implementation of a client complaint process.
c. The development and implementation of an incident reporting process.
d. The development and implementation of an employee satisfaction survey.
e. The development and implementation of a referral satisfaction survey.
f. Identification of/and proposed procedures for dealing with staffing problems.
g. Data collection processes and forms for development of the QIP.
h. Evaluation of data and development of performance improvement plans for areas needing attention.
i. Compliance management.

4. The QIP plan includes data from, but not limited to, the following:

a. Client satisfaction surveys
b. Staff satisfaction surveys
c. Client/employee incident reports
d. Referral satisfaction surveys
e. Communicable disease report log

f. Complaint log
g. Quarterly client record reviews
h. Quarterly personnel record reviews
i. Annual program evaluations
j. Financial statements

5. Quarterly, the data from each of the above items will be reviewed, aggregated, and analyzed by the Supervisor and/or QIP committee.Problems will be identified, and a plan of correction will be developed and implemented.
6. The Owner/Administrator will assume responsibility for continuous monitoring of plans for improvement and correction. The Owner/Administrator will modify policies and procedures as indicated by performance improvement review outcomes.
7. Annually, the Owner/Administrator will review, and the Supervisor will present a summary to the Owner of:

a. All quality assurance findings
b. The QIP and corrections implemented.
c. Annual Program Evaluation
d. The results of the corrections and the need for any further corrections

8. The Owner supports and participates in the QIP by allocating adequate resources to it, suggesting areas for QIP focus, and reviewing and commenting on QIP reports. The agency will also consider the following functions as important to the delivery of quality services:

a. Personnel selection
b. Personnel supervision
c. Orientation
d. In-service education

9. QIP records will be maintained for a minimum of three years.

REFERENCES

WAC 246-335-455 WAC 246-335-415

QUALITY IMPROVEMENT PROGRAM MONITORING

Policy

Responsibility for the Golden Years Home Health Supported Living LLC 's QIP ultimately rests with the Owner/ Owner/Administrator who assigns these responsibilities.

Purpose

To delineate the lines of authority and report on the quality of agency services.

Procedure

1. QIP reporting flows upward from the personnel to the Owner/Owner/Administrator, for Policy and procedural changes recommended for revision along with other policies and procedures adopted by the Owner/Owner/Administrator flow downward in reverse order.
2. The agency has an identified line of authority and responsibilities for performance improvement. They include:

a. Identifies and adopts a specific approach to performance improvement.
b. Authorizes adequate resources and support to establish and maintain an agency wide QIP.
c. Assumes ultimate responsibility for the provision of quality service.
d. Appoints in writing management staff who oversees the QIP.
e. At least annually, receives reports on all performance improvement activities, including the assessment of the effectiveness of QIP activities in identifying the quality of service, opportunities to improve client services, appropriate use of resources, and personnel competency.

Owner/Administrator

1. Oversees the QIP process.
2. Assigns the QIP coordination responsibilities and confers on a regular basis with the Supervisor and other personnel to review trends and assess problems.
3. Immediately informs the Owner of adverse events, including root cause analysis, actions taken, required follow-up, and monitoring measures.
4. Ensures systems are in place to support infection control, safety management, and other resource management functions.
5. Assists the Owner and other managers in setting agency priorities for all performance improvement activities.

Owner/Administrator

1. Establishes and maintains communication among various individuals, committees, and boards responsible for QIP activities and develops, presents, and evaluates QIP education.
2. Ensure that all appropriate QIP functions are accomplished within established time frames.
3. Notifies all members of scheduled meetings, develops the agenda, and serves as chair/recorder.
4. Collects, aggregates, analyzes, and reports data to the committee.
5. Serves as a resource and provides personnel education. QIP Team Members

1. Membership consists of at least the following:
    f. Supervisor
    g. Operational scope of service representatives

Supervisor and Assigned QIP Team Duties

1. Monitors compliance with performance improvement activities according to agency policies and procedures and, if indicated, accreditation guidelines.
2. Analyzes at least quarterly QIP reports (for the most current past 12-month period available) to identify trends in service, satisfaction surveys, record reviews, employee performance, financial operations report, adverse events, and incident reports, then utilize information to identify and prioritize performance improvement activities.
3. Develop best practices based on findings.
4. Integrates, coordinates, and monitors performance improvement activities.
5. Reviews and approves minutes of its meetings.
6. Maintains a performance improvement manual containing required documentation of specific assessment actions, applicable policies and procedures, and the annual calendar of performance improvement activities.

REFERENCES

WAC246-335-415 WAC246-335-455

ANNUAL PROGRAM EVALUATION

Policy

Golden Years Home Health Supported Living LLC will conduct an annual program evaluation summarizing all service and administrative operation components of the agency to analyze the effectiveness, appropriateness, quality, adequacy, and efficiency of the agency's performance. The evaluation will be an organized, effective, and documented evaluation of overall agency functioning conducted at least annually by a group that includes at least the Owner/Administrator, the supervisor and another health care staff employed by the agency. The documented evaluation will serve as a guide to revise and improve current programs with a plan for correcting deficiencies within specific time frames, for development of new programs as indicated by identified needs of the communities served by the agency, and to ensure confidentiality, integrity, and availability of PHI. As a new agency, the first annual program evaluation will be completed in the first quarter following the first full year of operation. Thereafter, the annual program evaluation will be due after the completion of the first quarter following the close of each fiscal year.

Purpose

1. To quantify and evaluate data from client and administrative operations as part of the performance improvement process for Owner review of trends and recommendations.
2. To provide a process to ensure the services and systems of the agency are reviewed and summarized at least yearly.

Procedure

Goals

The goals of the Annual Program Evaluation are:

1. To assess and improve the quality of current programs and services.
2. To ensure that programs are relevant to community needs.
3. To maintain accountability for programs and services
4. To demonstrate appropriate utilization of resources within the agency and community
5. To monitor the relevance of the program to our mission
6. To ensure confidentiality, integrity, and availability of protected health information
7. To ensure the effectiveness of the security of electronic protected health information

Scope

The scope of the agency's annual program evaluation will address the following:

1. Administrative performance, policies, and procedures on the adequacy of management and supervision to provide support to all staff through person-to-person, Internet, or telecommunications.
2. Staffing and personnel policies and procedures
3. Programs and services
4. Achievement of goals (recorded in previous year's strategic plan)
5. QIP summary report of data analysis, action plan, implementation, and evaluation
6. Equipment and supplies
7. Human resources functions include personnel evaluations.
8. Safety practices include the infection control program.
9. Risk management.
10. Financial data collection and processing
11. Information systems
12. Accessibility of agency policies and procedures manuals
13. Organizational structure
14. The total client unduplicated census, including client demographics such as age, primary diagnoses, geographic service area and distribution, length of service, and outcomes.
15. Clinical Record Review Annual Summary
16. New services will be evaluated after the first year of activity.
17. Annual policies and procedures manual review for needed revisions, deletions, or additions.
18. Supervisor and Supervisor will appoint and document review team members including involvement.
19. The numbers, qualifications, experience, and current competence of the individuals providing each service.
20. The numbers and types of visits conducted.
21. Any services provided which require specialized training.
22. Client, employee, and referral source service satisfaction survey results Resources

1. Adverse event report
2. Infection control report
3. Incident reports
4. Complaint reports

5. Client and referral source satisfaction reports
6. State and accreditation survey recommendations.
7. Financial reports
8. Human resource data
9. QIP data

Responsibility

1. The Owner/Administrator will assign a responsible manager for evaluating the program services within their scope of responsibility under the Supervisor's direction.
2. The following agency representatives are involved in the process:
    h. Administration
    i. Personnel
    j. Supervisor
    k. Office Manager/Support Staff

Format

The Annual Program Evaluation will be presented to the Owner for review and comments. The results of the review will be summarized, documented, and presented in its final format with the following items:

1. Those involved in its preparation.
2. Specific findings or results
3. Corrective action
4. Recommendations for further improvement
5. Identification of new services needed
6. Disposition of specific findings and recommendations

REFERENCES

WAC 246-335-455

CLIENT SATISFACTION SURVEY

Policy

As part of Golden Years Home Health Supported Living LLC's QIP, services are evaluated based on client survey results. The internal survey process utilizes a manual or homecare software client satisfaction survey application to assess client satisfaction with agency services and personnel for performance improvement suggestions.

Purpose

1. To assess client satisfaction with agency services.
2. To provide quality assurance monitoring, address client concerns, and improve agency services.

Procedure

1. The Supervisor will:
    l. Review the client/employee complaint log for the last month, analyze trends, and develop the data for the Client Satisfaction report. Complaints are handled as per the Client/Staff Complaints and Grievance Policy.
    m. Each client will be mailed a client satisfaction survey at the end of the first 30 days of service, again at 90 days of service and again at one year, and annually thereafter, if the client remains on service.
    n. At least monthly, data will be summarized for inclusion in the report.
    o. No less than quarterly, a summary report will be prepared for all findings and provided to the Owner/Administrator.
    p. An annual summary of results will be included in the Annual Program Evaluation.
2. The Owner/Administrator will:

a. Review the summary findings of the data.
b. Work with the Supervisor to ensure all issues are addressed in QIP and reviewed by members.
c. Prepare the report of findings for inclusion each quarter report.
d. Monitor identified problems or issues for improvement or further planning and implementation for improvement.
e. Incorporate input from the Owner into the Annual Program Evaluation and QIP.

REFERENCES

WAC 246-335-455

CLIENT/FAMILY COMPLAINTS AND GRIEVANCES

Policy

In its efforts to maintain high client satisfaction, Golden Years Home Health Supported Living LLC responds appropriately to all complaints by clients and their families, and will maintain an accurate, dated complaint log defining the issue and documenting actions taken to resolve the concern.

Purpose

1. To address individual client or family complaints and grievances.
2. To provide a means for early identification and resolution of problems.

Procedure

1. Clients and/or family will be informed verbally and in writing of the agency's complaints and grievance policies and procedures at the time of admission for Home care services. Anyone may lodge a grievance or complaint.
2. The complaint process will include a procedure for the receipt, investigation, and disposition of complaints regarding services provided under RCW 70.127.120(2); and a method to identify, monitor, evaluate, and correct problems identified by patients or clients, families, personnel, contractors, or volunteers;
3. The Supervisor/designee investigates the complaints made by a client, the client's family, or guardian regarding treatment or services that is (or fails to be) furnished, or regarding the lack of respect for the client's property or person by anyone furnishing services on behalf of the agency, and any other issue that may arise.

a. The Supervisor will begin an investigation of the complaint within 48 hours and will work diligently in a timely manner to complete the investigation.
b. The Supervisor will inform the complainant of the anticipated time for the investigation and will follow-up as necessary. The Supervisor will request that the complainant allow the agency to work on an acceptable resolution, but that the complainant has the right to contact the state or accrediting body hotlines and their hours of operation, should they so desire. The phone numbers are found in the client admission folder but will be provided again should the complainant request.
c. The agency may request that the complaint or grievance be submitted in writing to facilitate a clear understanding of the issue from the complainant's perspective.

4. A written response notice is provided to the complainant within 30 days of the complaint/grievance resolution.

5. If the complaint is still unresolved, the complainant may request, in writing, that the Supervisor submit the complaint to Owner/Administrator.
6. If the complaint remains unresolved, the complainant will be advised in writing of the right to file the complaint with the state or accreditation authority.
7. The Supervisor maintains both the complaint and its resolution in an administrative QIP file. Complaint and grievance data is included in the QIP program to evaluate trends and design a plan for correction for any identified process flaw.
8. A system to assess patient or client satisfaction is in place. See policy client satisfaction

REFERENCES

WAC 246-335-435 WAC 246-335-455

MANDATORY REPORTING OF ABUSE AND NEGLECT

Definitions

Mandatory reporter " means an administrator, authorizing practitioner, director of clinical services, health care professional, home care aide, home care aide, medical director, licensed nurse, social worker, supervisor of direct care services, therapist, therapy assistant, volunteer, or other individuals associated with an in-home services agency.

Physical Abuse: Any physical contact, act of negligence or act of omission, which is harmful to or jeopardizes the physical safety and welfare of the client. Physical abuse may include, but is not limited to kicking, squeezing, pinching, slapping, striking with or without an object, pushing and the use of excessive force or isolation when restraining an agitated client. Physical abuse includes not allowing clients to eat, drink, or service for physical needs such as elimination.

Emotional/Verbal Abuse: Actions, either overt or subtle, which may cause mental injury or emotional distress to the client. Emotional and/or verbal abuse would include, but is not limited to verbal teasing, humiliation, belittling or ridiculing a client; frequent or consistent use of profanities and/or obscenities in the presence of a client; use of derogatory names for a client; withholding of meals or withdrawal of privileges; making a client the “scapegoat” for any reason.

Failure to Provide Service: Lack of service by omission or neglect for the physical needs of dependent clients.

This includes but is not limited to:

a. Leaving incontinent clients in their own excrement for extended periods of time,
b. Intentional lack of attention to basic needs, such as food, clothing, and social interaction,
c. Failure to report health problems or changes in health conditions to the proper health care providers.

Sexual Abuse: Any sexual contact between personnel and client, physical or psychological, whether solicited or unsolicited, whether consented to or not. Thisincludes the following:

1) intimate caressing or fondling, exposure, intercourse, and any overtly sexual act;
2) permitting, encouraging, or forcing a client to participate in sexually explicit conduct knowing that the conduct will be photographed or be part of a live performance;

3) showing, selling, or otherwise distributing pornographic materials to a client. Verbal Sexual Abuse: Sexually suggestive remarks whether made to the client, about the client, or in the client's hearing, and the inappropriate sharing of sexual stories between personnel and clients or their families.

Policy

Golden Years Home Health Supported Living LLC complies with Washington state laws according to chapter 74.34 RCW requires all suspected abuse or neglect of children, dependents, or vulnerable adults to be reported to appropriate authorities to prevent further abuse and to safeguard the client.

All personnel must be alert for any signs of abuse and neglect of their clients. Any employee who has reasonable cause to believe that a child, dependent, or vulnerable adult has suffered abuse, exploitation, neglect, abandonment, or is otherwise in need of protective services must report the information immediately to the appropriate agency, CPS/APS online reporting or; the police if there is physical harm and or call 911 if it is an emergency case. The alert will then be reported to the Clinical Supervisor or designee.

A Mandatory reporter is an administrator, authorizing practitioner, director of clinical services, health care professional, home care aide, home care aide, medical director, licensed nurse, social worker, supervisor of direct care services, therapist, therapy assistant, volunteer, or other individuals associated with an in-home services agency.

Mandatory reporters shall report failure to comply with the requirements of chapters 246-335 WAC and 70.127 RCW to the Department of Health. The report must be submitted to Department of Health forms. Reports must be submitted within fourteen calendar days after the reporting person has knowledge of noncompliance that must be reported; Mandatory reporters shall report suspected abandonment, abuse, financial exploitation, or neglect of a person in violation of RCW 74.34.020 or 26.44.030 to the Department of Health of social and health services and the proper law enforcement agency. Reports must be submitted immediately when the reporting person has reasonable cause to believe that abandonment, abuse, financial exploitation, or neglect of a vulnerable adult has occurred;

Purpose

1. To protect the safety of the agency client.
2. To ensure that signs of abuse, neglect, and exploitation of children, dependent or vulnerable adults are recognized and properly reported.
3. To ensure compliance with state laws on abuse and neglect.
4. To inform personnel of their legal and ethical obligation to report abusive situations.

Procedure

1. All personnel providing home care services shall immediately report any incidents of suspected abuse or neglect to the Adult Protective Services (APS) at 1-877-734-6277, as mandated by Washington State law. If uncertain about the reporting process, personnel may contact the Director of Clinical Services for guidance. The Director will assist in ensuring the report is complete and submitted to the appropriate State or county agency. Additionally, personnel are required to make a written incident report, detailing the date, time, and chronological order of the information. Personnel may also contact END-HARM at 1-866-363-4276 if abuse or neglect of a vulnerable adult or child is suspected.
2. In cases where evidence of serious injury is observed, all personnel must promptly contact the Director of Clinical Services for further assistance and guidance on the best course of action to support the client.
3. In the event that a client is in imminent danger or requires urgent medical attention due to suspected abuse or neglect, personnel must prioritize the safety and well-being of the client above all else. In such cases, it is imperative to immediately call 911 to ensure swift intervention and assistance from law enforcement or medical professionals.
4. Providing comprehensive training for all staff members on mandatory reporting procedures and recognizing signs of abuse or neglect is essential. Proper training will empower personnel to identify potential instances of abuse or neglect promptly and take appropriate action to safeguard the welfare of vulnerable adults and children under their care. Regular training sessions should be conducted to ensure staff members are equipped with the necessary knowledge and skills to fulfill their reporting obligations effectively.

REFERENCES

WAC 246-335-425 WAC 246-335-335

Mandatory Reporter Training Topics | DSHS (wa.gov)

CLIENT/EMPLOYEE INCIDENT REPORTING

Policy

All accidents, incidents, or unusual occurrences involving clients or personnel will be reported immediately to the Owner/Administrator/designee of

Purpose

1. To facilitate the timely reporting and documentation of unexpected events.
2. To ensure monitoring of injuries.
3. To facilitate the factual collection of details of incidents involving clients or personnel.
4. To investigate and develop a performance improvement plan to reduce the future risk of injury to personnel and clients.

Procedure

1. An employee shall immediately report any incidents of suspected abuse or neglect to the state hot line for reporting abuse and neglect and then the direct supervisor who will assist in the documenting the abuse or neglect to ensure the report is complete and submitted to the jurisdictional state or county agency.
2. The employee will make a written incident report, including the date, time, and chronological order of the information. An incident may consist of, but not be limited to:
   a. Physical injury to either the client or an employee
   b. Misuse or abuse of medications by the client/representative or employee
   c. Threats of suicide
   d. Child or elder abuse
   e. Allegations of theft by an employee
   f. Breakage or damage of a client's property by an employee
   g. Suspicious death in the home
   h. Motor vehicle accidents
   i. Client administered medication errors
   j. Equipment or medical device malfunctions or failure
   k. Substantial safety hazards
   l. Witnessed or non-witnessed client fall
   m. Adverse event
3. Emergency services for an injured person should be initiated immediately, and the Supervisor notified us to determine what additional services should be initiated.

4. All client or employee accidents, injuries, or incidents are documented on the incident report form, within 24 hours of occurrence by the employee attending the client at the time of the incident and submitted to the Supervisor.
5. The Owner/Administrator should be notified immediately of any serious client or employee injury or occurrence. The Supervisor will be responsible for reviewing the incident/occurrence reports, determining the causal factors that contributed to the incident/occurrence, and will prepare a summary written report of his/her findings, which includes suggestions for actions to correct or prevent a similar occurrence.
6. The Supervisor will include the incident report in the QIP for review.
7. Performance improvement recommendations may include, but are not limited to, revisions to company policies, procedures and operating practices, personnel education, training and supervision implemented in a timely manner, and employee disciplinary action.
8. A corrective action plan will be reviewed and approved by the Owner/Administrator prior to implementation.
9. Guidelines for processing incident reports include the following:

a. Avoid copying an incident report.
b. Do not allow any unauthorized person, including state agencies and/or attorneys, to have access to incident reports without proper authorization.
c. Do not leave sensitive information in areas of public access.
d. Do not allow sensitive information to be distributed or placed in committee minutes.

10. Completed incident report forms will be kept in administrative files for seven years.
11. The agency's Supervisor will document all employee injuries and log in the OSHA 300 report, which is to be posted February 1 to April 30 each year.

INFECTION CONTROL

INFECTION CONTROL PRACTICES, COMMUNICABLE DISEASE TESTING, VACCINATIONS AND EXPOSURE CONTROL PLAN

Policy

Golden Years Home Health Supported Living LLC maintains policies and procedures for the service of clients with infectious and/or contagious diseases and for infection control practices by personnel that conform to OSHA/WISHA regulations and currently accepted standards of service. Agency personnel will protect the clients and themselves using universal precautions promulgated by CDC. Infection control practices, communicable disease testing, and vaccinations policies and procedures include, at minimum:

1. Standard precautions such as hand hygiene, respiratory hygiene and cough etiquette, and personal protective equipment;
2. Availability of personal protective equipment and other equipment necessary to implement client plans of care;
3. Tuberculosis (TB) infection control program. [See TB P&P} Key elements include, but are not limited to:
    a. Conducting a TB risk assessment for all new employees upon hire. Agencies must use a tuberculosis risk assessment form provided by the Department of Health. Based on risk assessment results, determine the agency's responsibility to conduct TB testing of new employees. If TB testing is required, follow the Department of Health's tuberculosis risk assessment form testing recommendations;
    b. Conducting an annual assessment of new TB risk factors for all employees. Agencies must use a tuberculosis risk assessment form provided by the Department of Health. Based on risk assessment results, determine agency's responsibility to conduct TB testing of employees. Retesting should only be done for persons who previously tested negative and have new risk factors since the last assessment; and
    c. Ensuring workers receive TB related training and education at the time of hire or during new employee orientation. Training and education must be consistent with the Department of Health tuberculosis program online posted educational materials.
4. Actions to take to report infectious diseases to the Department of Health using the list of notifiable conditions: https://doh.wa.gov/public-healthhealthcare-providers/notifiable-conditio ns/list-notifiable-conditions

5. Identified actions to take when personnel, volunteers, contractors, or clients exhibit or report symptoms of a communicable disease in an infectious stage in accordance with chapters 246-100 and 246-101 WAC; and will report and act accordingly.
6. Exposure to blood borne pathogens such as Hepatitis B and HIV and other potentially infectious materials in compliance with the Department of Health, Labor and Industries, chapter 296-823 WAC. Key elements include, but are not limited to:

a. Conducting an initial risk assessment of the environment in which personnel, volunteers, and contractors perform their assigned duties to determine occupational exposure. The results of the risk assessment will inform policy and procedure development and level of employee training and education. Annually, agencies must determine if significant changes have occurred that would require a new risk assessment to be performed;
b. If the risk assessment concludes that workers have a reasonably anticipated risk of occupational exposure to blood and other potentially infectious materials, agencies must offer workers the Hepatitis B vaccine series at the agency's expense. Workers have the right to decline the Hepatitis B vaccine series; and

7. The agency will document the annual review of applicable state health authority recommendations related to infection control practices, communicable disease testing, and vaccinations and update training and policies and procedures as necessary.

Purpose

1. To protect individuals from transmission of communicable/infectious diseases through infection control measures.
2. To maintain written policies and procedures based on infection control practices related to the services provided. These policies and procedures will be reviewed at least annually for required and/or appropriate revisions.
3. To educate personnel in the prevention and control of infections on hire and at least annually thereafter or as needed per change in policies. Clients, family members, and/or caregivers will receive education regarding basic infection control measures on admission and as needed during the delivery of services.
4. To maintain a surveillance system to track the occurrences and transmission of infections related to the services provided with reporting of data quarterly.
5. To comply with current applicable local, state, and regulatory body

regulations, including OSHA and CDC guidelines.

Procedure

Exposure Control Plan

1. If a client is suspected or known to have an infectious or contagious disease, agency personnel will implement the procedures specific to the suspected disease.
2. In addition, personnel shall implement infection control procedures with regard to other clients, personnel, and their environment.
3. The agency will provide all client care personnel with appropriate protective equipment which may include but not limited to gloves, gowns or aprons.
4. If a client develops a condition requiring quarantine or other aggressive actions, the agency will refer the client to a recommended facility suitable for the care or quarantine. This includes but is not limited to calling 911 in case of emergency.

General Infection Control

1. Client infection control procedures include, but are not limited to:
   a. Wearing and changing gloves as necessary during the delivery of client service
   b. Appropriate handling and disposal of waste products
   c. Frequent hand washing by agency client service providers
      i. Before and after the provision of direct and indirect client services
      ii. Before working in the kitchen
      iii. After handling soiled or contaminated materials iv.After going to the toilet v. After removing gloves
   d. Covering nose and mouth when coughing or sneezing
   e. Covering open sores or cuts on fingers or hands with clean bandages or gloves
   f. Use of protective equipment including gloves, gowns and masks when indicated
2. Environmental infection control procedures include, but are not limited to:

a. Maintaining a clean work environment, e.g., clean counters, tables, and shelves where food is stored
b. Refrigerating food promptly, covering food, closing cartons, and replacing lids
c. Rinsing cans and bottles before disposal in the garbage
d. Using assistive means to pick up broken glass
e. Washing garbage cans, dirty pails, and trash cans with hot soapy water
f. Disposing of garbage properly by draining off liquid before putting garbage in paper or plastic lined pails
g. Keeping clean and dirty items separate
h. Keeping the client environment, especially the bathroom, clean, neat and orderly, and keeping supplies off the floor and out of reach of children.

i. Regularly cleaning client equipment, such as commodes, bedpans, urinals, suction machines and measuring containers.

j. Decontamination of equipment prior to servicing or shipping and properly labeling equipment as clean or contaminated.

k. Using protective coverings such as aluminum foil and plastic and replacing coverings if contamination is thought to have occurred.

3. To comply with OSHA/WSHA requirements and the agency's QIP, policies, and procedures regarding infection control and exposure, the agency will:

a. Periodically evaluate all job responsibilities for potential risk.

b. Review the program's effectiveness at least annually and revise as necessary.

c. Evaluate exposure incidents and revise policies and procedures as indicated.

d. Educate personnel in the infection control program and any subsequent revisions upon employment when changes occur and at least annually.

e. Monitor compliance with the program through on-site supervisory visits, performance evaluations, and in-service training requirements.

f. Monitor and evaluate the performance of any individual or provider contracted to perform any portion of the program such as waste disposal, transportation services for lab specimens, and cleaning companies.

4. The agency will maintain procedures for service to clients with infectious and/or contagious diseases.

5. Personnel providing client services may require periodic disease screening.

6. Personnel with a known or suspected infectious and/or contagious disease will be restricted from providing client services until a statement is received from the employee's physician stating that the employee is free from contagious diseases and is able to return to work.

7. will report to authorities those communicable diseases required by state regulations.

8. The agency will comply with state requirements for reporting communicable disease.

9. Personnel management activities that can minimize infectious risks, include but are not limited to:

a. Verification of employee qualifications

b. Verification of work experience and reference

c. New employee orientation

d. In-service/continuing education program
e. Employee supervision, especially when caring for clients with a known infection
f. Employee performance evaluation
g. Employee counseling/discipline

10. All personnel will, at orientation and annually thereafter, receive training regarding the agency's infection control procedures including: use of universal precautions, blood borne pathogens, Hepatitis B virus, Tuberculosis, preventative practices, waste disposal procedures, handling and cleaning procedures, communicable disease reporting, reporting of exposure, and post-exposure procedures.
11. Disposable gloves will be worn when handling or cleaning objects, surfaces or materials that are contaminated with body fluids.
12. Disposable gloves and CPR masks will be provided to each caregiving employee by the agency for emergency use. The agency does not supply gloves for the clients, only for use by the agency personnel. A few gloves may be left in the home in the event that the client's supply is gone or there is an emergency need that presents.
13. Any surfaces contaminated with body fluids must be cleaned with a disinfectant solution made of one part of household bleach to 10 parts of water. Clean the surface with soap and water before it is disinfected with bleach solution to avoid the release of noxious fumes.
14. Contaminated water from cleaning should be flushed down the toilet.
15. Body fluids sponged or wiped up should also be flushed down the toilet, not the kitchen or bathrooms sink.
16. Sponges or kitchen towels used to clean counters and dishes should never be used to clean the floor or to clean bathroom spills.
17. Mops or articles used to clean body fluids should be washed and soaked using the above disinfectant solution, do not rinse in the kitchen sink.
18. Do not clean and disinfect these articles in areas where food preparation and cooking occur.
19. When cleaning equipment used by the client, use the disinfectant bleach solution described above unless otherwise directed by the client or family.
20. Soiled clothing, towels, linens, etc., should be washed in water with a

detergent and bleach (if washing colored articles, use bleach safe for

colored fabrics).

21. When handling clothing or articles soaked with body fluids, wear gloves and wash the clothing separately from other items.

22. If any personnel suspects exposure to pathogens during client services (i.e., exposure to blood or body fluids), the employee will immediately notify the supervisor and arrange for a post-exposure evaluation and follow-up.

23. Only those client infections that develop within 48-72 hours following admission to the agency will be reported and evaluated. If the client had a diagnosed infection on admission, the infection is treated, and precautions are taken but it is not reported as post-admission acquired infection.

24. Client infections will be reported to the Supervisor or designee, clients doctor and department of health. These include but are not limited to:

a. Temperature of greater than 100.5
b. Vomiting
c. Diarrhea
d. Flu-like symptoms
e. Open draining lesions
f. Purulent productive cough
g. Purulent discharge from any site
h. Reported positive cultures
i. Antibiotics ordered
j. Urinary tract infections, especially in clients with catheters
k. Positive or suspected communicable disease

Below is a summarized procedure for handling a client exhibiting or reporting symptoms of a communicable disease, including reporting to the client's doctor and the Department of Health (DOH) if necessary:

a) Client Assessment: If a client exhibits or reports symptoms of a communicable disease, the caregiver should promptly assess the situation, taking notes of symptoms and any relevant medical history.

b) Isolation and Precautions: If the symptoms suggest a communicable disease, the caregiver should take appropriate precautions to prevent the spread of infection. This may include isolating the client from others and using personal protective equipment (PPE) as necessary.

c) Medical Attention: The caregiver should encourage the client to seek medical attention from their primary care physician or healthcare provider. If the client is unable to do so, the caregiver should assist in arranging a medical evaluation.

d) Communication with the Doctor: If the client consents, the agency should contact the client's doctor to report the illness and provide relevant information about symptoms and any care provided.

e) Reporting to DOH: If the disease is reportable according to state guidelines (such as those listed in the DOH guidelines), the agency should notify the Department of Health as required by law. This notification should include all pertinent information about the client's condition and any actions taken by the agency.

f) Documentation: The agency should document all actions taken, including communication with the client's doctor, any referrals made, and notifications sent to the Department of Health. This documentation should be thorough, accurate, and maintained securely according to privacy regulations.

g) Follow-Up Care: After reporting the illness and providing necessary care, the agency should continue to monitor the client's condition and follow any guidance provided by healthcare professionals or public health authorities.

25. Agency personnel who are immunosuppressed will not be assigned to clients where there is a significant risk of harm to them from an infectious disease.

26. Agency personnel who are known to be pregnant will not be assigned to clients where there is a significant risk of substantial harm to the fetus from infectious diseases.

REFERENCES

WAC 246-335-420 WAC 246-335-320

https://apps.leg.wa.gov/wac/default.aspx?cite=246-335-301

https://apps.leg.wa.gov/wac/default.aspx?cite=246-335-320

TUBERCULOSIS EXPOSURE CONTROL

Policy

It is the policy of Golden Years Home Health Supported Living LLC to protect clients, personnel, and community members from exposure to tuberculosis (TB) through early identification, use of protective barriers, immediate treatment, and appropriate notification to health Department of Health officials. The agency will comply with Centers for Disease Control and Prevention (CDC), state Department of Health, and OSHA guidelines for occupational exposure to TB. For clients in the home setting whose environment cannot be safely maintained from TB exposure for client and agency personnel, a transfer of the client to a suitable general inpatient setting may be arranged for the initial treatment phase. It is the policy of Golden Years Home Health Supported Living LLC that the Tuberculosis (TB) infection control program. Key elements include, but are not limited to:

1. Conducting a TB risk assessment for all new employees upon hire. The agency uses a tuberculosis risk assessment form provided by the Department of Health. (Adult Tuberculosis Risk Assessment attached) https://doh.wa.gov/sites/default/files/legacy/Documents/Pubs//343-144-AdultTBRiskAssessment.pdf?uid=624c7 40b8cc40
2. Based on risk assessment results, the agency's responsibility is to conduct TB testing of new employees.
3. If TB testing is required, follow the Department of Health tuberculosis risk assessment form testing recommendations;
4. Retesting should only be done for persons who previously tested negative and have new risk factors since the last assessment; and
5. Ensuring workers receive TB related training and education at the time of hire or during new employee orientation. Training and education must be consistent with the Department of Health tuberculosis program online posted educational materials

Purpose

1. To define and perform tuberculin testing on all direct service personnel.
2. To identify and implement work practice controls for early identification, investigation, and control of TB.
3. To identify clients with TB symptoms or being admitted with TB prior to admission and to implement the TB control measures for clients in the home setting or as necessary for client and employee safety.

Procedure

Tuberculin Skin Testing of Personnel

1. Upon hire, new employee who will have direct client contact will receive TB screening using a tuberculosis risk assessment form provided by the department. https://doh.wa.gov/sites/default/files/legacy/Documents/Pubs//343- 144-AdultTBRiskAssessment.pdf?uid=624c740b8cc40
2. The tuberculosis test, if required, shall be administered by a licensed health care professional who is specifically trained for the procedure.
3. Employees who present evidence of a previous positive TB test he or she has previously been treated for TB infection or disease shall be excluded from the TB screening.
4. has implemented a written policy regarding employees who develop or sustain symptoms of infectious diseases to determine when employees shall be removed from contact with clients.
5. For any employee exposed to an active TB case, follow-up testing will be done as soon as possible after exposure to establish a TB status baseline. A second TB skin test will be repeated 12 weeks after exposure to identify positive conversion.
6. Skin test conversions will be documented on the appropriate OSHA log and reported to the DHCS.
7. Results of TB skin testing or x-ray reports will be:

a. Maintained in personnel health records with other pertinent documentation of health information.
b. Be stored in such a manner as to be protected from loss, destruction, or unauthorized disclosure or use.
c. Be retained for a minimum of three years following termination of employment.

8. An agency risk assessment of personnel occupational exposure to TB will be performed annually by the Owner/Administrator or designee identifying:

a. The number of agency clients with active/confirmed cases of TB in the past year.
b. The number of TB test conversions among personnel tested during the previous year.
c. Review the agency TB policy and revise it as necessary to meet CDC and health Department of Health guidelines in response to:
    i. A census of six or more TB clients during the previous year.
    ii. Two or more employee TST conversions during the previous year.

iii. Exposure of personnel to drug resistant TB or a significant increase in the number of clients with HIV served by the agency.

TB Infection Control Procedures

1. The agency will screen client intake referrals for indications of cases of suspected or confirmed infectious TB. No prospective clients with active TB will be admitted for services.
2. The CDC defines suspected cases as those having symptoms consistent with TB, which include:
   a. productive cough,
   b. coughing up blood,
   c. weight loss, and loss of appetite,
   d. lethargy/weakness,
   e. night sweats or fever.
3. Staff are educated to be aware of the above symptoms and report them immediately to their supervisor.
4. The supervisor will investigate the report and determine if further action is needed.
5. Clients found to have active TB will be transferred to an appropriate service facility.
6. All personnel exposed to the client will be tested for TB.
7. The state health Department of Health will be notified of all exposed individuals.
8. Personnel who are immunosuppressed or pregnant or lactating will not be placed on active or suspected case of TB.
9. For immune-compromised persons or young children living with a client who has suspected or confirmed pulmonary or laryngeal TB, temporary relocation should be considered until the client has negative sputum smears.
10. Any employee working with a client with confirmed or suspected active TB in the home will take proper respiratory precautions upon identifying any signs and symptoms consistent with infectious TB. The Owner/Administrator will be notified of any client with signs and symptoms consistent with TB.

11. Respiratory precautions in the home may be discontinued when the client is improving clinically, cough has decreased, and the number of organisms in the sputum smear is decreasing. This usually occurs within two to three weeks after TB medications are begun. Failure to take medications as prescribed and the presence of drug-resistant disease are the two most common reasons for a client's failure to improve clinically. Agency personnel should educate clients about the importance of taking medications as prescribed.
12. Personnel entering the home of an active or suspected TB client will wear OSHA approved properly fitted, respiratory protection respirator, mask, gloves, and/or eye protection as warranted by the client circumstances and CDC guidelines.
13. Clients with suspected or confirmed TB should be instructed to cover their mouths and noses with tissue when coughing or sneezing.
14. Direct care contractors are held to the agency's TB infection control policies and procedures.
15. TB incidents will become part of the QIP reports.

REFERENCES

WAC 246-335-335 WAC 246-335-425 https://apps.leg.wa.go v/wac/default.aspx?cit e http s://apps.leg.wa.go v/wac/default.aspx?cit e

HEPATITIS B PREVENTION/VACCINATION

Policy

It is the policy of Golden Years Home Health Supported Living LLC to protect job classification-specific personnel from the Hepatitis B virus by offering Hepatitis B vaccine within 10 days of employment. The agency will comply with CDC, state health Department of Health, and OSHA, and OSHA guidelines for the Hepatitis B vaccine protocol. The cost of the Hepatitis B vaccine series and related testing will be the agency's responsibility.

Purpose

1. To educate new agency personnel about Hepatitis B virus risks and Hepatitis B vaccine benefits.
2. To provide the Hepatitis B vaccine series to those agency personnel who agree to the vaccine.
3. To maintain a signed record of each agency employee who declines the Hepatitis B vaccine in the employee's health file.

Procedures

1. All new personnel whose job classification includes exposure to blood will receive written educational materials about the Hepatitis B virus and its risks.
2. Hepatitis B is spread through the exchange of bodily fluids, such as semen, vaginal fluids, and blood. It is 50-100 times more infectious than HIV. 3,00 Americans die of Hepatitis B each year. It is a contagious liver disease that can be acute or chronic. It is preventable.
3. Within 10 working days of employment, the agency will make the three-dose series of Hepatitis B vaccine available to all personnel at no cost and followed by the post-vaccination antibody titer per CDC and OSHA guidelines.
4. Exemptions from the vaccine are made for an employee who has previously received the complete Hepatitis B vaccination series, antibody testing reveals employee immunity, the vaccine is contraindicated for medical reasons, or the employee refuses for personal reasons. Circumstances of the exemption are recorded and maintained in the employee's health file.
5. Hepatitis B virus prescreening may be offered to the employee but is not a prerequisite for receiving vaccination.
6. Personnel with occupational exposure, who decline vaccination, must sign a statement to that effect within 10 working days of employment. The signed statement is kept in the employee's health file.

7. Personnel with potential Hepatitis B virus exposure will be provided with appropriate protective equipment.
8. Complete and detailed documentation of all Hepatitis B virus exposure is maintained. A confidential medical evaluation, counseling, source, and employee blood testing and follow-up care will be offered to any employee experiencing an occupational exposure to Hepatitis B virus.
9. The Hepatitis B vaccine will be administered and documented by an appropriately licensed professional following recommended guidelines for administration.
10. Should an employee initially decline the vaccine but later decide to accept the vaccination, the agency will make the vaccine available.

REFERENCES

WAC 246-335-420

COVID -19 GUIDANCE AND TRAINING

Policy

Integrity is committed to protecting our clients and our staff from contracting and spreading the coronavirus. As such, we provide our staff with appropriate training, education and personal protective equipment to avoid the transmission of COVID19. Such PPE includes, but is not limited to face masks, gloves, gowns, hand sanitizers and goggles or face shields. It is further our policy for all our caregiving staff to wear face masks while in a client's home even when staff have no symptoms of COVID 19, since they may be asymptomatic, but able to spread the virus. Integrity uses multiple sources of information to accomplish this and follows the Washington Department of Health guidance as a basis for training, education and practices as it relates to COVID-19 "The Washington State Department of Health (DOH) recognizes the need for COVID19 infection prevention guidance to help home care, home health, and hospice agencies that provide care to clients/patients in their homes. This work includes helping people around the house, helping them with personal care and providing clinical care. Those over 60 years old and those with serious medical conditions of any age are at higher risk for severe illness from COVID-19. It is important that agencies help their staff take precautions to protect their clients and themselves currently."

Purpose

1. To prevent staff from becoming infected.
2. To prevent staff from infecting clients
3. To provide a safe and protected environment for staff and clients

Procedure

Before Going to the Home of a Client, Golden Years Home Health Supported Living LLC management staff will:

1. Explore remote ways of providing or preparing to provide care, such as:
    a. Telehealth and electronic charts.
    b. Plan important care services by phone video chat or on a webinar.
2. Identify and prepare for times when staff might come into direct contact with the client:

a. Providing medication, meal prep in the home, cleaning, laundry, or other times where there is direct client or item contact in the home.

3. Limit staff on hand to provide only essential care needs currently.

4. Ask all clients and other household members to wear a mask while the staff member is in the home.
5. Screen clients and household members for symptoms and possible exposures to COVID-19 prior to entering the home. Ensure staff know your agency's protocols for
6. screening and follow them for every client encounter.

For In-Home Clients/Patients with Known or Possible COVID-

1. Establish them in a separate bedroom with a separate bathroom away from others if possible.
2. Ensure that all people in the home have personal protective equipment to use (at least gloves and facemask) and use healthy habits (such as covering their coughs and sneezes, throwing used tissues away, and washing their hands often). Family members should follow CDC's guidance for Caring for Someone Sick at Home.
3. Make sure that pets in the home are cared for by others.

Discharge Planning

1. If caring for clients/patients with COVID-19, the agency will share infection status with the receiving service provider and transport staff before the discharge/transfer. Infection Control Planning for Agencies; The Agency management will:
2. Educate client family members about steps taken to prevent spread of COVID-19:

a. Monitor and exclude ill staff and visitors.
b. Keeping in touch with loved ones from a distance (such as, phone, video call, etc.).
c. CDC advice for people living with someone with COVID-19.

3. Screen staff for symptoms daily before beginning work.
4. Share policies for flexible medical leave with staff and instruct them on how and who to tell if they believe they are sick with COVID-19. Sick staff should stay home.

a. CDC Guidance on Strategies to Mitigate Healthcare Personnel Staffing Shortages

5. Identify staff at higher risk for severe COVID-19 and have them work with other clients/patients or stay home.
6. Establish a system to track the date and time of all visitors, health care providers, and others in the home.

7. Schedule home visits with people who have or may have COVID-19 at the end of the day to keep risk of spreading COVID-19 during other visits. Protecting Staff and Clients: Integrity In-Home Care management will:

1. Train staff on standard contact, and droplet precautions.
2. Provide staff with appropriate personal protective equipment (PPE) and provide training for proper use:
   a. See L&I and DOH Respirator and PPE for In-Home Care Agencies: Employer responsibilities for respiratory protection program and provision of personal protective equipment (PPE).
3. When community transmission is moderate or high (>25/100,000 cases in 2 weeks according to DOH COVID-19 cases and testing by County), healthcare providers should wear a surgical face mask and eye protection for ALL client care encounters, regardless of patient/client COVID-19 status.
4. CDC's Optimizing the Use of PPE provides guidance on actions to take when PPE is limited or unavailable. Crisis measures should be avoided if possible as it places healthcare personnel at increased risk for disease.

a. Prioritize gowns for use during activities that produce splashes and sprays and during high-contact client care activities.
b. Prioritize eye protection for use during activities that produce splashes and sprays and during activities where prolonged face-to-face or high-contact occurs.
c. As possible, staff should doff PPE outside of the home. Once outside, follow the CDC's guidance for removal.

5. Have the fewest amount of people present during aerosol-generating procedures such as a client or patient receiving a procedure or undergoing an activity for which respirators are required due to the generation of aerosols that pose a COVID19 transmission hazard. This includes, but is not limited to, procedures identified by DOH, CDC, and the following: administering medicines with a nebulizer, spirometry, use of a BiPAP or CPAP, deep or forced breathing exercises, or collecting or handling specimens from known or suspected COVID-19 patients. Infection Control Guidelines for Caring for Clients/Patients with COVID-19 uses the following guidelines for training staff:

1. Wash hands often with soap and water or use 60% alcohol-based hand sanitizer. For staff tips, see Clean Hands Count for Healthcare Providers.
2. Cough and sneeze into the elbow or into a tissue. Throw away the tissue right after using and wash hands.

3. Clean and disinfect surfaces often. Clients/patients with COVID-19 can worsen quickly and it is important that all caregivers and household members are able to call healthcare providers or 911. Inform staff that they should call 9-1-1 if their patients/clients are experiencing any of the following symptoms:
   a. Blue lips
   b. Problems breathing or oxygen saturation <92%
   c. Change in consciousness
   d. High fever
4. All Care givers will follow CDC guidance

Environmental Cleaning and Disinfection

1. Staff will clean surfaces with soap and water or other cleaning agents to disinfect. At least once during a visit, wipe high-touch areas (doorknobs, handrails, etc.) with a disinfectant.
2. Staff will only use EPA approved disinfectant from List N along with CDC guidance for cleaning and disinfection.
3. Diluted household bleach (at least 1000 ppm sodium hypochlorite: 5 tablespoons [1/3 cup] bleach per gallon of water or 4 teaspoons bleach per quart of water) can be used to disinfect on surfaces not damaged by bleach.
4. Staff are to read the label before use of any disinfectant and follow the label's
5. instructions. Never mix household bleach with ammonia or any other cleanser.

Existing Guidance for Home Health and Hospice Home care agencies should prioritize infection prevention in all client interactions and care delivery, following the Washington State Department of Health (DOH) recommendations and the applicable Washington Administrative Codes (WAC) for home care agencies. Agencies should implement infection prevention protocols to ensure the health and safety of clients and caregivers. For additional guidance, home care agencies may refer to:

1. Washington State Department of Health resources on infection prevention for home care settings.
2. Guidance on providing home care for individuals not requiring hospitalization.
3. Recommendations for caring for someone at home while preventing the spread of infection.

More COVID-19 Information and Resources

Stay up to date on the current COVID-19 situation in Washington, Governor Inslee's proclamations, symptoms, how it spreads, and how and when people should get tested. See our Frequently Asked Questions for more information. A person's race/ethnicity or nationality does not, itself, put them at greater risk of COVID-19. However, data are revealing that communities of color are being disproportionately impacted by COVID-19. This is due to the effects of racism, and in particular, structural racism, that leaves some groups with fewer opportunities to protect themselves and their communities. Stigma will not help to fight the illness.

Share only accurate information to keep rumors and misinformation from spreading.

a. WA State Department of Health 2019 Novel Coronavirus Outbreak (COVID-19)
b. WA State Coronavirus Response (COVID-19)
c. Find Your Local Health Department or District
d. CDC Coronavirus (COVID-19)
e. Stigma Reduction Resources

Remote training via the internet will be used to limit exposure to one another. We will have staff return-demonstrate how to use PPE, once they've been trained and have our trainer watch them via real-time internet connection (Zoom, Google Chat, etc.), or in-person, if need be, in a well-ventilated space with the trainer and caregiver maintaining at least 6 feet between them and wearing masks.

Caregivers will also be trained in the procedures they are to take in case either a client or the caregiver him/herself show any of the symptoms of COVID-19. The trainer will document the name of the caregiver, the date the COVID-19 Training was completed, the length of the training and include the trainer's name. The trainer will then make sure this document is placed in the caregiver's personnel file.

STANDARD PRECAUTIONS

Policy

Golden Years Home Health Supported Living LLC will require the following as part of its overall infection control program. All field personnel will receive universal precautions education upon hire as part of their orientation, and annually thereafter. Confirmation of compliance with universal precautions will be part of supervisory visits.

Purpose

1. To protect clients and personnel from infections through transfer of contamination.
2. To ensure compliance with universal precautions.
3. To ensure employee education is provided on universal precautions and safe practices.

Procedure

All field personnel will demonstrate universal precaution competency, as a component of the annual performance evaluation. It should be assumed that client blood and all body fluids, whether visible or not, are potentially infectious. The following rules apply:

1. Wash Hands
    a. Before and after direct client contact
    b. After contact with bodily fluids or excretions, mucous membranes, non-intact skin or wound dressings
    c. Before handling supplies or equipment for client service whether gloves are used
    d. When moving from a contaminated body site to another site during service
    e. After contact with inanimate surfaces and objects in the immediate client area
    f. After removing client service gloves
    g. Before handling medication or preparing food
    h. If hands are visibly dirty or soiled with blood or other body fluids
    i. After using the toilet
2. Hand Washing Techniques

a. Hand washing should take at least 20 seconds.
b. Wash hands with soap and water when visibly dirty or visibly, soiled with blood or other body fluids, after using the toilet, or if exposure to potential spore-forming pathogens is strongly suspected or proven.

c. Use an alcohol-based hand rub as the preferred means for routine hand antisepsis in all other situations. If hands are not visibly soiled or if alcohol-based hand rub is not obtainable, wash hands with soap and water.

3. Gloves

a. Vinyl or latex gloves must be worn when cleaning reusable equipment, when having direct contact with blood, body fluids, mucous membrane or non-intact skin, or when handling equipment contaminated with blood or body fluids. This includes, but is not limited to the following:

   i. Emptying of suction and drainage devices

   ii. Catheter care

   iii. Dressing changes

   iv. Handling of grossly contaminated linens

   iv. Providing oral hygiene

   vi. Cleaning client rooms, bathrooms, emptying trash or changing linens

b. Gloves will be changed between client contacts. When gloves are removed, thorough hand washing is required. Gloves DO NOT take the place of hand washing.

4. Goggles or protective glasses should be worn when there is potential for a splash with blood or body fluids.

5. Gowns or aprons should be worn when there is a potential for blood or body fluid splatters or sprays or there may be potential for contact with grossly soiled linens, clothing, etc.

6. Masks should be worn if there is a chance of splashing or splattering or the client is on respiratory precautions.

7. Airways: One-way airway, mouthpiece, resuscitation bag, or other ventilation device will be in possession of all Home Care Aids while on duty.

8. In the event of contamination with blood or body fluids, body surfaces should be washed immediately with soap and water.

9. Contaminated waste will be disposed of in a doubled plastic bag and placed in the client's trash container. Areas and equipment contaminated with blood should be cleaned immediately with 1:10 bleach solution or Lysol.

10. Soiled linens should be handled as little as possible and with minimum agitation to prevent gross microbial contamination of the air and of persons handling the linen.

11. Personnel cleaning contaminated equipment should wear gloves and a gown.
12. If an agency-issued thermometer is used, it should be wiped with alcohol and air-dried before and after use. Probe covers are to be used each time a temperature is taken.
13. All equipment used during client services will be cleaned with alcohol/approved disinfectant per manufacturer's instructions prior to use.
14. The agency will maintain a log describing the collection, transportation and disposition of hazardous waste, as per state regulations.

REFERENCES

WAC 246-335-425

HAND HYGIENE POLICY

Policy

Golden Years Home Health Supported Living LLC's personnel education includes hand washing hygiene. All members of the healthcare team will comply with current Centers for Disease Control and Prevention (CDC) hand hygiene guidelines.

Purpose

Effective hand hygiene reduces the incidence of healthcare-associated infections.

Procedure

Indications for Handwashing and Hand rubbing

A. Indications for Handwashing

1. When hands are visibly dirty or contaminated with proteinaceous material or are visibly soiled with blood or other body fluids, wash hands with either non-antimicrobial soap and water or an antimicrobial soap and water.
2. Before eating and after using a restroom, wash hands with nonantimicrobial soap and water or with an antimicrobial soap and water.
3. Handwashing may also be used for routinely decontaminating hands in the following clinical situations:
   a. Before having direct contact with clients
   b. Before inserting indwelling urinary catheters, peripheralvascular catheters, or other invasive devices that do not require a surgical procedure
   c. After contact with a client's intact skin (e.g., when taking a pulse or blood pressure, and lifting a client)
   d. After contact with body fluids or excretions, mucous membranes, non-intact skin, and wound dressings, even if hands are notvisibly soiled
   e. When moving from a contaminated body site to a clean body site during client care
   f. After contact with inanimate objects (including medical equipment) in the immediate vicinity of the client
   g. After removing gloves

B. Indications for Hand rubbing

If hands are not visibly soiled, an alcohol-based hand rub may be used for routinely decontaminating hands in the following clinical situations:

- Before having direct contact with clients

- Before inserting indwelling urinary catheters, peripheral vascular catheters, or other invasive devices that do not require a surgical procedure
- After contact with a client's intact skin (e.g., when taking a pulse or blood pressure, and lifting a client)
- After contact with body fluids or excretions, mucous membranes, nonintact skin, and wound dressings, only if hands are not visibly soiled
- When moving from a contaminated body site to a clean body site during client care
- After contact with inanimate objects (including medical equipment) in the immediate vicinity of the client
- After removing gloves Non-Surgical Hand Hygiene Technique

A. Handwashing with soap and water (either non-antimicrobial or antimicrobial)

1. Wet hands with running water
2. Apply hand washing agent to hand
3. Vigorously rub hands together for at least 15 seconds, covering all surfaces of hands and fingers
4. Rinse hands thoroughly with water and with hands angled down in the sink
5. Dry hands thoroughly with a disposable towel(s)
6. Use disposable towel to turn off the water

B. Alcohol-based hand rub

1. Apply product to palm of one hand
2. Rub hands together, covering all surfaces of hands and fingers
3. Continue to rub until hands are dry

C. Nails

1. Artificial fingernails or extenders may not be worn if duties include direct contact with clients
2. Natural nail tips shall be less than ¼ inch long Gloves and Hand Hygiene Gloves reduce hand contamination by 70 – 80 percent, prevent cross-contamination and protect clients and health care personnel from infection.

However, the use of gloves does not eliminate the need for hand hygiene.

1. Wear gloves when contact with blood or other potentially infectious materials (other body fluids, secretions and excretions), mucous membranes, non-intact skin and contaminated items will or could occur.
2. Change gloves during client care if moving from a contaminated body site to a clean body site.

3. Remove gloves promptly after use, before touching non-contaminated items and environmental surfaces, and before caring for another client.
4. Decontaminate hands after removing gloves. Glossary of Commonly Used Hand Hygiene Terms Alcohol-based hand rub. An alcohol-containing preparation designed for application to the hands for reducing the number of viable microorganisms on the hands. In the United States, such preparations usually contain 60%–95% ethanol or isopropanol. Antimicrobial soap. Soap (i.e., detergent) contains an antiseptic agent. Antiseptic agent. Antimicrobial substances that are applied to the skin to reduce the number of microbial flora. Examples include alcohols, chlorhexidine, chlorine, hexachlorophene, iodine, chloroxylenol (PCMX), quaternary ammonium compounds, and triclosan. Antiseptics handwash. Washing hands with water and soap or other detergents containing an antiseptic agent. Antiseptic hand rub. Applying an antiseptic hand rub product to all surfaces of the hands to reduce the number of microorganisms present.

Cumulative effect. A progressive decrease in the numbers of microorganisms recovered after repeated applications of test material.

Decontaminate hands. To reduce bacterial counts on hands by performing antiseptic hand rub or antiseptic handwash.

Detergent. Detergents (i.e., surfactants) are compounds that possess a cleaning action. They are composed of both hydrophilic and lipophilic parts and can be divided into four groups: anionic, cationic, amphoteric, and nonionic detergents.

Although products used for handwashing or antiseptic handwash in health-care settings represent various types of detergents, the term “soap” is used to refer to such detergents in this guideline.

Hand antisepsis. Refers to either antiseptic handwash or antiseptic hand rub.

Hand hygiene. A general term that applies to either handwashing, antiseptic handwash, antiseptic hand rub, or surgical hand antisepsis.

Handwashing. Washing hands with plain (i.e., non-antimicrobial) soap and water.

Persistent activity. Persistent activity is defined as the prolonged or extended antimicrobial activity that prevents or inhibits the proliferation or survival of microorganisms after application of the product. This activity may be demonstrated by sampling a site several minutes or hours after application and demonstrating bacterial antimicrobial effectiveness when compared with a baseline level. This property also has been referred to as “residual activity.” Both substantive and non-substantive active ingredients can show a persistent effect if they substantially lower the number of bacteria during the wash period.

Plain soap. Plain soap refers to detergents that do not contain antimicrobial agents or contain low concentrations of antimicrobial agents that are effective solely as preservatives.

Substantivity. Substantivity is an attribute of certain active ingredients that adhere to the stratum corneum (i.e., remain on the skin after rinsing or drying) to provide an inhibitory effect on the growth of bacteria remaining on the skin.

Surgical hand antisepsis. Antiseptics handwash or antiseptic hand rub performed preoperatively by surgical personnel to eliminate transient and reduce resident hand flora. Antiseptic detergent preparations often have persistent antimicrobial activity.

Visibly soiled hands. Hands showing visible dirt or visibly contaminated with proteinaceous material, blood, or other body fluids (e.g., fecal material or urine).

Waterless antiseptic agent. An antiseptic agent that does not require exogenous water. After applying such an agent, the hands are rubbed together until the agent has dried.

REFERENCES

To access the CDC's hand hygiene guidelines in their entirety, see the CDC website

at: http://www.cdc.gov/mmwr/preview/mmwrhtml/rr5116a1.htm

EMPLOYEE EXPOSURE TO BLOODBORNE PATHOGENS

Policy

While Golden Years Home Health Supported Living LLC's personnel education focuses on prevention of exposure to blood borne pathogens and although most agency clients do not pose a risk from exposure, if an event occurs the agency will make available to personnel, at no cost, a confidential medical evaluation and follow-up should an exposure incident occur.

Purpose

To ensure that agency personnel receive appropriate treatment following exposure to blood borne pathogens.

Procedure

1. The post exposure process shall include:
    h. Documentation of the routes and circumstances of exposure and testing of the source individual, if possible, in accordance with state and local laws.
    i. Testing of the exposed employee's blood and the source individual's blood as soon as feasible upon consent.
    j. Post-exposure prophylaxis.
    k. Counseling and evaluation of the reported illness.
2. The agency has arranged for post-exposure services that are confidential, available to all direct service personnel and at no expense to the employee.
3. Within 15 days of the completion of the exposure evaluation, the agency will obtain a written statement from the health care professional examining the exposed employee that states the employee has been informed of the result of the evaluation and talked about any medical conditions resulting from the exposure which will require further follow-up.
4. All other findings or diagnoses remain confidential and shall not be included in the written report.
5. The statement must be given to the employee; however, if the information by law is confidential, the agency may not be privileged to the information.
6. The agency complies with local and state regulations regarding confidentiality and ensures that the employee receives the required information in a confidential and personal manner.

7. Any health information provided to the agency shall be filed separately from the personnel record and maintained on an exposed employee for the period of employment plus 30 years.
8. If an employee is exposed to a known Hepatitis B virus and has not previously received the vaccine or is not known to have adequate immunity, the employee will be referred to an agency-approved licensed physician or licensed health care professional for treatment.
9. Hepatitis B vaccine treatment will be at the agency's expense and available at a reasonable time and place. The agency will ensure that an accredited laboratory, at no cost to the employee, conducts laboratory testing.
10. If the employee refuses treatment, a signed statement will be obtained which indicates that the employee has been counseled regarding the risk, treatment has been offered, and the employee has refused.
11. An incident report shall be completed and processed according to agency policy.
12. All appropriate workman's compensation forms (according to state regulations) must be completed if the required amount of time is lost from work.

At Risk Personnel

1. The agency will maintain a list of employee job classifications and specific tasks that might result in possible exposure and provide appropriate training to such personnel.
2. OSHA standards on occupational exposure to blood borne pathogens applies to all personnel who may handle any body fluid such as transporting lab specimens to a laboratory or disposing of items contaminated with body fluids, such as sharps containers and other hazardous waste brought back to the agency, and to personnel responsible for cleaning areas of possible contamination.
3. The following job classifications have been identified as having responsibilities or as performing procedures that place them at risk of exposure to blood borne pathogens:
    a. RN (if on staff as Supervisor)
    b. LPN (if on staff as Supervisor)
    c. Basic Personal Care Assistant
    d. Homemaker

REFERENCES

WAC 246-335-425

HAZARDOUS WASTE DISPOSAL

Policy

Although Golden Years Home Health Supported Living LLC's services do not generally produce hazardous waste, there may be situations in the client home which do. Using EPA guidelines, the agency ensures that personnel properly and safely dispose of or arrange for the disposal of all hazardous waste to protect the clients, personnel, and the environment from unnecessary exposure to hazardous substances.

Purpose

1. To protect clients, personnel, and the environment from hazardous waste risks.
2. To ensure proper disposal of hazardous waste per local, state rules.
3. To ensure that personnel are aware of the procedures for proper hazardous waste disposal.

Procedure

1. Each client with hazardous waste or syringes must acquire a puncture proof container.
2. Filled containers are to be placed in a large sealable polyethylene or polypropylene bag that is labeled with a distinctive warning to the waste disposal company and a new puncture-proof container provided.
3. Unused portions of drug(s) and equipment used to administer the drugs are to be disposed of according to the established agency policy and local authority procedures.
4. Personnel will be advised not to dispose of unused drugs or contaminated solutions in drains or toilets, but to use the original vial, I.V. bag/cassette, or other closed container, which is to be placed in a labeled, double plastic bag.
5. Contaminated materials, i.e., gloves, gowns, masks, vials, tubing, and absorbent pads, shall be double bagged in leak-proof, puncture-proof sealable bags or containers made of polyethylene or polypropylene that should include a prominent and distinctive warning label unless labeling is not allowed due to local regulations for waste disposal. An example of a warning label is CAUTION: Antibiotic or Chemical Waste.
6. The plastic bags should be placed in a lined cardboard box, which is labeled, sealed, and taped for disposal. Arrangements for disposal pick-up should be made with a waste management firm that has been licensed to dispose of toxic waste materials.
7. Chemotherapy clients should know the proper care and handling of their urine, stool, vomitus, or other bodily fluids and proper hand-washing techniques. Agency personnel will have these instructions on their service plan. A spill kit should be in the home.

8. It is recommended that serious consideration be given to the possible dangers to pregnant or lactating women or young children who may reside in the home and be exposed to chemotherapeutic drugs or waste products. Pregnant or lactating personnel should not be assigned to clients receiving chemotherapy.
9. In the event of accidental exposure to hazardous waste, personnel should be instructed to:
    e. Remove contaminated gloves or gowns immediately and discard them properly.
    f. Wash skin with soap (not a germicidal agent) and water.
    g. Flood an eye that is accidentally exposed with water or isotonic eyewash for at least five minutes.
    h. Obtain a medical evaluation as soon as possible and document the incident according to established policies.
10. Preventing accidental spills and protecting articles of clothing, bedding, and furniture from contact with hazardous waste is imperative. The spill should be reported as an incident.
11. If a spill occurs on the client's clothing or sheets, they should be handled with gloves and washed in hot water separated from other articles.
12. If a spill occurs on an unprotected piece of furniture, the employee wearing gloves should scrub the area with detergent solution and water, rinse with clean water, and dispose of contaminated materials.
13. All these procedures for hazardous waste cleanup require the use of double un-powdered latex gloves.
14. Document the incident according to established agency policies.

REPORTABLE DISEASES

Policy

Golden Years Home Health Supported Living LLC will report to authorities those communicable diseases required by state regulations.

Purpose

1. To protect clients and personnel from exposure to communicable diseases.
2. To comply with state rules for infectious disease reporting.

Procedure

1. The agency will provide the following information to all staff at orientation and yearly when required in-services on infection control and universal precautions'
2. The information will also be available online to staff.

Notifiable Condition	Time Frame for Notification	Notifiable to Local Health Department	Notifiable to State Department of Health
Acquired Immunodeficiency Syndrome (AIDS)	Within 3 business days	√	
Animal Bites (when human exposure to rabies is suspected)only including, but not limited to, West Nile virus, eastern and western equine encephalitis, dengue, St. Louis encephalitis, La Crosse encephalitis, Japanese encephalitis, and Powassan)	Immediately	√	
	Within 3 business days	√	
Disorders	Monthly	√	
Birth Defects	Monthly	√	
Botulism (foodborne, infant, and wound)	Immediately	√	
Brucellosis (Brucella species)	Within 24 hours	√	
Burkholderia mallei (Glanders) and pseudomallei (Melioidosis)	Immediately	√	

Campylobacteriosis	Within 3 business days	√	
Chancroid	Within 3 business days	√	
Chlamydia trachomatis infection	Within 3 business days	√	
	Immediately	√	
Cyclosporiasis	Within 3 business days	√	
bioterrorism origin	Immediately	√	
producing E. coli"	Immediately	√	
Emerging condition with outbreak potential	Immediately	√	
Giardiasis	Within 3 business days	√	
Gonorrhea	Within 3 business days	√	
Granuloma inguinale	Within 3 business days	√	
Haemophilus influenzae (invasive disease, children under age 5)	Immediately	√	
Hantavirus pulmonary syndrome	Within 24 hours	√	
Hepatitis A (acute infection)	Within 24 hours	√	
Hepatitis B (acute infection)	Within 24 hours	√	
Hepatitis B surface antigen + pregnant women	Within 3 business days	√	

Hepatitis B (chronic infection) - Initial diagnosis, and previously unreported prevalent cases	Monthly	√	
Hepatitis C (acute infection)	Within 3 business days	√	
	Monthly	√	
	Within 3 business days	√	
Hepatitis E (acute infection)	Within 24 hours	√	
Herpes simplex, neonatal and genital (initial infection only)	Within 3 business days	√	
Human immunodeficiency virus (HIV) infection	Within 3 business days	√	
Influenza, novel or unsub typable strain	Immediately	√	
Influenza-associated death (lab confirmed)	Within 3 business days	√	
Legionellosis	Within 24 hours	√	
Leptospirosis	Within 24 hours	√	
Listeriosis	Within 24 hours	√	
Lyme Disease	Within 3 business days	√	
Lymphogranuloma venereum	Within 3 business days	√	
Malaria	Within 3 business days	√	

Measles (rubeola) - Acute disease only	Immediately	√	
Meningococcal disease (invasive)	Immediately	√	
	Within 24 hours	√	
Outbreaks of suspected foodborne origin	Immediately	√	
Outbreaks of suspected waterborne origin	Immediately	√	
	Within 24 hours	√	
Pesticide poisoning (hospitalized, fatal, or cluster)	Immediately		√
Pesticide poisoning (all other)	Within 3 business days		√
	Immediately		√
Psittacosis	Within 24 hours	√	
Q Fever	Within 24 hours	√	
Rabies (Confirmed Human or Animal)	Immediately	√	
Rabies suspected human exposure (suspected human rabies exposures due to a bite from or other exposure to an animal that is suspected of being infected with rabies)	Immediately	√	
Relapsing fever (borreliosis)	Within 24 hours	√	
Rubella (including congenital rubella syndrome) (acute disease only)	Immediately	√	
Salmonellosis	Within 24 hours	√	
Immunizations	Immediately	√	
	Within 3 business days	√	

Shiga toxin-producing E. coli infections (enterohemorrhagic E. coli including, but not limited to, E. coli O157:H7)	Immediately	√	
Shigellosis	Within 24 hours	√	
	Immediately	√	
Tetanus	Within 3 business days	√	
Trichinosis	Within 3 business days	√	
Staphylococcus aureus (not to include vancomycin intermediate)	Immediately	√	
	Within 24 hours	√	
Varicella-associated death	Within 3 business days	√	
Vibriosis	Within 24 hours	√	
	Immediately	√	
Other rare diseases of public health significance	Within 24 hours	√	
Unexplained critical illness or death	Within 24 hours	√	

REFERENCES

WAC 246-101-101

SAFETY

SAFETY MANAGEMENT

Policy

Golden Years Home Health Supported Living LLC will plan for a safe environment, addressing physical safety in the workplace and the client's environment. A Safety Committee will be appointed by the Owner/Administrator/designee and will meet at least annually for fire drills and to address safety issues both in the office and in the client's residences. Safety committee members will perform an annual office safety assessment.

Purpose

1. To promote a safe working environment for personnel working in the agency office environment or in the client's residence.
2. To provide for a planned orderly process for assessing and addressing potential workplace safety risks per OSHA, Food and Drug Administration (FDA), local, state, and regulatory body requirements.

Procedure

1. Safety management is always the responsibility of each employee in the work place and when in the client's home.
2. The planning process will include conducting safety assessments to determine personnel and client risks.
3. All personnel, clients and caregivers will receive applicable safety instruction including, but not limited to:
    a. Basic home safety
    b. Fire prevention and response
    c. Electrical safety
    d. Environmental safety
    e. Bathroom safety
    f. Safe transfers and ambulation
    g. Safe use of medical equipment
    h. Safe storage, handling, delivery and access to supplies, medical gases and drugs, especially, controlled substances, parenteral and enteral nutrition solutions
    i. Use of gloves

j. Safe Refrigeration

k. Safe disposal of needles in a puncture-resistant container

l. Infection control

m. Proper hand washing

n. Universal precautions

o. Hazardous waste handling and disposal

4. Clients/caregivers will acknowledge in writing the receipt of verbal and written instructions regarding safety management.

5. Client care personnel shall monitor and document the client/representative's understanding and compliance with safety management on an ongoing basis.

6. Client-related safety hazards will be documented in the clinical record and brought to the attention of the client/representative for discussion of hazard reduction techniques.

7. All client care personnel will attend in-service education on safety management upon hire, annually and if there is an identified need.

8. All accidents or injuries will be reported to the Supervisor and documented in an Incident Report. If the accident involves the client, appropriate actions will be initiated, and the physician will be notified to obtain specific follow-up orders.

9. A report on safety related incidents will be presented to the Owner/Administrator including the causal factors and actions to prevent a similar incident.

10. If an accident or incident involves equipment malfunction and serious injury, illness or death, the incident will be reported to the vendor and/or manufacturer and/or the FDA within 10 days of incident notification following the Medical Device Act protocols.

11. A semiannual report summarizing incidents occurring in the previous 6 months must be filed with the FDA. Reports are due January 31 and July 31 of every year. If no incidents have occurred, no report is needed.

12. The OSHA 300 Log of Work-Related Injuries and Illnesses will be maintained by the agency according to OSHA regulations. The Owner/Administrator or Safety Committee will post OSHA Form 300A Summary of Work-Related Injuries and Illnesses from February 1 to April 30 of the year after the year covered by the Log even if there were no incidents.

13. All incidents will become part of the Annual Program Evaluation and QIP reports. Safety Committee

1. The Safety Committee will be comprised of at least two employees appointed by the Owner/Administrator and chosen to represent both

agency operational and client service areas per OSHA requirements.

2. The Safety Committee will be organized to distribute safety information, observe and identify areas of safety concern, develop corrective action plans to improve or resolve situations, and evaluate the outcome of the implemented actions.
3. Safety Committee members will be responsible for safety in-services, monitor compliance, maintain and update the OSHA compliance notebook, and update and post the OSHA 300 log per OSHA standards.
4. Safety Committee members will be knowledgeable in safety subject matter and conduct or participate in safety training in–services
5. The Safety Committee meets at least annually and as needed to address special issues or situations. Minutes will be taken and maintained in the Safety Manual.
6. All site inspections will be documented in a Safety Manual.
7. The Safety Committee will conduct annual fire drills.
8. Safety Training records will be maintained for three years and will include:
    a. Dates of training sessions and names and job titles of attendees
    b. Contents or a summary of training session
    c. Names and qualifications of persons conducting the training
    d. The site below will allow; you to download the OSHA 300 log: https://www.osha.gov/recordkeeping/new-osha300form1-1-04 FormsOnly.pdf

WORKPLACE SAFETY

Policy

To protect its personnel, Golden Years Home Health Supported Living LLC will maintain a safe working environment for all that meets OSHA, CDC, ADA, and/or local and state environmental safety protection.

Purpose

1. To protect the health and safety of all agency personnel.
2. To meet regulatory requirements.

Procedure

1. Adequate fire equipment is available and maintained.
2. Exit signs are marked clearly and properly illuminated.
3. Building fire routes are posted.
4. All biologicals are stored separately in refrigerators away from food.
5. All caustic and poisonous materials are clearly marked and unavailable to most personnel (i.e., cleaning and disinfecting supplies, etc.).
6. Personnel receive emergency preparedness and disaster training.
7. Hurricanes, earthquakes, and tornados are addressed with educational programming, and the safest places in the building, in case of an occurrence, are identified.
8. Utilities, sanitation, and agency equipment are maintained in good working order.
9. Fire drills are performed and documented at least annually or as required by local regulations.

REFERENCES

RCW 49.17

WISHA (the Washington Industrial Safety and Health Act of 1973)

PERSONNEL SAFETY

Policy

In accordance with OSHA regulations, Golden Years Home Health Supported Living LLC personnel will be provided with adequate nonhazardous conditions in which to work. The agency plans for a secure environment and addresses security concerns to protect personnel and clients from potential harm.

Purpose

1. To ensure that agency personnel work in a safe environment.
2. To ensure that agency personnel are educated regarding safety measures in the environment.

Procedure

1. No employee will be required to work in an unsafe situation or atmosphere.
2. If conditions are felt to be unsafe while in the field, the employee should leave the area, go to a phone in a safe location, and phone the Supervisor or Scheduler/Client Care Coordinator for further instructions.
3. The Supervisor will contact the client to discuss the situation and reschedule the visit.
4. All unusual incidents or circumstances should immediately be reported to the Supervisor who will determine the plan for providing services and for notifying other personnel of the plan.
5. It is important to the agency that personnel are always safe. To maximize their personal safety while working in the community and home setting, the agency will:
    e. Provide safety education for personnel, clients, and caregivers.
    f. Reinforce the right of employees to immediately leave any situation, neighborhood, or home in which they feel unsafe. Personnel must notify the supervisor as soon as possible of the situation.
    g. Set up a system for personnel to inform a contact person in the hospice agency of their location.
    h. Provide escort services and/or equipment i.e., cellular phones, handheld alarm/noise devices, sprays, etc. for all field personnel as deemed necessary.
    i. Promptly report violent incidents to the local police or Department of Health.
    j. Inform victims of workplace violence of their legal right to prosecute perpetrators.
    k. Utilize safety-related incidents as an opportunity for teaching and learning.
    l. Provide driver safety and weather emergency actions while driving in the community.

m. Hold annual fire drills.

REFERENCES

RCW 49.17

PETS IN THE HOME

Policy

Golden Years Home Health Supported Living LLC recognizes the need to protect personnel in the course of their duties and provides for individual differences and needs as it relates to pets and/or animals in the client's home or on their property.

Purpose

1. To ensure that caregivers are not assigned to clients with pets that the caregiver is either allergic to or has a fear of.
2. To establish a routine for inquiring regarding the presence of pets in the home on the initial assessment visit to the home.
3. To establish guidelines for when there are pets in the home that may pose a threat to the assigned employee.

Procedure

1. All new clients will be assessed for pets in the home or on the property. The pets present in the home or on the property will be identified as to type (cat, dog, etc.) and location of pet during visits.
2. Personnel with identified allergies or fear of the identified pets will not be assigned to the clients with pets.
3. Arrangements are made in advance for the containment of any pets that are deemed a potential threat to the employee(s) assigned to services for the client.
4. Clients/families who will not contain pets that are deemed a threat to employee(s) will be notified that services will not be provided until the pet is routinely contained. The Supervisor will work with the client/representative to resolve the situation, but repeated failures to contain the animal will result in services being cancelled, and alternate discharge/transfer plans will be offered to the client/representative.

FOOD HANDLING

Policy

Food handling and preparation will be done using appropriate food handling techniques. The agency's admitting employee will discuss with the client any food concerns and add food preparation to the service plan.

Agency training around food services will include but not limited to safe food storage, preparation and handling practices consistent with the United States Food and Drug Administration's recommendations for "food safety at home" for personnel, contractors, and volunteers involved in food preparation services on behalf of clients. Personnel, contractors, and volunteers may not provide clients with homemade food items or baked goods that they themselves have prepared.

Purpose

To safeguard the health of agency clients and prevent foodborne illness.

Procedure

All staff who are required to cook for clients as part of their duties will be required to obtain and keep current a food handler's card and follow the following basic steps while handling food.

1. Clean: always wash your food, hands, counters, and cooking tools.
    a) Wash hands in warm soapy water for at least 20 seconds. Do this before and after touching food.
    b) Wash your cutting boards, dishes, forks, spoons, knives, and counter tops with hot soapy water. Do this after working with each food item.
    c) Rinse fruits and veggies.
    d) Do not wash meat, poultry, fish, or eggs. If water splashes from the sink in the process of washing, it can spread bacteria.
    e) Clean the lids on canned goods before opening.
2. Separate (keep apart): keep raw foods to themselves. Germs can spread from one food to another.
    a) Keep raw meat, poultry, seafood, and eggs away from other foods. Do this in your shopping cart, bags, and fridge.
    b) Do not reuse marinades used on raw foods unless you bring them to a boil first.
    c) Use a special cutting board or plate for raw foods only.
3. Cook: foods need to get hot and stay hot. Heat kills germs.

a) Cook to safe temperatures:
b) Beef, Pork, Lamb 145 °F
c) Fish 145 °F
d) Ground Beef, Pork, Lamb 160 °F
e) Turkey, Chicken, Duck 165 °F
f) Use a food thermometer to make sure that food is done. You can't always tell by looking.

4. Chill: put food in the fridge right away.
   a) 2-Hour Rule: Put food in the fridge or freezer within 2 hours after cooking or buying from the store. Do this within 1 hour if it is 90 degrees or hotter outside.
   b) Never thaw food by simply taking it out of the fridge. Thaw food:
      - In the fridge
      - Under cold water • In the microwave
   c) Marinate foods in the fridge.

In case any staff think to have any food illness they should;

✔ Call your health care provider and get medical care right away.

✔ Save the food package, can, or carton. Then report the problem.

✔ Call USDA at 1-888-674-6854 if you think the illness was caused by meat, poultry, or eggs.

✔ Call FDA at 1-866-300-4374 for all other foods.

✔ Call your local health department if you think you got sick from food you ate in a restaurant or from another food seller.

REFERENCES

WAC 296-335-425 WAC 246-335-430

Food Safety at Home (fda.gov)

FALL PREVENTION

Policy

All Golden Years Home Health Supported Living LLC clients are assessed at the time of admission to determine their risk for falls. Clients identified to be at risk will be given information on safety precautions that may be helpful in preventing falls. If the risks are significant, clients will be asked if they would like a referral for a skilled home care services evaluation visit to assess client eligibility for insurance covered skilled care to address the causes of the fall risks.

Purpose

1. To prevent potential falls for identified clients believed to be at risk.
2. To suggest interventions that will assist the client/representative to decrease or prevent falls.

Procedure

1. A falls risk review will be included in the client assessment for non-skilled cases.
2. The fall risk assessment will review predisposing conditions that may be potential fall risk factors.
3. A home safety inspection will be completed, and the client/representative will be provided with findings and recommendations to improve safety in the home.
4. Fall risk prevention activities will be on the service plan and assessed on each visit.

SMOKE FREE WORKPLACE

Policy

Because Golden Years Home Health Supported Living LLC recognizes the hazards of exposure to environmental tobacco smoke, it is the policy of the agency that the offices, company owned vehicles, and all client care residences and facilities will be designated as smoke-free workplaces for all personnel, contractors, and office visitors. The policy covers the smoking of any tobacco product or use of oral tobacco products.

Purpose

1. To protect personnel and visitors from smoke related hazards while in the office.
2. To prevent the production of smoke hazards in company vehicles or client's residences.

Procedure

1. No use of tobacco products will be allowed within the agency's offices or vehicles at any time.
2. No agency employee or contractor will be permitted to smoke in client residences.
3. If a designated smoking area outside the office is authorized by the agency:

n. It will be at least 20 feet from the main entrance.

o. Smoking materials will be extinguished and properly disposed of in an appropriate receptacle.

4. Breaks for smokers or non-smokers are based on agency guidelines
5. No smoking signs will inform personnel and visitors of the smoke-free workplace.
6. Violations of this policy will be handled through the corrective action process.
7. Strong smoking odors may cause adverse reactions for personnel and clients alike and offending personnel will be coached on methods to control the odors.

DRUG AND ALCOHOL-FREE WORKPLACE

Policy

Golden Years Home Health Supported Living LLC is committed to protecting the safety and health of all its personnel and clients by providing a drug and alcohol-free workplace. This policy balances respect for individual's rights and the need to maintain an alcohol and drug-free environment. This policy applies whenever an employee or contracted employee represents or working on behalf of the agency during all working hours or while on-call. Personnel with identified drug or alcohol issues are encouraged to voluntarily seek assistance.

Purpose

1. To be compliant with the Drug-Free Workplace Act of 1988.
2. To provide personnel with information about the drug and alcohol-free workplace.

Procedure

1. In compliance with the Drug-Free Workplace Act of 1988, the agency follows this policy regarding the work-related effects of alcohol and drug use and the unlawful possession of controlled substances on the agency premises or those of a client during working hours and assigned on call time and abides by the following:
   a. It is a violation of this policy to use, possess, sell, trade, and/or offer for sale alcohol, illegal drugs, misused prescription drugs, or intoxicants. Alcohol and drugs are strictly prohibited prior to or during work hours. Personnel are expected to report to work physically and mentally ready to provide safe and appropriate services to agency clients and to be fully functional in their work in or out of the office on behalf of the agency.
   b. The manufacture, distribution, possession, or use of a controlled substance on agency premises, or while conducting agency business off-site is absolutely prohibited. Policy violation will result in corrective action up to and including a requirement to participate in rehabilitation, termination of employment, and may have legal consequences.
   c. Personnel dealing with alcohol or drug addiction problems are encouraged to use available health resources and health insurance plans as appropriate. A concerted effort to seek such help will not jeopardize any employee's job and will not be noted in any personnel records if there are no incidents involving either drugs or alcohol.

   d. Personnel must, as a condition of employment, abide by the terms of the above policy and report any convictions under a criminal drug statute for violations occurring on or off agency premises while conducting business. A report of a conviction must be made within five days after the conviction. (This requirement is mandated by the Drug-Free Workplace Act of 1988.) The agency will take appropriate action within 30 days of notification. State contracting agencies will be notified when appropriate.

2. Communication about alcohol or drug incidents will be held as confidential.
3. Drug and alcohol testing procedures are covered under a separate policy and include pre-employment, random, and reasonable suspicion testing for drugs or alcohol.
4. Applicants who fail drug testing may not re-apply for one year and must successfully pass pre-employment drug testing.
5. To support its personnel, the agency will make available a current list of qualified community professionals for treatment of alcoholism and drug abuse.
6. The agency will confidentially investigate any reported or suspected incidents of work-related alcohol or drug abuse and expect the employees' full cooperation in the investigation.
7. If an employee is required to enter rehabilitation by court order and fails to successfully complete it or repeatedly violates this policy, they will be terminated from employment. Nothing in this policy prohibits the employee from receiving corrective action or termination for other violations and/or performance problems.
8. When an employee has successfully completed the prescribed program, he/she may be restored to available employment on the job held before entering rehabilitation.
9. The agency will cover the cost of required employee drug testing.
10. It is management's responsibility to:

   a. Inform personnel of the drug and alcohol-free workplace policy.
   b. Investigate reports of work-related alcohol or drug incidents.
   c. Document identified performance problems and/or results of investigations.
   d. Counsel personnel as to expected performance improvement.
   e. Clearly identify consequences of policy violations.
   f. Maintain confidentiality of alcohol and drug related incidents.

VIOLENCE IN THE WORKPLACE

Policy

Golden Years Home Health Supported Living LLC has a developed plan to reasonably prevent and protect personnel from violence in the workplace.

1. To ensure, as much as possible, protection of personnel from workplace violence.
2. To educate personnel regarding violence risks and appropriate steps to take in the event of a violent occurrence.

Procedure

1. The agency will, as a part of the initial client admission assessment, identify any potential for acts of violence and take appropriate steps as warranted.
2. Violence prevention will be a routine part of orientation and ongoing education for all agency personnel and will address the following topics:

g. General safety procedures in as described in several safety policies in this manual
h. Personal safety procedures in the office, the community, or client home
i. Violence escalation cycle and verbal and physical techniques to de-escalate and minimize violent behavior
j. Violence predicting factors
k. Strategies to avoid physical harm
l. Restraining techniques, only if necessary
m. Documentation and reporting of incidents
n. The process whereby personnel affected by violence may be counseled

3. A record will be kept of all violent acts against an employee, client or visitor in the agency office, client's home, or the community while on work assignment. The records will contain at a minimum:

    a. The Golden Years Home Health Supported Living LLC and address
    b. The date, time and location where the act occurred
    c. The name, job title, and social security number of the employee involved
    d. The name, demographics if the victim was a client/representative, or visitor
    e. Define whether the victim was a client, visitor, or employee
    f. Describe the type of violence as follows:
        i. Threat of assault with no physical contact

   ii. Physical assault with contact but no physical injury
   iii. Physical assault with mild soreness, surface abrasions, scratches, or bruises
   iv. Physical assault with major soreness, cuts, or large bruises
   v. Physical assault with severe lacerations, bone fractures, or head injuries
   vi. Physical assault with loss of limb or life

g. Description of any weapon used
h. Witnesses to the violent act or those in the area at the time of occurrence

4. A full-written report will be made describing the actions taken by the employee and the agency subsequent to the violence. The incident will be reported on the agency's Incident Report form. If the victim was an employee, OSHA Form 301 may need to be completed depending on the severity of the incident. OSHA 300 will be notified.
5. The reports will be incorporated into the QIP Plan.
6. All incidents are maintained in a confidential file.
7. If the violence resulted in injuries a medical evaluation of the victim will be sought.
8. If the involved client, employee, or visitor did not contact the local law enforcement to report the incident, the agency will make the report and fully cooperate in the investigation.

INTIMIDATION, DISCRIMINATION, AND HARASSMENT

Definitions

Intimidation is defined as undue pressure, bullying, or threats that negatively affect the job performance of both involved individuals.

Discrimination is defined as an unfair bias or prejudice that negatively affects and influences behavior toward another individual or group.

Harassment is any unwelcome nuisance, annoyance, or stalking either explicit or implied.

Sexual harassment is any unwelcome sexual advancement. All such behavior is strictly prohibited.

Policy

Golden Years Home Health Supported Living LLC holds that all personnel are entitled to a work environment free from all forms of intimidation, discrimination or harassment, including sexual harassment. This policy applies to all personnel and contractors. Any conduct, whether verbal, visual, or physical in nature is inappropriate, the agency will take all such reports seriously. If investigation confirms the activity occurred, corrective action will be initiated up to and including termination for cause.

Purpose

1. To ensure that the agency provides a positive work environment free of incidents of intimidation, discrimination, and harassment.
2. To ensure that the agency follows state and state rules regarding, intimidation, discrimination, and harassment, including sexual harassment regarding employee education, incident investigation, and follow-up.

Procedure

1. The agency will make every effort to ensure that all complaints related to intimidation, discrimination, and harassment or the existence of a hostile work environment are fully investigated.
2. Personnel making a report may do so without fear of retaliation.
3. Any employee found to be in violation under this policy will be subject to corrective action up to and including termination.
4. If the incident is the result of negative client/representative behavior toward an employee, the issue will be discussed with the client/representative and if unresolved could result in discharge or transfer of the client from agency services.

5. Any personnel who feel that they are being subjected to sexual harassment should immediately report the incident to their supervisor. If the offender is the supervisor, then the employee should report the incident to the Owner/Administrator.
6. All reports will be filed in a confidential file maintained by the Owner/Administrator.

REFERENCES

WAS 246-335-425

BEHAVIOR MANAGEMENT

Policy

Golden Years Home Health Supported Living LLC is aware that clients may display disruptive behavior because of their physical, mental, or emotional condition and will be treated with respect and dignity. Agency personnel receive instructions during orientation on the handling of disruptive and aggressive behaviors by clients/family. Punishment of any kind is never used to correct inappropriate or challenging behaviors.

Purpose

To ensure that all personnel are aware of the policy and are able to manage disruptive or aggressive behavior in an appropriate manner to resolve the situation.

Procedure

1. The agency experiences two primary forms of problem client behaviors: aggressive and non-cooperation.
2. These behaviors are documented in a narrative form by the agency personnel:
   a. The behavior that is presenting the challenge.

b. What the employee did to attempt to improve the behavior and the results.

3. The agency personnel notify the Supervisor of the behavior, what they did to gain cooperation, and the outcomes.
4. The Supervisor contacts the client/representative to review and discuss the behavior and the employee's attempts to gain cooperation. The discussion is an attempt to gain the client/representative assistance in altering the client's behavior. If this is successful, the actions needed to encourage compliance are added to the service plan.
5. An incident report is filed in the client file describing the circumstances and result.
6. In the event that client/representative cooperation cannot be achieved, the Supervisor will suggest the family contact the physician for alternate options for resolution of disruptive or aggressive behavior, and the service plan may be revised to include situational triggers or other positive behavior reinforcement techniques. If this does not work, the client may need to be transferred and/or discharged.
7. The agency educates its caregiving personnel in the area dealing with challenging behaviors and encourages the use of positive behavior reinforcements to alter challenging behaviors including:

a. Offering alternatives, discussing options and consequences of different behaviors

b. Redirecting client attention with soft spoken encouragement
c. Quietly ignore inappropriate behavior until client calms down

8. In an extreme case when an employee is threatened or at risk of harm, law enforcement officials may be summoned to assist.

EMERGENCY CARE

Policy

Golden Years Home Health Supported Living LLC personnel will be trained in emergency care procedures to implement appropriate emergency actions for clients in a crisis. If a client has a signed POLST form and any section of the POLST form is not completed, it is implied that there will be full treatment for that section. In the event of a client medical emergency and agency staff are present, provide emergency medical personnel with a client's signed POLST form

Purpose

To establish guidelines for actions to be taken by agency personnel in emergency situations.

Procedure

1. Medical emergency/life threatening situations:
   a. If an agency client has an emergency event, the employee should call the emergency rescue service (911) and follow the instructions of the dispatcher. Notify the Supervisor of the situation in progress.
   b. If the client has a DNR [POLST] order, do not initiate CPR.
   c. Stay with the client until the emergency rescue service arrives
   d. Document completely in the client record the circumstances of the emergency
2. Non-life-threatening emergencies for Home Care Aids and Homemakers:

a. Contact the Supervisor and report on the situation.
b. The Supervisor will determine appropriate interventions for the situation and assist in implementing a plan of action.

3. Inability to gain access to the client’s residence:

a. If an agency employee cannot gain access to the client’sresidence, (i.e., client does not answer the door or employee has reason to believe that the client may not be safe), the employee should pursue ascertaining the client’s condition.
b. Notify the Supervisor who will contact caregiver to determine if there is a reason for the client’s failure to respond.
c. With supervisory/family approval, contact the police to gain access to the home if it remains uncertain regarding the client’s whereabouts or condition.

EMERGENCY/DISASTER PREPAREDNESS AND PROCEDURES

Policy

Golden Years Home Health Supported Living LLC has an established plan for the services of its clients in the event of inclement weather, natural disaster, terrorist attack, or other emergencies. The emergency/disaster plan provides an orderly procedure to be implemented to ensure that the immediate needs of clients continue to be met to the greatest extent possible. All personnel are oriented to the plan and their responsibilities in carrying out the plan. Possible emergency or risk factors will be identified for each client and appropriate emergency plans discussed with the client and/or the person responsible at the time of admission. The name and telephone number of an emergency contact is obtained at the time of admission so that the plan can be implemented to the benefit of all clients. As part of its preparations, the agency has established a triage process for clients based on the severity of the emergency, so emergency resources can be allocated based on availability of employees, supplies, transportation, utilities and communication systems. Due to the variations in emergency and disaster circumstances, clients' triage must be based on realistic expectations of meeting client needs versus allocation of resources. As a result, the agency will, on admission and when client experiences significant changes, assign a triage category to each client to be used in an emergency if the Owner/Administrator authorizes triage considerations. The most common natural disasters in areas of operation are Blizzards, Snow & Ice, floods & fires.

Purpose

1. To provide direction and support for personnel during work interruptions.
2. To ensure that those clients are triaged to best serve their needs and efficiently allocate the agency's resources.

Procedure

1. The Emergency/Disaster Plan may be invoked for the following:
    d. Natural Disaster: Blizzards, Snow & Ice, floods, fires, hurricanes, tornados, etc.
    e. National Disaster: War, poisonous gas, nuclear accident, bioterrorism
    f. Community Emergency: Strike, transportation accident, blackout, pandemic, etc.
2. The Owner/Administrator/designee is responsible for declaring that an emergency plan is to go into effect and implementing the plan as follows:

a. Receiving communication regarding a disaster and disseminating it.
b. Activating, implementing, and discontinuing the plan.

c. Notifying the telephone answering service, if functional, of implementation of the emergency plan.

d. Appointing emergency coordinators who are available to assist with triage, transportation, staffing, client communications, and follow-up.

3. Should the emergency be a fire in the office, the agency shall have an evacuation plan with marked exits and create a map for evacuation of office premises. The fire plan will include designated sites for fire extinguishers and emergency and first aid supplies. Staff evacuation and securing key documents and files will begin as soon as the emergency is identified and 911 called. A manager will assemble personnel at the designated location and is responsible for assuring that all personnel and visitors leave the building.

4. The agency will maintain a list of all active personnel, their home and cell phone numbers, as well as those personnel who have emergency-prepared vehicles, i.e., four-wheel drive vehicles, snowmobiles, boats, etc.

5. The Owner/Administrator/designee will assign an activity to all key personnel (internal disaster emergency assignments) such as who will answer the phone, be in charge, and gather and remove key items from the office (client's charts, schedules, payroll etc.)

6. The emergency coordinators will maintain contact with the Owner/Administrator to report on employee status and will assign client contacts as directed by the Clinical Supervisor. The client roster will be used by the Scheduler/Client Care Coordinator to notify all clients of emergency/disaster arrangements.

7. The agency will on admission, at recertification, or because of significant change in client's condition assign a triage category (based on client needs and the likelihood of emergency/disaster survival) to each agency client to be used in an emergency if the Owner/Administrator authorizes the use of the triage considerations. The triage information will be located on the client roster.

8. Access to the client roster with triage assignments will be accessible to management personnel either in the manager's disaster kit or mobile device, both of which must be secured to protect PHI.

9. Depending on the severity of the emergency/disaster, the triaging of clients will be less or more significant. The goal is that all client needs will be met, and circumstances will determine how realistic that goal will be. The initial triage rating is:

a. Lowest: those with good caregiver support

b. Medium: those with some essential ADL needs and some caregiver support

c. Highest: those with significant ADL/IADL needs and little or no caregiver support.

10. An office employee will be assigned as an on-call employee in an emergency and is responsible for collecting, disseminating, and maintaining supplies and first aid kits.

11. A telephone tree for the in-office personnel to be notified of the needs of the office is developed and updated no less than quarterly.

12. If phones are usable, they may be used to give out assignments. The main telephone line will be forwarded to this direct line, and this line will be forwarded by the lead disaster Owner/Administrator on site or through the phone company's lines if they cannot get to the office.

13. In the event that the electricity goes out and the landlines are not usable, cell phones will be used to contact personnel. If cell phones are not usable, the direct line phone established for emergency use will be used for communication with personnel and located in the staffing office. Call forwarding and call waiting are installed on this line, so that it can be used as the main line of communication if no electricity.

14. Staff on assignment with a client will remain with the client until it is safe for them to leave the client.

15. There will be designated meeting sites established by the administration so that personnel can meet to take assignments. Both office and field personnel will be expected to go to the established site nearest their home or their last assignment.

16. Office personnel attempt to contact the families of the workers who are on assignment to let them know of the employee's situation and to communicate to the personnel anything they need to know about or from their family.

17. Staff not on assignment will be requested to assist the agency with services, as well as the authorities who will be supervising the services needed to the community.

18. All personnel will be oriented to the emergency preparedness plan and their assignments in the event of such an occurrence.

19. The agency will attempt to contact and remain in contact with the Emergency Disaster System in the event of a disaster and will coordinate efforts with them on behalf of the clients.

Provision of Services in Preparation for an Emergency

1. On admission, the agency representative will discuss with client/representative the emergency provisions, including where and how the client is to evacuate, procedures for notifying the agency in the event that the client evacuates to a location other than the shelter identified in the client record, and a list of medications and equipment the client will need to take or be provided with if evacuated.
2. Each client record will include a description of how care or services will be continued in the event of an emergency or disaster.
3. The agency shall maintain a current prioritized list of clients who need continued services during an emergency. The list will be furnished by the county Department of Health and by local emergency management agencies, upon request with client permission related to PHI.
4. The agency will not be required to continue to provide service to clients in emergency situations that are beyond their control and that make it impossible to provide services such as when roads are impassable, when clients do not go to the location specified in their client records, or unnecessary travel is ordered by authorities during the emergency.
5. To the extent possible, recordkeeping during a disaster will be thorough and complete. Attempts to notify personnel and clients and the outcome, including alternate plans, should be documented.
6. The Owner/Administrator will document all disaster/emergency activities. Contacts with clients will be documented in the client record. As appropriate, contacts with employees may be documented in the employee's personnel record.
7. The Owner/Administrator will periodically and at least annually review the plan and propose any recommendations for change to the Owner. The review and subsequent recommendations will be based on a hazard vulnerability analysis to identify potential emergencies and the effects such emergencies may have on the agency
8. The Owner/Administrator will declare when the emergency/disaster plan is discontinued.

Inclement Weather

1. All personnel are expected to contact the Owner/Administrator/designee directly for instructions regarding operations and client service during inclement weather. All field personnel are to call the answering service to find out if the agency is open for business and receive further instructions. Office personnel able to report to the office are expected to do so unless instructed otherwise.

2. If inclement weather conditions exist prior to the opening of the office, the Owner/Administrator or their designee will advise the on-call personnel, at least one hour before the office opens, if possible, whether the office will be open, closed, or if the opening will be delayed.
3. If the office is open, all personnel will be expected to report to work unless otherwise directed by their supervisor. If the employee is unable to report to work, the employee must notify their supervisor, so client services visits may be reassigned. The employee may utilize annual leave for the missed time. Absence without proper notice will not be paid.
4. If inclement weather develops during the workday, the Owner/Administrator/designee will make a decision regarding closing the agency early.
5. According to the disaster plan, high-priority clients will be notified of the emergency to arrange for client support. Instructions or assistance may be provided with transportation to shelters.
6. If the office telephone service is out of order, operations will be maintained out of an alternate location designated by the Owner/Administrator, and the on-call personnel will be contacted to alert the telephone company of the need for priority service.
7. If no telephone service is available in the area, state and local police, fire stations, hospitals, and client's physicians and emergency contacts will be notified of high-priority clients. The agency will attempt to assist high-priority clients to obtain shelter and safety.

Fire, Earthquake, Tornado, Hurricane, Flood

1. If the building must be abandoned in a hurry, internal personnel will be assigned items that they should take with them. The on-call book, the payroll readout and/or paychecks, the client’s charts, etc. are assigned to those in the office who can get these items on their way out.
2. Supplies for the office to last at least 72 hours are stocked in a designated supply cabinet. The kit shall include:
   a. Non-perishable food and clean, bottled water
   b. Items to light the area, such as candles or flashlights
   c. Supplies to keep the personnel warm, such as blankets, extra clothes, etc.
3. Fire extinguishers, first aid kit(s), and other items are prepared and periodically reviewed and maintained.
4. A battery-powered radio is kept in the main supply cabinet. Fresh back-up batteries are kept in the fridge in the personnel’ office.

A Personal Emergency Care Kit for Field Personnel

1. The emergency care kit may include some of the following items:
   a. An emergency blanket
   b. Bottled water for drinking and for the radiator
   c. Extra oil, transmission or brake fluid
   d. Flat fixer (an aerosol can fill with a material that will inflate and plug the hole in the tire until it can be repaired)
   e. A spare tire
   f. Radiator fixer (similar to the flat fixer)
   g. Food
   h. A store of necessary medications for 2–3 days
   i. Emergency phone numbers
   j. A battery-powered radio
2. The American Red Cross pamphlet on emergency kits is handed out to personnel at orientation to help them prepare their autos for the worst.
3. All agency personnel are oriented to what is required of them in the way of preparation, and what is optional. If an employee is responsible for emergency supplies, it may be possible for the agency to assist with the cost of some of the items or negotiate a deal with a local supplier for a good price.
4. The American Red Cross lists information for emergency services that is maintained in the emergency preparedness binder. The sites of emergency shelters are updated depending on the disaster and are to be accessed to assist personnel in the field to make sure clients needing shelter are aware of the site and to make arrangements to get the client to the site.

Interruption of Public Transportation

1. Staff that use public transportation will be instructed to arrange for transportation from relatives, friends, or carpool to client's homes, if possible. Confidentiality of client information must still be maintained.
2. At the discretion of the Owner/Administrator, personnel may be provided with assistance with transportation by supervisors, taxi, or other personnel.

Electrical Backup Systems (Optional)

1. The agency shall maintain, test, and inspect its emergency power systems to its office, on a regular basis.

2. The agency tests all automatic transfer switches 12 times a year, with testing intervals not less than 20 days and not more than 40 days apart.
3. The agency tests each generator 12 times a year, with testing intervals not less than 20 days and not more than 40 days apart. These tests shall be conducted for at least 30 continuous minutes under a dynamic load that is at least 30% of the nameplate rating of the generator.
4. The agency will test all battery-powered lights required for egress. Testing includes (a) a functional test at 30-day intervals for a minimum of 30 seconds, and (b) an annual test for duration of 1.5 hours.
5. If a test(s) fails, the agency will perform a retest after making the necessary repairs or corrections.

Staff Emergency Preparedness Education

6. All staff will receive the Emergency Preparedness plan at orientation.
7. The plan will be reviewed annually with all internal and external caregiving staff.
8. The agency will conduct annual fire drills for the office and all client homes,
9. All clients will have an escape plan in place in the event of the need to evacuate the clients from their home.

REFERENCES

WAC 246-335-415

Hazard Vulnerability Analysis (HVA)

1. **Introduction**: acknowledges the importance of preparedness for natural disasters, man-made incidents, and public health emergencies to ensure the continuity of service delivery to our clients. This Hazard Vulnerability Analysis (HVA) aims to assess risks, develop emergency plans, and establish protocols to effectively respond to emergencies while adhering to state and local government policies and regulations.
2. **Risk Assessment and Emergency Planning**: a comprehensive risk assessment to identify potential hazards, as per the table Conduct below

NATURAL DISASTERS	MAN-MADE DISASTERS	TECHNOLOGICAL FAILURES	OTHER
✔ Hurricanes ✔ Heavy thunderstorms ✔ Flash flooding ✔ Flooding ✔ Mud/rock slides ✔ High winds ✔ Hail ✔ Severe winter weather ✔ Avalanche ✔ Extreme high heat ✔ Wildfire ✔ Earthquake ✔ Volcano eruption ✔ Tidal wave/Tsunami	✔ War (Conventional, biological, chemical or nuclear) ✔ Toxic material emission/spill (from a train or nearby plant) ✔ Riot or other civil disorder ✔ Nuclear plant melts down or other nuclear disaster ✔ Terrorism ✔ Fire	✔ Electrical ✔ Communications ✔ IT system ✔ Heating /cooling	✔ Disease outbreak ✔ Community infrastructure breakdown (Bridges collapse, Dam breaks, etc.) ✔ Utility failure ✔ Transportation failure

a. Develop emergency plans tailored to each identified hazard, outlining response procedures, evacuation routes, and resource allocation strategies.

3. **Hazard Vulnerability Assessment:**

a. Vulnerabilities to identify hazards will be evaluated based on factors such as geographic location, infrastructure, client demographics, and regulatory requirements, following the guidelines outlined by WACS.

Below are the hazard vulnerability assessments

b. Prioritize hazards based on their likelihood and potential impact on service delivery.

c. Implement mitigation measures to reduce vulnerabilities and enhance resilience.

Hazard Vulnerability Assessment

Event	Probability			Level of vulnerability / degree of disruption			Preparedness			Score
	High (3)	Moderate (2)	Low (1)	High (3)	Moderate (2)	Low (1)	Low (3)	Moderate (2)	High (1)	
Ice Snow		✓			✓				✓	4
Flooding			✓			✓		✓		2
Earthquake			✓			✓			✓	1
Hurricane			✓			✓			✓	1
Hazardous Material Accident		✓			✓				✓	4
Fire		✓			✓				✓	4
Tornado			✓			✓		✓		2
Volcano		✓			✓				✓	4
Civil Disturbance			✓			✓			✓	1
Mass Causality Event			✓			✓			✓	1
Terrorist Attacks			✓			✓			✓	1
Pandemic/Infectious Disease		✓			✓				✓	4
Electrical failure		✓			✓				✓	4
Communications Failure		✓			✓				✓	4
Information System Failure		✓			✓				✓	4
Water failure		✓			✓				✓	4
Transportation Interruption			✓			✓			✓	1
Environmental/ Altered Air Quality Pollution			✓			✓			✓	1

4. Incident Command System (ICS):

While developing the (ICS) will put in mind and consideration of the state ICS structure and purpose. ICS is part of the broader incident management system outlined in the Department of Homeland Security's National Incident

Management System (NIMS).

a. Establish an Incident Command System (ICS) structure to facilitate efficient coordination and management of emergency response efforts.

b. Designate key personnel roles and responsibilities within the ICS hierarchy.

c. Conduct regular training and drills to ensure familiarity and proficiency with the ICS framework.

5. **Emergency Preparedness Plan**:
   a. Develop a comprehensive emergency preparedness plan that encompasses all aspects of emergency response and recovery.
   b. Include protocols for activating the emergency response plan, communication procedures, and resource management strategies.
   c. Review and update the plan regularly to reflect changes in hazards, resources, and organizational structure.

Emergency Preparedness Plan All-Hazard Emergency

This plan uses the term “all hazard” to address all types of incidents. An incident is an occurrence, caused either by humans or by a natural phenomenon, which requires or may require action by home care and emergency service personnel to prevent or minimize loss of life or damage to property and/or the environment.

Plan Activation/Deactivation

The Director, who serves as the Incident Commander, has the authority to activate and deactivate this Emergency Preparedness Plan based on information known to her/him at the time which indicates such need. If the Director is not available, the Assistant Director, and then the Chief Clinical Officer will have the authority to activate the response plan.

Goal: Allow smooth transition of patient services and ensure continuity of care for all patients served by this agency.

Objectives

- To identify the chain of command /Incident Command System
- To identify primary and alternative command centers
- To allow for the timely identification of the patients who are affected in case of an emergency.
- To provide those patients with the care and assistance that they need in the event of an emergency.
- To be readily available to assist emergency responder personnel in first aid care for those in the community.
- To assess patient’s home environment for safety and assist them to a safe environment if needed.

- To coordinate Agency staff members in patient care and evaluation, as well as any Agency personnel assistance with care of those in the community who are affected by the emergency. Identifying staff roles and responsibilities

6. **Communication Plan**:
   a. Establish a robust communication plan to facilitate timely and accurate dissemination of information during emergencies.
   b. Identify primary and alternate communication channels for internal staff, clients, families, and external stakeholders.
   c. Ensure mechanisms for two-way communication to receive feedback and address concerns promptly.
7. **Coordination of Service Delivery with Emergency Personnel**:
   a. Foster partnerships with local emergency management agencies, healthcare providers, and community organizations to coordinate service delivery during emergencies.
   b. Participate in collaborative planning efforts and joint exercises to enhance interoperability and resource sharing.
   c. Establish protocols for seamless integration with emergency response systems to meet emergent needs of clients.
8. **Staff Training**:
   a. Provide regular training sessions for staff on emergency preparedness, including hazard recognition, response procedures, and personal safety measures.
   b. Offer specialized training for designated emergency response teams, such as first aid, CPR, and incident command.
   c. Encourage ongoing education and skill development to maintain readiness for emergencies.
9. Business Continuity Plan:
   a. Develop a business continuity plan to ensure the uninterrupted provision of essential services during and after emergencies.
   b. Identify critical functions, dependencies, and recovery strategies to minimize disruption to operations.
   c. Test and refine the business continuity plan through simulation exercises and post-event reviews.

In implementing the Hazard Vulnerability Analysis (HVA) and associated emergency preparedness measures, will adhere to relevant state and local government policies,

regulations, and Washington Administrative Codes (WACs) to promote compliance and ensure the safety and well-being of our clients, staff, and community.

References:

- Washington state Emergency Management Agency (FEMA) guidelines and resources.
- Washington Administrative Codes (WACs) related to healthcare facility emergency preparedness.
- Local emergency management plans and procedures.
- Best practices in emergency management and healthcare continuity planning.

HOME ENVIRONMENT AND SAFETY ASSESSMENT

Policy

All admission.

PURPOSE

Golden Years Home Health Supported Living LLC clients will have a home safety assessment as part of assessment. To ensure that the home environments of agency clients are safe and free from potential safety hazards.

Procedure

1. As part of the admission process, the home environment will be checked for any potential hazards and identified to the client/representative.
2. A request will be made to remove hazards or instructions will be given to minimize the potential for harm.
3. The discussion of the identified hazards and the request to remove or minimize will be documented in the assessment form.
4. A floor plan with escape routes identified will be drawn and left in the home. A copy will be put in the client's file.
5. The evacuation plan will be discussed with the client/representative and will be available to other personnel providing client services.

OSHA 300/300A/301 LOGS

Policy

Golden Years Home Health Supported Living LLC will maintain a record of occupational injuries and illnesses. The information will be documented on the OSHA 300/300A and 301 Logs maintained as a confidential administrative file. A summary of this information will be incorporated into QIP and reported to the Governing Body at least annually.

Purpose

1. To identify areas for performance improvement and to strive to provide a safe work environment for personnel.
2. To meet OSHA requirements for recording and maintaining records of occupational injuries and illnesses.

Procedure

1. The Owner/Administrator/designee is responsible for completing, maintaining, and posting the OSHA 300 and 300A Logs, if required in accordance with OSHA recordkeeping requirements. Log 301 is maintained in a confidential OSHA file regarding the specific employee's injuries and incident.
2. All injuries are recorded. Work–related injuries requiring medical treatment (other than first aid) or involving loss of consciousness, restriction of work or motion, or transfer to another job are recorded on the OSHA 301 form and reported within seven days.
3. Medical treatment does not include: one–time treatment and/or subsequent observation of minor scratches, cuts, burns, splinters, etc., which do not ordinarily require medical care provided by some physician or registered professional personnel.
4. Exposure to and/or contraction of reportable communicable diseases in the course of work performance are considered reportable on the log.
5. Employee needle sticks with contaminated needles must be recorded on the log.
6. Each injury or illness is recorded within five days after learning of the occurrence and maintained for each calendar year.
7. The OSHA 300 Log is retained for a five-year period and available for inspection by personnel, former personnel, and authorized Washington state officials.
8. The "Annual Summary" half of the OSHA 300 Log for the preceding year is posted with employee notices each February 1 to March 1. The log is posted even if there are no recorded injuries or illnesses.
9. The OSHA 300 Log may be obtained from the regional OSHA office or www.osha.gov.

EMPLOYEE PROTECTIVE EQUIPMENT

Policy

To protect clients and personnel from cross-contamination exposure, Golden Years Home Health Supported Living LLC will provide protective equipment or materials (at no cost) to all personnel involved in client services with the potential for exposure to bodily fluids or contaminants. This equipment may be included but is not limited to gloves, gowns, aprons, masks, eye protection, facemasks, and CPR shields.

Purpose

1. To protect clients from cross-contamination and assure that personnel are protected against possible exposure to infectious pathogens.
2. To comply with CMS, CDC, Agency for Healthcare Research and Quality (AHRQ), state practice acts, and generally accepted standards of practice for infection control and prevention.

Procedure

1. The agency will use the CDC website for current information on best practices for infection control and prevention procedures
2. The employee will use protective equipment when there is exposure or the possibility of exposure to blood, body fluids including urine, vomitus, feces, sputum, amniotic, pericardial, peritoneal, pleural, synovial, and cerebrospinal fluids, semen and vaginal secretions, or anybody fluid visibly contaminated with blood.
3. The agency will ensure that personal protective equipment in appropriate sizes is issued to the employee and carried into the client's place of residence when performing services. If a garment is contaminated, it should be removed as soon as feasible and disposed of in accordance with the agency's policy on infectious and hazardous waste.
4. Gloves, gowns, aprons, and other protective equipment shall be worn at any time an employee is at risk of occupational exposure.
5. Infection control is an integral part of the agency QIP program and incidents of non-compliance or variances will be included in the performance improvement collection process for analysis and determination of remedial actions.

APPENDIX — CAREGIVER PROGRESS NOTES

Home care aide · homemaker · home health aide visit notes — page 1 of 2

Client name: _____ **Record #:** _____ **Month / year:** _____

Primary caregiver: _____ **Service:** _____

Write a note at the end of every visit, in ink, before you leave the home. Describe the care you gave (per the care plan), what you observed, any change in the client's condition, mood or appetite, and how the client responded. Report falls, injuries, new pain, skin changes, refusals or anything unusual to the Supervisor right away and note who you told. Do not leave blank lines between entries; if you make an error, draw one line through it and initial it.

#	Date	Time in / out	Care provided · observations · changes in condition · client response	Caregiver name & initials
1				
2				
3				
4				
5				
6				
7				
8				

CAREGIVER PROGRESS NOTES

Home care aide · homemaker · home health aide visit notes — page 2 of 2

Client name: _____ **Record #:** _____ **Month / year:** _____

Primary caregiver: _____ **Service:** _____

#	Date	Time in / out	Care provided · observations · changes in condition · client response	Caregiver name & initials
9				
10				
11				
12				
13				
14				
15				
16				

Reviewed by (Supervisor / RN) — signature _____ Date _____